

**A Descriptive Survey Examining the Compared Experiences of Non-Artist Adults
Working With Collage and With Drawing Materials in an Initial Art Making
Session**

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Table of Contents

LIST OF TABLES	viii
LIST OF FIGURES.	ix
ABSTRACT.....	x
1. INTRODUCTION	1
2. LITERATURE REVIEW	9
Overview	9
Anxiety and/or Fear in Early Stages of Therapy.....	9
Importance of Addressing Anxiety Early in Therapy	12
Anxiety and/or Fear in Early Stages of Art Therapy.....	17
Materials and Structure in Art Therapy.....	22
Collage	33
Summary	39
3. METHODOLOGY	41
Design.....	41
Location of Study	42
Data Collection Time Frame	43
Enrollment Information.....	43
Participant Type	43
Subject Source.....	43
Recruitment	44
Participant Inclusion Criteria	44
Participant Exclusion Criteria	44

Instrumentation.....	45
Questionnaire	45
Anxiety Scale	45
Investigational Methods and Procedures.....	45
Data Collection.....	46
Data Collection I - Informed Consent.....	46
Data Collection II - Preliminary Information Collection.....	47
Data Collection III - Art Process	47
Data Collection IV – Interview.....	49
Data Analysis	50
Operational Definitions of Concepts and Terms.....	51
Possible Risks and Discomfort to Participants.....	51
Special precautions to minimize risks or hazards	51
4. RESULTS	53
Major Findings	53
Participants	54
Quantitative Data.....	55
Verbal Associations in Relation to Anxiety Scale	58
Increased Anxiety During the Art Tasks	58
Anxiety Change During Drawing Portion	58
Anxiety Change During Collage Portion.....	59
No Changes in Anxiety Level.....	60
Time Spent on Art Tasks	60

Interviews	60
Working With Collage	66
Collage Seems Easier and Less Intimidating Than Drawing.....	66
Challenges to Working With Collage	68
Collage Encourages Exploration and Insight.....	71
Collage “Easy, But It’s Not Complete”	71
Need to Provide Appropriate Materials	72
Working With Drawing Materials.....	72
Aesthetic Preconceptions	72
Fear That Art Could Be Misinterpreted or Lose Meaning.....	73
Feeling Intimidated by Blank Page and Not Having Ideas	74
Working With Unfamiliar Materials.....	75
Drawing Offers More Opportunity for Freedom of Expression	76
Comparing Collage and Drawing.....	78
Artwork	79
Analysis of Art and Associations	102
Collage Influencing Composition of Drawing.....	104
Collage Allowing the Depiction of Strengths.....	105
Major Themes From Anxiety Scales, Interviews and Art.....	105
5. DISCUSSION	107
Overview	107
Collage	108
Collage as Structured Medium.....	110

Mechanics of Collage-Making.....	113
Allows for Depiction of Strengths	116
No Aesthetic Preconceptions	117
Focuses Creativity.....	118
Not Culture Bound.....	119
Influence of Directive	120
Drawing.....	121
Aesthetic Preconceptions	122
Fear of Being Misunderstood.....	124
Anxiety When Facing a Blank Page	125
Using Unfamiliar Materials	125
More Opportunity for Free Expression.....	126
Clinical Implications	130
Collage Selection	130
Clinical Applications	132
Limitations and Delimitations of the Study	146
Implications for Future Research	148
6. SUMMARY AND CONCLUSIONS	152
LIST OF REFERENCES.....	156
APPENDIX A: RECRUITMENT FLYER.....	161
APPENDIX B: INFORMED CONSENT FORM	162
APPENDIX C: PRELIMINARY INFORMATION FORM	167
APPENDIX D: ANXIETY SCALE	168

APPENDIX E: INTERVIEW GUIDE.....	169
APPENDIX F: SAMPLE COLLAGE SHEET – IMAGES	170
APPENDIX G: SAMPLE COLLAGE SHEET – WORDS AND PHRASES	171

List of Tables

1. Participant Information	55
2. Order of Task and Self-Reported Anxiety Ratings.....	56
3. Paired T-Tests Comparing Means of Anxiety Scale Ratings	57
4. Aggregation and Summary of Open Ended Survey Responses.....	62
5. Summary of Analysis of Artwork.....	80
6. Tony – Analysis of Artwork	82
7. Joy – Analysis of Artwork	84
8. Emma – Analysis of Artwork	86
9. Raul – Analysis of Artwork	88
10. Hannah – Analysis of Artwork	91
11. Sid – Analysis of Artwork	94
12. Lia – Analysis of Artwork	97
13. Bill – Analysis of Artwork.....	100

List of Figures

1. Tony	81
2. Joy	83
3. Emma	85
4. Raul	87
5. Hannah	90
6. Sid	93
7. Lia	96
8. Bill	99

Abstract

A Descriptive Survey Examining the Compared Experiences of Non-Artist Adults Working With Collage and With Drawing Materials in an Initial Art Making Session

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This study examined the experiences of non-artist adults making a piece of art about themselves using two popular art therapy approaches, collage and drawing, with the purpose of understanding how the characteristics of each approach might affect the anxiety experienced in initial art therapy sessions. The study used a descriptive survey design in which eight volunteer student participants, ages 18-27, responded to their experience by rating their anxiety level before and after each task, by choosing which approach to start with, and by describing their experience. Clinically significant findings were that drawing evoked significantly more anxiety than collage because of:

- preconceptions about what constituted an acceptable piece of art
- concerns that meaning could be lost or misunderstood when an idea was translated onto the page because of limited drawing abilities
- concerns about facing the challenge of the blank page and coming up with ideas for the artwork, and
- concerns about the challenge of having to communicate using the unfamiliar medium of drawing materials

Findings also indicate, however, that drawing:

- can be more personal, because the imagery produced is unique to the creator
- is not bound by rules or structure
- promotes creative problem-solving to address artistic limitations, and

- can provide a positive sensory experience that collage does not seem to offer.

Collage was approached with significantly less anxiety than drawing because:

- the process itself was perceived as being easier
- it provided structure in the form of visual stimuli, and
- it involved a controlled, cognitive process that allowed for the safe exploration of self, insight and self expression, and for the depiction of strengths.

Limitations identified when working with collage were that:

- it required the individual to work with someone else's selection and someone else's imagery, and
- it required a level of organization that seemed to exacerbate anxiety in some individuals.

While drawing seemed to evoke the fears of failure, being misunderstood, shamed or revealing too much of oneself too soon that many clients new to therapy often experience, collage seemed to address these concerns. Seven participants used collage to make pieces that were considered more revealing of emotional content. At the same time, there were limitations imposed by the collage-making process, in that it compelled the artist to choose from images selected and prepared by someone else. Four of the drawings depicted feeling states, while the collages primarily addressed self-identity. This may have been due to the directive, but also could be due to the nature of collage, which involves selecting disparate images and words and assembling them into one piece. Findings suggest that collage can help address the anxiety of new clients because of its non-threatening nature, while drawing can be used to exacerbate that anxiety when needed. Collage can also act as a bridge to creativity by taking the familiar and asking

individuals to use it in a new way. Collage can also be used as a basis for the incorporation of less familiar materials. Directive and selection are important considerations. Once the client has gained some familiarity with drawing materials, however, and a therapeutic relationship has been established, it would appear that drawing is more effective as a tool for the expression and examination of emotions.

CHAPTER 1: INTRODUCTION

The purpose of this study was to describe a non-artist adult's experience of making a piece of art about him or herself using two approaches, collage and drawing materials. The objective was to understand how the characteristics of each approach might affect the anxiety experienced in initial art therapy sessions. The issue being addressed was the anxiety and fear that individuals may experience when starting art therapy.

My interest in this subject stemmed from an experience I had, during my first practicum, with an extremely resistant adolescent client. The one drawing she made during our time together was so fraught with anxiety that she could barely complete drawing the ground on which her house was supposed to be standing. Every line in the drawing was made with small, anxious strokes. At that point, I was a little dismissive of collage as an art therapy approach, but I decided to try it with this client, with her consent. Rather than hand her magazines that she could then spend the session reading, I did some research and found Vick's (1999) article on the use of pre-selected materials with adolescent clients. I cut out a few images based on Vick's (1999) recommendations regarding categories, and presented them to the client. In about 10 minutes, she had completed a collage that not only was full of information, but that she was willing to talk about. I believe that the relationship we ultimately developed began at that point, and that if it had not been for the non-threatening nature of collage, we might never have established it. The approach became part of my arsenal as an art therapist, but my curiosity was definitely aroused as to what it is about collage, in comparison to drawing,

that makes it so less threatening to some. This study was designed to try to answer that question.

The study used a descriptive survey design in which adult students who are unused to making art were asked to make two pieces of art about themselves on 11 x 14 inch paper, one using a selection of magazine images, phrases and words, and one using drawing materials, in this case a #2 pencil, colored pencils, thin markers, oil pastels and chalk pastels. Before and after each art task, participants were asked to rate their level of anxiety on a scale of 0 – 5 that was specifically designed for this study. The researcher then asked them to describe their experiences, using an open-ended interview guide.

Findings of this study may add to the art therapist's knowledge of how individuals new to therapy might respond to being asked to make art using collage and traditional drawing materials, two approaches that are popular with, and widely used by, the art therapy community. An enhanced awareness of the characteristics of the two approaches will help art therapists make more informed decisions about which one to use when working with new or anxious clients.

Individuals starting therapy often experience anxiety and fear (Brodsky, 2011; Gabbard, 2005; MacKinnon, Michels & Buckley, 2006; Restifo, 2010; Tantam, 2002). Therapy can be a frightening, disempowering experience in which the client is apprehensive about being judged, rejected, or shamed by the therapist (Gabbard, 2005; MacKinnon et al., 2006; Restifo, 2010; Tantam, 2002). The process can also be perceived as intrusive (Brodsky, 2011) and requiring insight and change that will be distressful and shameful (Gabbard, 2005; MacKinnon et al., 2006; Restifo, 2010).

In art therapy, this initial anxiety can be exacerbated by the need to express oneself in new ways (Comfort, 1985; Stone, 1991; Wadeson, 1987; Waller, 1993; Wittenberg, 1974). Anxiety may stem from the fact that many individuals have not used any art materials since leaving school (Lowenfeld & Brittain, 1987; J. Rubin, 2010; Sarason, 1990; Waller, 1993). There are also aesthetic expectations about what constitutes art, and accompanying fears of being judged on the final product (Sarason, 1990; Wadeson, 1987; Waller, 1993). Fear of revealing too much of oneself too soon may be another factor (Wadeson, 1987; Waller, 1993).

Unless the new client's feelings of anxiety are addressed early in therapy, their engagement in the therapy process may be at risk. As far back as 1913, Freud described the need to establish rapport between therapist and client before interventions are possible (Freud, 1913). Over the past decades, research has consistently linked therapeutic outcome to the nature of the alliance (Gomes-Schwartz, 1978; Horvath, 2001; Horvath, Del Re, Fluckiger, & Symonds, 2011; Horvath & Symonds, 1991; Martin, Garsky, & Davis, 2000). Horvath et al. (2011) undertook a research synthesis of the relationship between alliance and therapy outcome, and found a robust overall relationship in individual psychotherapy, regardless of type of therapy, how the alliance is measured, and other variables (Horvath et al., 2011).

Verbal therapists can help address feelings of anxiety by asking non-threatening, routine questions and by using body language, tone of voice and appropriate verbal responses to put the client at ease (Hilsenroth & Cromer, 2007; Horvath et al., 2011; MacKinnon et al., 2006). Hilsenroth and Cromer (2007) conducted a review of the research on therapeutic alliance and found that:

specific therapist attitudes that convey empathy, support, exploration, activity, confident collaboration, appreciation, trust, warmth, attunement, potency, competence, respect, attentive, engaged listening, and appearing to understand the patient in a nonjudgmental and welcoming fashion have also been found to significantly improve alliances with patients in pretherapy and initial sessions of psychotherapy (p.216).

Addressing a client's initial anxiety is no less vital in art therapy. Robbins (1987) wrote that to facilitate the therapeutic process, it is important to show empathy when creating the appropriate holding environment. This empathy must take into consideration not only the normal anxiety felt at the start of therapy, but also the concerns relating to the need to express oneself in unfamiliar ways, using unfamiliar materials (Comfort, 1985; J. Rubin, 2010; Sarason, 1990; Wadeson, 1987; Waller, 1993; Whalen, 2004).

In addition to the attitudes adopted by verbal therapists to establish empathy and encourage the alliance, art therapists also have a powerful tool in the range of art materials, and in the directives, available to them. Art therapists are aware that art media at the structured and more familiar end of the materials continuum, such as pencils, markers and collage, can be less daunting than looser materials like clay or paint. Not only are they easier to control and offer less chance for regression or overstimulation (Linesch, 1988; Robbins & Sibley, 1976), but they are also more familiar to clients, most of whom may not have made any art since leaving elementary or middle school. Art therapists also know that different materials and different levels of structure tend to elicit different responses from clients (Amabile & Gitomer, 1984; Brubaker, 1996; Carr &

Vandiver, 2003; Hartwich & Brandecker, 1997; Henderson, Rosen & Mascaro, 2007; LaGiglia, 2007; Moon, 2010; Robbins & Sibley, 1976; J. Rubin, 1984; L. Rubin, 2000).

One of the most important decisions an art therapist can make, therefore, is what material to use with what client in order to address any initial fears and overcome initial discomfort. Wadeson (1980) wrote that media must be “selected purposefully” (p.18) and that to make that selection, “the art therapist [must be] familiar with what may be evoked by the different media, what advantages each offers and what limitations each has” (p.18). J. Rubin (2010) underscored the need for thoughtful, creative attention when selecting the right materials and directives for each individual. Naumburg (1966) noted that “for those patients who have never drawn or painted before undertaking art therapy (this includes the majority of patients), it is necessary to have simple art media which can be easily and quickly manipulated” (p.15). The therapist who carefully selects materials or approaches that meet the needs of the client in the initial sessions of art therapy is exhibiting the empathy, support, attunement and attentiveness that researchers have found will contribute to a positive working alliance (Hilsenroth & Cromer, 2007; Horvath et al., 2011; MacKinnon et al., 2006; Robbins, 1987).

Several papers have been written describing the observations of clinicians who have used collage with different populations. These clinicians have found it to be less threatening and more structured than other approaches, and effective in focusing the client and promoting verbalization and communication (Comfort, 1985; Landgarten, 1993; Linesch, 1988; Moriarty, 1973; Shields, 1991; Takata, 2002; Vick, 1999; Wadeson, 2000; Williams, 2000). Johnston (2007), in a master’s thesis, used a phenomenological design with two adult student participants to study the subjective experience of working

with collage. Johnston (2007) wrote that the use of pre-fabricated images seems to give individuals something onto which they can project thoughts and feelings, while at the same time allowing them to deny these projections by virtue of the fact that they did not originally create the images. The author recommended further study into the experience of working with different media, to explore how other approaches elicit substantive artwork from clients (Johnston, 2007). Her conclusions seem to suggest that one possibly appealing aspect of the collage approach is that by using images created by someone else, clients have a chance to distance themselves from the thoughts and feelings they have projected onto the artwork, while at the same time producing a strong piece that provides the art therapist with some insight into the client's dynamics (Johnston, 2007).

While collage has been clinically shown to be effective in addressing performance fear and initial anxieties, the precise qualities that might make it so have not been fully investigated, especially as it compares to another popular approach: drawing. Alderfer (2005) undertook a master's thesis study using a qualitative phenomenological design to examine how participants perceive what is being communicated by the art therapist. Alderfer (2005) recruited three male participants, with the purpose of exploring their responses to a single pre-selected, pre-glued stimulus image, and their responses to a free drawing. She found that responses to the pre-selected stimulus ranged from feelings of enjoyment at the challenge posed by having to work around a pre-chosen image, to feelings of being restricted and manipulated (Alderfer, 2005). Alderfer's (2005) study used a single pre-selected image, however, rather than providing the range of imagery usually available when collage is offered to individuals.

This present study sought to provide a deeper understanding of the psychological aspects of using a full range of collage imagery and words, as it compared to drawing. The objective was to give art therapists a greater awareness of how individuals might experience each approach, to allow them to make more purposeful decisions about when to use collage or drawing to elicit the largest amount of information as possible from new, anxious clients in the initial stages of therapy.

The research question was: how does the experience of working with collage compare to that of working with drawing materials as seen through the art making and interview? The specific research objective was to gain a richer understanding of the nature of collage as compared to drawing, in order to examine what qualities might or might not make it a less intimidating and more reassuring approach for individuals who are unused to making art. By asking participants who are not accustomed to using art for self-expression to compare their experiences using collage and drawing materials, by having them rate their anxiety level before and after each art task, and by examining the art produced, it was possible to identify characteristics specific to each approach that enhance our understanding about when and how collage and drawing might be effective in this context.

This study found that collage evoked significantly less anxiety than drawing because it was perceived as being easier and less intimidating. Findings suggested several reasons for this. The visual stimuli and the perceived ease of the process offered structure and control. The process also provided a reflective distance that allowed for safe exploration, insight and self-expression. Collage made it possible for participants to depict their strengths, thereby addressing fears of being shamed or misunderstood.

In comparison, drawing evoked significantly more anxiety and resulted in fewer art pieces that were considered revealing. The study identified concerns about artistic abilities, worries that meaning would be lost because of limited artistic skills, fear of the blank page, and unfamiliarity with using drawing materials. There are, however, inherent limitations to collage relating to the process itself and to the need to adapt to already-existing imagery, while participants generally felt that drawing would ultimately allow for more freedom of expression.

Limitations of the study included the small sample size, the high functioning nature of the student volunteers, and communication problems resulting from the fact that for most of the participants, English was not their first language. Delimitations included the data collection procedure, which began with the gathering of demographic information and which may have put the participants at ease, allaying some of their initial nervousness and influencing the self-reported anxiety ratings. Another delimitation was that participants were allowed to choose which art approach to use first, possibly affecting the way they addressed the second art task in terms of content and composition. The interviewer's inexperience as both therapist and researcher was also a delimiting factor. Excessive caution not to cross the line between therapist and researcher may have resulted in a reluctance to ask more probing questions about the art experience that could have provided more information about how each individual responded to the two approaches. The fact that she belongs to the dominant culture may also have contributed to the atmosphere of the sessions, given that the majority of participants were foreign.

CHAPTER 2: LITERATURE REVIEW

Overview

This chapter examines the literature relevant to a study of the qualities of two common art therapy approaches, drawing and collage, and how these characteristics may address or exacerbate the anxiety that clients new to therapy often experience. A brief overview of anxiety in the early stages of therapy is given, followed by a review of the literature on the importance of addressing this anxiety early in the therapy process. Anxiety as it manifests itself in the early stages of art therapy is then examined, primarily as it influences the client's resistance to engaging in creativity. This literature review then discusses theories and research into the qualities of art materials and the responses they may evoke from different populations. Finally, literature pertaining to the use of collage is reviewed, to provide a context for the need for a deeper examination of how it compares to drawing as an approach that might induce less anxiety in new clients.

Anxiety and/or Fear in Early Stages of Therapy

Starting therapy can be a frightening experience, even for individuals who are entering into it willingly (Brodsky, 2011; MacKinnon, Michels, & Buckley, 2006; Restifo, 2010; Tantam, 2002). One of the main issues therapists may find themselves dealing with is client ambivalence over the decision to talk about oneself to a stranger (Brodsky, 2011; MacKinnon et al., 2006; Restifo, 2010; Tantam, 2002). On the one hand there is the desire to talk to someone, as well as hope in the therapy process, while on the other hand, there may be feelings of shame surrounding the problems and behaviors that will have to be discussed, and a resulting fear of being judged and possibly rejected by the therapist. MacKinnon et al. (2006), writing about how to interview and assess a new

client, stated that “it is scary to open oneself up to a stranger” (p. 9), because there is the risk that a discussion of the problems that have brought people to therapy will generate feelings of embarrassment or shame.

Tantam (2002), in an overview of the practice of psychotherapy and counseling, described therapy as a potentially “disempowering” (p. 23) experience for new clients that can evoke fear and anxiety. In the therapist-client relationship, there is the perception on the client’s part that the therapist knows a lot, and the client little (Tantam, 2002). As a result, clients may see the therapist as someone who has the ability to judge and reject them as being unsuitable for therapy. There is fear about being asked to discuss intimate matters (Tantam, 2002).

There are also potential feelings of embarrassment about personal behaviors (Tantam, 2002). Tantam (2002) concluded that a new client’s “high hopes” (p. 23) over finding someone who will listen can be offset by such negative expectations.

Restifo (2010) noted that clients may also feel anxiety over their ability to pursue the treatment goals that have been established, and fear the possibility of failure. Restifo (2010) used a case study to examine the conflicting emotions some patients have when starting therapy. He used the term “performance anxiety” (p. 228) to define the fear of failing to respond to treatment, and the subsequent fear of once again being a disappointment to those who have certain expectations of them (Restifo, 2010).

Change itself can be frightening (Brodsky, 2011; Gabbard, 2005; MacKinnon et al., 2006; Restifo 2010). Gabbard (2005), in an overview of the basic principles of dynamic psychiatry, wrote that clients tend to want to maintain the status quo because attempts to change it, and to gain insight into what is causing the symptoms that have

brought them to therapy, can generate distressful feelings such as anger, guilt, shame, anxiety, love and hate. New clients may feel anxiety when asked to exchange a familiar coping method for one in which they have little confidence (Restifo, 2010). They may, in fact, actually be afraid of succeeding in therapy and of having to become independent (Restifo, 2010). This “fear of success” (Restifo, 2010, p. 227) over gaining personal independence can be seen as a form of resistance that the therapist must address. For clients to be able to address this fear and make the necessary change, they have to feel confident about the therapist’s understanding of what they are experiencing and about the proposed treatment (Restifo, 2010).

According to MacKinnon et al. (2006), individuals will be anxious about the practicalities of undergoing psychiatric treatment, including the necessary examination of defense mechanisms and ways of interacting with others that have so far seemed to be working well for them. Clients may also be unwilling to give up the secondary benefits of their illness, such as the sympathy and attention they receive (MacKinnon et al., 2006).

Brodsky (2011), in an examination of therapy with coerced and reluctant clients, also discussed resistance to treatment and the factors that may cause this resistance in some individuals. He cited the perception that therapy will be intrusive, unpleasant, and a sign of vulnerability and weakness, noting that while some clients want to improve their functioning and effectiveness, they may not want to do the work because they do not want to disrupt the status quo (Brodsky, 2011).

A final factor that may contribute to anxiety at the start of therapy is the obligation some clients will feel to get better (Restifo, 2010). The measureable outcomes

that mental health providers are looking for these days can exacerbate these feelings of obligation and the accompanying anxiety even further (Restifo, 2010).

It is important to address these feelings early in therapy in order to establish the alliance that research has shown to be a consistent indicator of positive therapy outcome.

Importance of Addressing Anxiety Early in Therapy

The importance of addressing the anxiety felt at the start of therapy is a consideration that dates back to the early days of psychotherapy, when Freud (1913) wrote that interventions are only possible in therapy once a “proper” (p. 139) rapport has been established between client and therapist, and that the therapist’s first concern is to engage the client in the therapy process and establish a client-therapist relationship. Freud (1913) believed that it was important to arouse the patient’s intellectual interest and understanding as part of this process to ensure the client’s collaboration. He added that once internal resistances, which would include those common at the start of any form of treatment, were addressed, the therapist would be able to begin his or her work (Freud, 1913).

Since Freud’s paper on this topic, there has been a wealth of research on the client-therapist alliance and its association with a positive outcome in therapy (Gomes-Schwartz, 1978; Horvath, 2001; Horvath, Del Re, Fluckiger, & Symonds, 2011; Horvath & Symonds, 1991; Martin, Garske, & Davis, 2000). Gomes-Schwartz (1978) recruited 35 unmarried male college students with elevated scores for depression, psychasthenia and social introversion on the Minnesota Multiphasic Personality Inventory (MMPI), for a study attempting to assess the influence of theoretical orientation and training on the interactions in therapy, and to determine the impact on therapy outcome of the quality of

the therapeutic relationship and degree of patient involvement. Independent raters analyzed audio taped segments of therapy sessions. The main findings were that theoretical orientation and experience influenced the process but not the outcome of therapy, and that the most consistent predictor of therapy outcome was the patient's active involvement in the process (Gomes-Schwartz, 1978).

Several meta-analyses examining research on the alliance in therapy have indicated reliable associations between the alliance and outcome (Horvath et al., 2011; Horvath & Symonds, 1991; Martin et al., 2000). Horvath and Symonds (1991) examined 24 studies that related the quality of the working alliance to therapy outcome. Findings included a moderate but reliable association between a good working alliance and positive therapy outcome, regardless of the form of therapy and length of treatment. One interesting result was that reports of the alliance made by clients and by observers were more predictive of outcome than the judgments of therapists, suggesting that a client's experience of the relationship with the therapist is what matters. The authors warned, however, that individuals who experience the therapy process favorably may be more likely to interpret the outcome of therapy more positively, something that they called the "halo" effect (Horvath & Symonds, 1991, p. 147), which might have inadvertently influenced the results.

Martin et al. (2000), exploring the relationship between therapeutic alliance and outcome and other variables, examined data from 79 published and unpublished studies, in an effort to reanalyze the Horvath and Symonds comparisons from their 1991 meta-analysis. The authors found that alliance is moderately but consistently related to outcome, as already suggested by the Horvath and Symonds (1991) meta-analysis.

Martin et al. (2000) did find that most of the studies used participants from an outpatient population and covered a range of diagnoses, which made it impossible to determine whether the type of diagnosis affected the alliance. In addition, most studies included both male and female patients, which also made it impossible to determine whether gender affects the alliance-outcome relationship.

A research synthesis of the relationship between alliance and therapy outcome in individual therapy by Horvath et al. (2011) looked at 201 research reports based on 190 independent data sources. Findings indicated a robust overall relationship between alliance and outcome in individual psychotherapy regardless of how the alliance is measured or the type of therapy involved, among other variables (Horvath et al., 2011).

Studies also suggest that an early alliance is important (Hilsenroth & Cromer, 2007; Horvath, 2001; Horvath et al., 2011; Martin et al., 2000; Restifo, 2010). Horvath (2001) combined two of the above-referenced meta-analyses (Horvath & Symonds, 1991; Martin et al., 2000) with the findings of 10 other studies performed from 1997-2000, to form a data set of 90 independent clinical investigations. After correlating the data, Horvath (2001) concluded that “it is likely that a little over half of the beneficial effects of psychotherapy accounted for in previous meta-analyses are linked to the quality of the alliance” (p. 366). He found that most investigators “report better outcome prediction based on early rather than later alliance” (Horvath, 2001, p. 367). Interestingly, he also found two studies indicating that a high initial alliance may be related to poor outcome and early termination, possibly because it reflected unrealistic expectations (Horvath, 2001). Similarly, Restifo (2010) noted that one way to address feelings of anxiety and ambivalence is by not offering clients a reassurance and encouragement in which they

will have no confidence. By not giving false hopes, the therapist can prevent the establishment of expectations that might feel unrealistic to the client (Restifo, 2010).

Hilsenroth and Cromer (2007), reviewing the literature on therapeutic interventions and therapist characteristics that seemed to contribute to a positive alliance, wrote that an early therapeutic alliance “is predictive of later positive psychotherapy outcomes” (p. 214) and that “this underscores the importance of psychotherapists’ active attempts to foster positive working relationships as early as possible in treatment” (Hilsenroth & Cromer, 2007, p. 214).

In their meta-analysis, Martin et al. (2000) wrote that:

patients tend to view the alliance as stable, whereas therapists and observers tend to indicate more change over time in their alliance ratings. The implications of this finding are clear: Because patients tend to view the alliance consistently throughout treatment, they are more likely to view the alliance as positive at termination if their initial assessment was positive. Thus, therapists must be effective at establishing positive alliances with their patients early in the therapy process (Martin et al., 2000, p. 447).

In their recommendations for therapy practices, Horvath et al. (2011) suggested that “the development of a ‘good enough’ alliance early in therapy is vital for therapy success” (p. 15) because it both prevents clients from dropping out and it creates an environment in which the work can take place (Horvath et al., 2011).

Research indicates that there are a variety of factors that can contribute to a positive therapeutic alliance (Hilsenroth & Cromer, 2007; Horvath et al., 2011; MacKinnon et al., 2006). Hilsenroth and Cromer (2007) established three broad

categories of interventions that helped foster a working alliance: treatment frame, the focus of the sessions, and the type of feedback provided by the therapist. Within the treatment frame, the authors found that it is important to conduct more in-depth interviews, to take a collaborative stance towards clients, and to use clear, concrete language to show clients that the therapist is interested in understanding their experience. The session focus category includes allowing clients to take the lead regarding what issues will be discussed, clarifying causes of distress, and allowing clients the chance to express their emotions in a way that feels safe to them. Feedback interventions allow the therapist to give clients specific information about the treatment and about their problems, and to develop specific treatment goals (Hilsenroth & Cromer, 2007). The authors also listed the therapist qualities that promote an alliance with the client: “empathy, support, exploration, activity, confident collaboration, appreciation, trust, warmth, attunement, potency, competence, respect, attentive, engaged listening, and appearing to understand the patient in a nonjudgmental and welcoming fashion” (Hilsenroth & Cromer, 2007, p. 216).

MacKinnon et al. (2006), in their chapter covering the general principles of the interview, described different ways that therapists interviewing new clients can put them at ease, introduce them to the therapeutic process, and generally try to engage them in the work. They suggested that the therapist, in the initial sessions, can offer reassurance by showing an interest in the client’s assets and strengths, and not just in the problems that have brought them to therapy (MacKinnon et al., 2006). The authors noted that since the therapist, in the early sessions, is trying to understand the new client, he or she must adopt a nonjudgmental, interested and concerned attitude that gives the client permission

to talk without fear of being shamed or rejected. The therapist should encourage the client, not make false promises about what treatment will accomplish, and pick the right moment for any intervention (MacKinnon et al., 2006).

Horvath et al. (2011) recommended that instead of looking to build an alliance *per se*, it will be forged in the process of persuading the client to collaborate in his or her own treatment. They also state that establishing the alliance early in therapy is essential (Horvath et al., 2011). A third recommendation was that the therapy tasks be tailored to meet the client's needs, expectations and capabilities. Fourth, since the therapist and client do not always perceive the alliance the same way, the therapist must monitor the client's alliance throughout treatment. The alliance will fluctuate, so the therapist should respond in a non-defensive way to negativity and hostility. Finally, Horvath et al. (2011) recommended training for therapists to hone the skills needed to for the development of the alliance.

The art therapist must contend with all the anxieties commonly felt by individuals starting any form of therapy, as well as the feelings evoked by being asked to express oneself creatively, using an unfamiliar form of communication: art.

Anxiety and/or Fear in Early Stages of Art Therapy

Art therapy is founded on the belief that the process of making art and the product itself provide therapists with information about the client's unconscious processes, give therapists and clients opportunities for communication and for understanding the nature of those communications, and is in itself inherently healing (Kramer, 2001; Naumburg, 1966; Wadeson, 1980). Art expression allows unconscious feelings and perceptions that often predated the ability to use words to emerge in a tangible form that can then be

explored from a safe distance. The imagery is less likely to be censored than words would be, since for most people art is a less familiar form of expression. The resulting object is permanent, and therefore not subject to the distortions that memory can cause. There are no rules of logic in art, so that it better reflects the way people respond to the world. Wadeson (1980) called this the “spatial matrix” (p. 11) that allows individuals to depict many relationships and feelings in one single product. Art making energizes (Wadeson, 1980), it gives people a chance to redirect unacceptable impulses into a more acceptable form (Kramer, 2001), and at the end of it, the individual has a product that both enhances communication and can stand alone as an achievement in its own right (Kramer, 2001; Naumburg, 1966; Wadeson, 1980).

Being asked to engage in creative tasks can nevertheless exacerbate the anxiety experienced at the start of any kind of therapy (Comfort, 1985; Stone, 1991; Wadeson, 1987; Waller, 1993).

To begin with, using art materials is something that many individuals stopped doing when still young (Lowenfeld & Brittain, 1987; J. Rubin, 2010; Sarason, 1990; Waller, 1993). Lowenfeld and Brittain, (1987) writing about the importance of art education in promoting flexibility and creativity in individuals, noted that artistic development can end around the age of 12. This may be in part because young people do not develop the artistic abilities needed to progress with their work (Lowenfeld & Brittain, 1987; J. Rubin, 2010). A growing self-consciousness and self-criticism, with an increasing awareness that their art product is not adequately reflecting the reality they are trying to depict, can discourage children who are entering adolescence from continuing to make art (Lowenfeld & Brittain, 1987; J. Rubin, 2010). Art education can help further

artistic skills, but in many schools, art is not given much importance. Because of all these factors, adults who have not deliberately pursued art activities will tend to make drawings typical of twelve year-olds (Lowenfeld & Brittain, 1987). Waller (1993) noted that most people tend not to pursue artistic activities as adults, with the result that making art in the therapy setting may be the first time that they have engaged in this endeavor in years.

The anxiety of being asked to use unfamiliar art materials may be heightened by cultural expectations as to what constitutes art, with accompanying fears of being judged for what is produced (Sarason, 1990; Wadeson, 1987; Waller, 1993). According to Sarason (1990), who examined the ways individuals can find satisfaction and growth through creativity, people in Western society stop engaging in the arts due to cultural expectations about what constitutes a piece of art, as well as their own dissatisfaction over their ability to represent reality appropriately and in a way that they feel meets those cultural expectations. He called this a “form of learned helplessness” (p. 5) that inhibits creative expression (Sarason, 1990). Furthermore, in our society, artistic activities in childhood are not given the same importance as reading, writing, math and objective thinking. This overemphasis on the objective and scientific ends up influencing the way individuals view such activities as they grow older (Sarason, 1990). He noted, however, that the desire to express oneself artistically is never fully extinguished. Nevertheless, asking new clients to make art can still induce anxiety and add to their distress (Sarason, 1990).

Wadeson (1987) wrote that performance fear is one of the “greatest roadblocks to creative self-expression” (p. 56). Being asked to be spontaneously creative can be

accompanied by fears of producing something that is subpar and will be judged negatively, especially given cultural expectations of what constitutes art (Wadeson, 1987).

Waller (1993), citing Maclagan, wrote that the unfamiliar act of creating something has the added potential of producing something that can be “potentially humiliating” and “of dubious value” (p. 37). Comfort (1985), writing up two case studies in which he used published pictures as stimuli for encouraging self-expression, described the deeply-rooted anxiety that can be generated when people who are engaged in talk therapy are asked to make art. He wrote that clients, when asked to draw or paint, show reluctance and often fall back on the argument that they have no talent or have never been artistic (Comfort, 1985). Comfort (1985) felt that this anxiety was based on the belief that to be an artist, one must have a gift, and that true art requires the depiction of something in a realistic fashion.

Another source of anxiety is the fear of revealing too much of oneself too soon that may stem from the belief that the art therapist is all-knowing and can read the art without the client’s input (Wadeson, 1987; Waller, 1993). People who are used to talking may feel threatened when asked to use art materials to express themselves because they are afraid that they will be unable to control what they produce (Wadeson, 1987; Waller, 1993). Wadeson (1987) called this the “fear of self-revelation” (p. 61).

This fear of being unable to control one’s product can also be seen in other creative art therapy modalities. Austin and Dvorkin (1993), in a paper about resistance in music therapy and ways to work through it, wrote that clients are often hesitant to make music because “experience has shown them, or they suspect, music can bypass defensive

barriers and access feelings more effectively than words” (p. 425). Clients may experience regression, and may fear the aspects of themselves that might be revealed through this regression (Austin & Dvorkin, 1993).

Wittenberg (1974), when describing an art therapy program at a residential treatment facility for adolescent drug abusers, mentioned her clients’ fears and insecurity when asked to make art, sparked by the fact that they had failed at almost everything they had attempted up to that point, and were afraid of failing again. This made them suspicious of trying new things. Wittenberg (1974) underscored the importance of addressing this performance fear early in therapy by structuring sessions to help relieve the anxiety and carefully choosing tasks and materials.

Some adults will resist drawing, deriding it as being a childish activity (Wadeson, 1987), because they feel that they will not be able to excel at it (Whalen, 2004), or because they are embarrassed about expressing themselves through art (Gladding & Newsome, 2003). Stone (1991) undertook a small-scale descriptive study to obtain a clearer picture of how older adults view their own artistic work. Stone (1991) used observation, interviews and a questionnaire to collect data from 15 senior citizens in a painting class at a senior center. The objective of the study was to gather information about the students’ attitudes towards their own art, their interests and attitudes to art in general, and general demographic information (Stone, 1991). Findings suggested that older adults may have unrealistic expectations about their art creations, since most participants commented negatively about their own work. The higher the education level, the more negative the self-perception of the participant. However, more experience with art making resulted in a more positive attitude. Another finding was that the more

importance participants attributed to art, the more negative their attitude to their own work. Finally, the belief that there is such a thing as artistic talent also negatively influenced a person's opinion of their own work. A factor that did not seem to influence self-perception was the amount of art taken in elementary, middle and high schools (Stone, 1991).

Art materials can be powerful instruments in addressing the anxiety caused by being asked to make a piece of art in the materials. By knowing their materials and by being aware of how individuals might respond to them, art therapists can use different materials to help establish the relationship that research has shown to be consistently associated with positive therapy outcome.

Materials and Structure in Art Therapy

Addressing the anxiety that accompanies the request to make art is important not only to help establish a collaborative alliance that will engage the client in therapy, but also to encourage the creative communication that is at the heart of art therapy. While art therapists can avail themselves of the same resources as verbal therapists to address new clients' fears, they also have a powerful tool in the very range of art materials available and in the amount of structure they can provide within the sessions. The range of art media available is vast. Each material has its own particular set of qualities that are going to evoke a distinct response from the person using it (Kagin & Lusebrink, 1978; Moon, 2010; Robbins & Sibley, 1976; Wadeson, 1987). Robbins and Sibley (1976) described the characteristics of art materials in terms of their touch and texture, color, movement and rhythm, space or inherent boundaries, the body movements required to use them, their concrete or abstract qualities, the level of risk-taking required when using

them, and the opportunities they provide for encouraging feelings of mastery and competence. Some media:

address themselves to the ego-organizing capacity of the patient. Other materials tap very deep libidinal levels, while still others have an exploratory quality. Some challenge a sense of mastery; others provide just pure fun. Hopefully, an art therapist can work towards becoming an expert in the broad use of materials which supply the needed communicative links in everyday therapeutic situations (Robbins & Sibley, 1976, p. 201).

Familiarity, level of comfort with a medium, level of development or regression, the degree of control that the individual requires, and therapy goals will determine choice of, and response to, a material (Kagin & Lusebrink, 1978; Lusebrink, 1990; Moon, 2010; Schlossberg, 1983; Wadeson, 1987). Kagin & Lusebrink (1978) discussed the Medium Dimension Variables (MDV) that characterize art media. A medium can be fluid versus resistive, depending on its physical qualities; simple versus complex, depending on the number of steps required for its use; and structured versus unstructured, depending on whether the art therapist gives specific instructions for the art experience (Kagin & Lusebrink, 1978). Some materials require the use of a mediator or tool for their use, such as a paintbrush or chisel, which distances artists from their product to some extent, allowing them to separate themselves from the tactile impressions, thereby increasing the cognitive distance between the experience and the artist's thoughts about it while decreasing the affective involvement with the art process and product (Kagin & Lusebrink, 1978). Kagin and Lusebrink (1978) called this the reflective distance, saying that different media can either increase or decrease this distance. Unmediated media, on

the other hand, allow individuals to interact more directly with the material, providing a more sensory, less cognitive, experience (Kagin & Lusebrink, 1978). Kagin and Lusebrink (1978) also theorized a model called the Expressive Therapies Continuum (ETC) that brings together the MDV and the way individuals process information and express themselves. According to the ETC, a material's qualities, when used by an individual, will evoke responses that can be kinesthetic or sensory, perceptual or affective, cognitive or symbolic, or a combination of all of these levels. An art therapist who is attuned to the patient and to the materials can move clients along this continuum in a way that meets their psychodynamic needs (Kagin & Lusebrink, 1978).

An example of how different people may respond to the same material is given by Schlossberg (1983), who described two cases where clay was used with contrasting results. For a young woman with a diagnosis of borderline personality disorder who had been hospitalized following a suicide attempt, clay became a successful vehicle for the externalization and containment of her feelings, unlike drawing and painting, which she had been unable to use effectively. Clay had the opposite effect on a young man with a diagnosis of paranoid schizophrenia, who responded with disgust and distress to the medium, to the point where he refused to join the art therapy group for some time (Schlossberg, 1983).

According to Wadeson (1987), another characteristic inherent in art materials is what she called commitment, or whether a mark, once made, can be undone or not. She gave the example of markers, which are indelible and cannot therefore be undone, and clay, which is eternally changeable as long as it is kept damp and pliable.

Cultural, social and political associations are also going to influence an individual's response to a medium (Moon, 2010). In her comprehensive examination of materials and media in art therapy, Moon (2010) noted that art materials are the "raw components of the exchange between client and therapist" (p. 85), and as such, art therapists have to be aware of their potential connotations to a particular individual or group. Moon (2010) cited the example of the constraint generated among a group of art therapy students in Israel, where she was working as a visiting professor, when one student, the lone German in the group, incorporated into her artwork the hair of another student, which had been thrown away. Moon (2010) reported being caught off guard because of her lack of awareness of the cultural significance of the appropriation of another person's hair to a group with a history overwhelmed by the concentration camp experience, for which hair can symbolize the stripping of identity, dignity and control.

Structure, in the form of a directive or specific task, can also be used to help address client needs (Brubaker, 1996; McNeilly, 1983; J. Rubin, 2010; Wadeson, 1987). Although Wadeson (1987) wrote that she preferred a nondirective approach, she believed that in some situations more structure is called for, for example, in a short-term psychiatric unit where goals have to be specific, or when gathering information for treatment planning. Freer, more non-directive approaches are more likely to succeed with high-functioning clients, whereas more disorganized or more impaired individuals will benefit more from a more structured approach (J. Rubin, 2010). Art therapists therefore have to consider the goal of the art activity and the needs of the client when deciding how specific the directive should be (J. Rubin, 2010; Wadeson, 1987).

In a 1983 paper on the use of art therapy in general psychiatry in which he examines directive and non-directive approaches, McNeilly wrote that by presenting a theme to the group, there is the risk that powerful feelings will be exposed too quickly, before an individual can tolerate them. McNeilly (1983) questioned the long-term impact of such catharsis. He also felt that the leader, by giving a theme to the group, becomes the focus, inhibiting the interaction of group members since their goal then becomes their desire to please the therapist (McNeilly, 1983). The natural dynamics of the group are altered, members do not relate to each other but instead relate to the leader, and true emotions are inhibited (McNeilly, 1983). In fact, McNeilly (1983) believed that therapists who offered structure in the form of a theme were playing it safe, effectively avoiding the anger that freedom of choice can evoke and the resulting rich source of material with which the therapist would have to work.

Studies on the influence of structure on different populations show varying results (Amabile & Gitomer, 1984; Brubaker, 1996; Carr & Vandiver, 2003). Brubaker (1996), in a master's thesis study, undertook a pilot study using observation and the collection of verbal responses to examine the preferences of a geriatric population for structure in an art therapy task. Forty-five participants aged 65-100 were asked to choose between four pieces of paper: one blank, one with a triangle, one with a circle, and one with a square drawn on it. Ninety-one percent of participants chose a paper with something already drawn on it, with the square being the most popular choice. While participants were not asked the reasons for their choices, Brubaker (1996) suggested that more structure allowed for more success in view of their failing physical and cognitive abilities, and also mirrored the structure of their lives in the nursing home. Familiarity with the geometric

shapes could also have been encouraging, and may have acted as a stimulus (Brubaker, 1996).

Amabile and Gitomer (1984) used a mixed-methodology design to explore the effects of choice of materials on children's artistic creativity. The study recruited 28 children ages 2-6, whose teachers completed a brief questionnaire on the creativity of each child at the start of the study (Amabile & Gitomer, 1984). The authors found that children who were allowed to select their collage materials produced art that was judged to be more creative than those who were given a specific set of collage materials. The first group of children also played more with the collage materials in a post-experimental free play session than the no-choice group (Amabile & Gitomer, 1984).

Carr and Vandiver (2003) studied the impact that level of instruction and choice of art materials had on 10 children aged 4-13 who were living in a homeless shelter and consequently under a great deal of stress. The researchers found that the art project that required some instruction and a limited range of materials was more likely to promote positive behavioral responses and artistic impressions than the projects that required many steps and offered a wide range of materials or no instructions and a wide choice of materials, suggesting that for children in a stressful situation, a balance has to be found between too much and too little structure (Carr & Vandiver, 2003).

Specific structured tasks and approaches have been found to be beneficial with different populations (Hartwich & Brandecker, 1997; Henderson, Rosen & Mascaro, 2007; L. Rubin, 2000). Hartwich and Brandecker (1997) discussed the use of computer-based art therapy with acute inpatients in a hospital in Germany. Through case studies, they described how the strict rules of making art on a computer, the distance it provides

between patient and image, and the possibility it offers patients for maintaining and protecting their defense mechanisms, can be useful with this population. They noted, however, that the approach does not work with very disorganized individuals who cannot understand the rules for using the computer (Hartwich & Brandecker, 1997).

L. Rubin (2000) described the use of paint-by-number (PBN) art with his clients. He noted that PBN is something familiar that makes art accessible to everyone. Because of that familiarity, it can evoke memories and feelings that can then become material for the therapy sessions (L. Rubin, 2000). The images can be used as projective tools. The act of painting can energize and provide the artist with a finished product that they can value, which can boost their self-esteem. PBN is less threatening than a blank page, and it can also be the starting point for more independent art making, since it teaches the artist about color, composition and brush technique (L. Rubin, 2000).

Henderson et al. (2007) undertook an empirical study of the healing nature of mandalas for individuals with post traumatic stress disorder (PTSD). After being prescreened for PTSD, the 36 participants recruited were randomly assigned to two groups. The authors found that participants in a group that made mandalas showed reduced PTSD symptom severity one month following the art making session, as reported on the Posttraumatic Diagnostic Scale (PDS) self-report measure for PTSD symptoms, in comparison to the control group. The experimental group was told to draw a large circle and fill it with representations of feelings or emotions relating to their trauma, while the control group was given specific drawing assignments. Limitations included the small sample size and the fact that 28 out of the 36 participants were female (Henderson et al., 2007).

There is a growing body of research on cognitive and/or emotional responses to different levels of structure and different materials, many at the master's level (Alderfer, 2005; Boylan, 2001; Crisp, 1997; Foster, 1997; Foy, 1997; Johnston, 2007; LaGiglia, 2007; Radl, 2005; Schlossberg, 1983; Stager, 2004). Foster (1997) discussed her observations in a psychiatric hospital of the fear patients with schizophrenia exhibit towards clay and plasticine. She hypothesized that patients avoided this medium because of their use of projective identification as a defense mechanism, projecting the bad aspects of their selves into the clay, and the symbolic equating of clay to bodily products, feces in particular. This observation, combined with her belief as a sculptor in the therapeutic effectiveness of three-dimensional media, led her to make very careful, deliberate choices in order to introduce clay as a way to promote self-development and help develop a more traditional way of relating to the therapist (Foster, 1997).

As already described, Schlossberg (1983) used a case study design to describe the radically different responses two individuals had to the same material: clay. For one person, the medium helped her externalize, organize and contain her feelings in a way that drawing materials and paint could not. For another individual, clay was too regressive, causing so much anxiety that he dropped out of the art therapy group for a long period (Schlossberg, 1983)

Foy (1997), in a master's study, used timed observations, and analyzed the transcriptions, of a group of four patients with chronic schizophrenia to explore how their interpersonal behaviors differed when asked to use structured art media, in this case colored pencils on Bristol board, and wet potter's clay. Her findings showed that clay engaged the patients more, helped increase focus, stimulated a discussion of more

personal experiences, and promoted more independent interaction between members (Foy, 1997).

Crisp (1997), in another master's thesis study, used a multiple case study design to explore the possible benefits of three-dimensional art media with hospitalized geriatric medical patients. She undertook three individual case studies of medically ill geriatric patients, and found that results were individual to each patient, since there was a range of moderating variables (Crisp, 1997). However, all three participants chose white clay and craft sticks over colored clay and assorted wood shapes, commenting that the white clay was soft, comfortable and malleable. The craft sticks were used more as a mediator and to provide structure. Findings indicated that the patients seemed comfortable using three-dimensional media, which appeared to stimulate their senses and to suit a range of functioning levels (Crisp, 1997).

A master's study by Boylan (2001) looked at the use of two- and three-dimensional media for increasing focus in children diagnosed with Attention Deficit Hyperactivity Disorder (ADHD). Boylan (2001) used a mixed methodology design consisting of time measurements and an analysis of art content and observed behavior. Her subjects consisted of five children ages 8-14, diagnosed with ADHD. The researcher found that three-dimensional media, in the form of clay, contributed to an increase in the child's ability to concentrate, in comparison to two-dimensional media, in this case, markers, and suggested that the stimulating and kinesthetic qualities of clay may have helped sustain attention (Boylan, 2001).

Stager (2004) used a phenomenological approach in her study to explore the experiences of visually impaired individuals when using two- and three-dimensional

media. The two participants were offered a range of two- and three-dimensional media with which to work. Results suggested that materials that were more tactile seemed to allow for more playfulness and exploration, while more structured media evoked greater anxiety (Stager, 2004).

Alderfer's (2005) thesis sought to explore how individuals perceive what is being communicated by the art therapist when presented with varying levels of structure, and how this perception affects their responses to art therapy and the therapist. Alderfer (2005) used a phenomenological design in which three male participants, meeting as a group, were asked to make a free drawing on a blank piece of paper, and then to draw on paper on which a pre-selected image had been glued. Responses to the pre-selected stimulus image ranged from feelings of enjoyment at the challenge of having to build on an already-existing image, feelings of being restricted by the image, and feelings of being manipulated by the researcher (Alderfer, 2005). Alderfer (2005) hypothesized that the range of responses reflected how open her participants might be to others' points of views and ideas. She noted, however, that a different image might have elicited a different response, underscoring the need for art therapists to be aware of polarities that might exist in the way individuals respond to the same task and materials (Alderfer, 2005).

In her study, Radl (2005) examined the impact of structure versus non-structure on anxiety level, using a mixed quantitative and qualitative design. Thirty participants were divided into two groups. Group A was given a specific art directive, while Group B was asked to draw how they were feeling at that moment. Findings showed that although all participants showed a decrease in anxiety level once their task had been completed, the decrease was statistically significant in Group B (Radl, 2005). One factor that

emerged was that the metaphor selected for the structured task evoked anxiety in some participants. These findings suggested that college students respond more favorably to less structure, and that the metaphor chosen when structuring a task is important and must be relevant to the here and now of the group (Radl, 2005). Limitations of this study included the small sample size and the high functioning nature of the student participants (Radl, 2005).

LaGiglia (2007) undertook a phenomenological study of five healthy adults to examine how individuals respond to different paper sizes. Findings showed that the participants, when instructed to make a free drawing on three different-sized papers, experienced the smallest size (4x6 inches) as being comforting, easier and allowing for more detailed work than the two larger sizes (9x12 and 18x24). The middle-sized paper, despite being familiar and commonly used, was not perceived as being the easiest to work on, and the largest sheet was considered overwhelming (LaGiglia, 2007).

Johnston (2007), in a master's thesis, also used a phenomenological design to study the subjective experience of collage-making by two university students. She found that the collage-making process seemed to involve four phases: a free associative or unconscious thought process during the selection of the images; the emergence of more coherent personal memories and reflections as images are organized and assembled; the synthesis and evaluation of the thoughts that emerged; and finally, logical and defensive judgments about the context and unexpected results in the finished product (Johnston, 2007). Johnston's (2007) participants expressed some anxiety over the time constraints and uncertainty about the art making process, but also noted that the selection of images helped focus their work. In her conclusions, she wrote that the use of pre-fabricated

images seems to give individuals something onto which they can project thoughts and feelings, while at the same time allowing them to deny these projections by virtue of the fact that they did not originally create the images. She recommended further research into the experience of working with different media, to explore how other approaches elicit substantive art work from clients (Johnston, 2007).

One approach that is popular among art therapists and widely used to engage clients in art making and address resistance is collage.

Collage

In a 2003 survey of art therapists working with cancer patients in New York City, Bromberg explored what art materials and techniques were thought to be most effective with this population. The 37 art therapists who responded indicated that collage, along with modeling clay, paint, pencils and watercolor, was one of the most popular approaches for use with patients. Collage was used by 79 percent of respondents, and was given the highest effective rating by 42 percent. Limitations of the study were its being restricted only to those art therapists currently working with cancer patients, and the absence of information on the gender of patients, which made it impossible to gauge whether gender has an impact on receptiveness to specific materials (Bromberg, 2003).

Several papers have been written describing the observations of clinicians who have used collage with different populations. Many consider it to be less threatening and more structured than other approaches, especially to individuals who are not used to expressing themselves creatively (Comfort, 1985; Landgarten, 1981; Linesch, 1988; Wadeson, 1987; Wadeson, 2000). Collage can be used to “facilitate creativity and to lessen superego demands on performance” (Landgarten, , 1981, p. 4), since it requires

little technical skill. Wadeson (2000) called collage “one of the least threatening activities” (p. 406) for new art therapy clients, as well as one that offers containment. She noted that “assembling images may be easier while still personal and expressive. Combined cut-outs from magazines can tell revealing stories” (Wadeson, 1987, p. 34). For a group of female chronic patients at a Hispanic community mental health center, collage was “an especially comfortable activity in the group’s early stages because it involved a series of steps compatible with the Hispanic way of socializing” (Wadeson, 1987, p. 220). Themes would emerge as the group leafed through the magazines, stimulating discussions among group members (Wadeson, 1987).

Wadeson (2000) described the work of therapist Kathleen Terra, who used collage with psychiatric patients in a prison, many of whom felt intimidated by being asked to create an image and who found it less daunting to select ready-made pictures that appealed to them personally (Wadeson, 2000). Because self-identity can be readily expressed through images, therapist Monique Prohaska, when working with emotionally and behaviorally disabled adolescents in special education, used collage to address their resistance to making art or revealing anything about themselves (Wadeson, 2000).

Linesch (1988) noted that in her work with adolescents, collage was an effective approach because it is a “controlled and controllable medium” (p.77) that is non-threatening, responds to their defensive needs, and gives them the tools for the self-expression and creativity that are a vital part of healthy adolescent development (Linesch, 1988).

Comfort (1985) used published pictures as psychotherapeutic tools to overcome resistance and anxiety in new clients who were afraid of producing something subpar or

of being too revealing. He used magazine, newspaper, greeting card, postcard and art images to introduce clients to the use of imagery as a form of therapeutic communication. He found that this approach actively involved the client in searching for an outer illustration of their inner experiences, and provided a metaphor that permitted the safe distance needed to reduce anxiety when discussing their feelings (Comfort, 1985).

Collage has also been used to help focus the client and promote verbalization and communication (Comfort; 1985; Landgarten, 1993; Moriarty, 1973; Stallings, 2010; Takata, 2002; Vick, 1999). Moriarty (1973) used this approach with female chronic patients suffering from schizophrenia, saying that collage provides freedom within structure and lessens dependency on the therapist, because:

it is more structured and less threatening. There is no concern about being unable to paint or draw; patients only have to cut out and arrange pictures. Collage-making also gives the patient less opportunity to wander off into autistic productions. it bypasses concerns about being able to draw or paint and also helps focus the patients' attention on the external world, through the pictures of everyday life (Moriarty, 1973, p. 153).

Vick (1999) also used collage to provide a structure that allows for flexibility. He described the use of prestructured art elements with adolescents in a partial hospital program. He used pre-selected magazine images and words, photocopied images, cut and torn paper, traced shapes and partial drawings as a way of streamlining the sessions and providing stimuli for creative engagement in groups where turnover rates were high and stays could be short. He found that this approach was effective in engaging the youngsters in making art (Vick, 1999). The adolescents sometimes used magazine words

as a way of defending themselves against anxiety, however, and in some cases it was difficult to redirect them back into more creative work (Vick, 1999).

There have been several studies done in Japan on the use of collage in different settings. A case study by Takata (2002) described the use of collage with two students with high absenteeism rates. The hypothesis was that collage can help individuals develop a sense of identity by giving them an opportunity to gain insight and self-understanding. The primary goal was to promote communication between the students and the nurse facilitating the sessions. Since the two adolescents engaged in collage-making in the school infirmary, together with other peers, the approach also encouraged socialization. Results indicated that collage therapy, as the process is called in Japan, successfully promoted the expression and discussion of the students' issues, helped them develop a stronger sense of self, and increased self-reliance and socialization skills (Takata, 2002). A significant limitation to this study was that sessions were not time-limited, with some running over two hours. It did not, therefore, address the issue of whether collage can help streamline the art making portion of a session when time is a factor.

Landgarten (1993) developed the Magazine Photo Collage (MPC) to address the need for an assessment tool that was not culture bound. In the MPC, the art therapist carefully selects images that relate to the experiences of the target population, and then gathers the clients' associations to the pictures they have selected. Images are grouped into two categories: pictures of people, individually, in dyads or in groups; and miscellaneous items, which include objects and scenes that may relate to specific issues that the clients might be dealing with, such as alcohol dependency, suicidal ideation or

physical abuse. Landgarten (1993) suggested having another person check the collection for any perceived bias on the therapist's part. The MPC provides the therapist with such information as modes of relating to others and self, perceptions of good and bad, coping mechanisms and problem-solving capabilities (Landgarten, 1993).

Stallings (2010) explored the use of collage and its potential to facilitate reminiscence in three older adult patients with dementia. Her hypothesis was that a modified version of Landgarten's (1993) Magazine Photo Collage (MPC) would give this population the opportunity to convey information that they might be incapable of verbalizing fully. The author found that all three participants seemed able to communicate nonverbally through their collages, the collages stimulated memories, and possible feelings of not being in control may have been offset by having control over the collage materials and process (Stallings, 2010). One limitation of this study was the small sample.

Collage has been found to be effective in clinical supervision (Shepard & Guenette, 2010; Williams, 2000). Besides being perceived as non-threatening, it is constructive, rather than destructive, and does not require finely honed artistic skills (Williams, 2000). Williams (2000) was not using collage as a tool for therapy, but as a way to help the supervisee describe a situation and begin to talk about his or her clinical experience. He gave three examples of supervision work using collage, and concluded that it opens up "avenues for discussion that might have been missed but for their metaphorical presentation as images on a collage" (Williams, 2000, p. 277). He also noted that even if supervisees choose to censor what they say about an internship

experience, the image is not censored and the experience is therefore brought to awareness (Williams, 2000).

Shepard and Guenette (2010) examined collage as a vehicle for members in group supervision to express their development as practitioners. Shepard and Guenette (2010) felt that the playful, liberating aspects of collage can help address anxiety and engage the interest of individuals, who have to look through a range of colorful images to find the ones that meet their needs. In this study, the authors met with three counselor students at the end of their practicum year, and at the beginning and end of their second-year practicum, for a total of three sessions. They found that the collage approach generated rich material for reflection and a way to examine the supervisory experience from different angles (Shepard & Guenette, 2010).

Although clinicians have been using collage extensively for decades, little research has been done to examine systematically the qualities of collage that seem to make it less threatening than, or how it compares to, other art approaches. Johnston (2007), in her examination of the subjective experience of collage-making by two university students, found that the process seemed to involve four phases, as has already been described. Although her participants expressed anxiety over time constraints and the art-making process, Johnston (2007) felt that a great deal of information was gathered in a short amount of time. In her recommendations for clinical applications of her findings, Johnston (2007) suggested that when using the magazine photo collage approach in a clinical setting, the art therapist must take into account “time allowance, paper size, amount of instruction or directives given as well as the response of the client to being in the presence of the therapist, or the stranger phenomenon” (p. 110), factors

that will depend on the degree of structure and containment needed by the client or group of clients (Johnston, 2007). Limitations of this study included the small sample size, the fact that both participants were healthy adult females, and possible bias in the selection of the collage materials (Johnston, 2007).

Summary

Anxiety in the early stages of therapy is something that must be addressed in order to engage the individual in treatment and to establish the collaborative alliance that has been shown to be a predictor of positive therapy outcome. In art therapy, anxiety about entering into therapy can be exacerbated by being asked to use art to express oneself. The art materials themselves can be powerful tools for helping to address these feelings and create a safe environment where the collaborative alliance can develop.

A review of the literature underscores the variety of responses one material can elicit, depending on the population. A review also indicates that the psychological aspects of different art media have not been extensively researched. Clinical observations suggest that collage is a structured, non-threatening approach that can engage clients in art making and encourage creativity in those who may feel intimidated by other art materials, such as drawing, painting and three-dimensional media. Although there have been studies comparing different materials and different levels of structure, there have been none that specifically investigate the compared characteristics of drawing and collage, two popular approaches in art therapy. This study attempted to fill that gap by exploring how participants responded to these two approaches in an initial art making session, and by examining what distinct qualities each approach possesses. The purpose was to deepen art therapists' understanding of how new clients might respond to the two

approaches, giving therapists the chance to make more informed decisions about when to use one over the other as a way of addressing anxiety in initial art therapy sessions.

CHAPTER 3: METHODOLOGY

Design

This is a descriptive survey that used open-ended verbal interviews, artwork and an anxiety scale for data collection. The overall purpose of this study was to describe a non-artist adult's experience of making a piece of art about him or herself using two approaches, collage and drawing materials, with the idea of understanding how the characteristics of each approach might affect the anxiety experienced in initial art therapy sessions.

Eight adult students who are not used to making art were recruited. In individual sessions, each was asked to make two pieces of art about him or herself, one using a selection of magazine images, phrases and words, and one using drawing materials. The researcher also asked them to describe their experiences, using an open-ended interview guide. Before and after each art task, participants were asked to rate their level of anxiety on a scale that was specifically designed for this study. The anxiety scale was intended to help determine whether each approach decreased or increased the participant's anxiety at being asked to make art in the presence of a stranger. The interview allowed the researcher to identify what qualities each approach had that may have contributed to decreasing or increasing any anxiety the participants may have felt. By using an interview guide, the researcher had a framework that listed areas of interest that she knew she wanted to cover (Weiss, 1994). Open-ended questions allowed the interviewer to follow up on responses that seemed relevant to the study, with the goal of gaining a fuller description of the participant's experience (Weiss, 1994).

For the purposes of this study, collage materials included a range of magazine images, phrases and words. Each participant was offered an identical selection of collage materials. Drawing materials consisted of a #2 pencil, colored pencils, thin markers, oil pastels and chalk pastels. The paper used was 11 x 14 inches white paper.

The goal was to gather information about the participants' experiences when asked to use two distinct, unfamiliar approaches to address the same task. Interviewing is an effective way to gather information about an individual's inner experience, since a descriptive survey design allows for fuller responses and for follow-up questions that help gain a better overall understanding of the art making experience (Weiss, 1994). Data for this study was gathered from an anxiety scale on which participants rated their anxiety level before and after each task, from a face-to-face interview using an open-ended interview guide, and from the artwork that was produced. Data collected from this survey has contributed to our understanding of the differences between working with collage and with drawing materials, in an examination of what qualities collage might have that seem to make it more effective than drawing in helping address the anxiety some individuals feel in the early stages of art therapy (Comfort, 1985; Landgarten, 1993; Linesch, 1988; Moriarty, 1973; Takata, 2002; Vick, 1999; Wadeson, 2000; Williams, 2000).

Location of the Study

The study was conducted at Drexel University's Center City Campus Bellet Building, in a private room, for purposes of confidentiality.

Data Collection Time Frame

Recruitment of participants began once approval was granted by the Drexel University Institutional Review Board (IRB), which occurred on August 1, 2011. The completion date for the study was set for July 31, 2012.

Enrollment Information

The subject group included eight (8) undergraduate or graduate students currently enrolled at Drexel University, aged 18-65. The rationale for selecting a small sample size was that this was an exploratory inquiry into the qualities of two different approaches that are widely used by the art therapy community, with the goal of obtaining an in-depth snapshot of the participants' experiences. A smaller sample allowed the researcher to spend more time with each participant and to explore complex variables that emerged during the interview (Patton, 2002). Eight participants nevertheless provided a wide enough range of responses that made it possible to identify common and variant themes.

Participant Type

Participants were Drexel University students who did not have a fine arts background and were not enrolled in any art or art-related programs. The rationale was that this study sought to compare the normative non-artist adult's experience when making two pieces of art, one using collage and one with drawing materials, in order to enhance our understanding of the differences between using the two approaches with individuals for whom art is not a familiar experience.

Subject Source

Participants were current students in Drexel University's Main and Center City campuses.

Recruitment

Participants were recruited through flyers that were posted on both campuses of the University (see flyer, Appendix A, p. 161). The flyer listed the title of the study, research objectives, eligibility requirements, remuneration being offered, location of the study, length of time required, and how to contact the researcher by telephone.

Remuneration consisted of a \$30 gift certificate for Barnes and Noble. Individuals responding to the flyer were asked to call the researcher to set up a meeting. The researcher either answered or returned the call, read the information on the flyer, and asked if the potential participant fulfilled all the requirements. If they said yes, they were recruited on a first-come basis. Once the participant was recruited, an appointment was set up for completing the informed consent (see Informed Consent form, Appendix B, p. 162) and for data collection, in one single session. The recruitment process was considered complete once eight people had been recruited. Flyers were first posted on September 23, 2011, and recruitment had been concluded, and the data collected, by October 28, 2011.

Participant Inclusion Criteria

- Participants were Drexel University undergraduate or graduate students aged 18-65.
- Participants could be of any gender and ethnicity.

Participant Exclusion Criteria

- A fine arts background or art training in drawing, painting and/or sculpture.
- Current enrolment in an art or art-related program, full or part-time, with Drexel University or any other institution.

- Students under the age of 18 or over the age of 65.

The rationale for excluding students who had experience in art making is that such individuals were unlikely to feel the same sort of anxiety that non-artists would feel when asked to make a piece of art about themselves. Because the object of the study was to explore how participants felt when asked to make art using two different approaches, in order to try to determine what it was about these approaches that might help calm or intimidate them, the goal was to have participants who were less likely to feel at ease with art materials.

Instrumentation

Questionnaire

A simple questionnaire was administered for the collection of preliminary data (see Preliminary Information questionnaire, Appendix C, p. 167).

Anxiety Scale

A simple anxiety scale was designed for the purposes of this study (see Anxiety Scale, Appendix D, p. 168). Each participant was asked to rate their level of anxiety before and after each art making task on a scale of 0-5, where 0 indicated no feelings of anxiety at all, and five indicated a high level of anxiety.

Investigational Methods and Procedures

Once participants were recruited, a single session was scheduled for reviewing and signing the informed consent and for data collection. Meetings were held individually, to maintain confidentiality and to prevent the experience from being influenced by the presence of other participants. Sessions were audiotaped. In order to protect the participants' confidentiality, the recorded information was transcribed after

each session and the researcher kept the transcriptions in a secure location. Following completion of the study, transcriptions will be kept in a secure, locked cabinet in the Creative Arts Therapies office for three years. The participants' names will not be used on any of the audiotapes or transcriptions. Instead, a pseudonym has been assigned. There is no identifying information on the artwork. Instead, a pseudonym is being used. Once data collection was complete, the artwork was photographed for inclusion in the thesis. Participants were asked if they wanted to keep the originals. They all refused, but two participants asked for digital copies to be emailed to them, which was done. Since the participants chose not to keep their art, it was destroyed and only the reproductions that are being used in the thesis are being kept.

Data was collected in four sections: informed consent, collection of preliminary data, the art making exercises, and an open-ended interview about their art making experience, based on a pre-developed interview guide (see Interview Guide, Appendix E, p. 169). Before and after each art making task and the interview, participants were asked to rate their level of anxiety on a simple scale that was specifically designed for this study. A 10-minute break was offered between the two art making sections. Only two participants chose to take the break; the rest worked straight through the session. The total estimated time commitment was approximately two hours and 15 minutes, although the average time of the sessions ended up being about one hour and 15 minutes.

Data Collection

Data Collection I - Informed Consent (approximately 15 minutes)

The researcher went over the consent. To ensure that participants comprehended the nature of the study, the researcher asked them to repeat back in their own words their

understanding of its purpose and procedures. The researcher made sure that participants understood that the session would be audiotaped. Once participants were able to show that they understood the process, and they agreed to it, they were asked to sign two copies of the consent form. One was given to them, while the other will be stored in a locked, secure file in the Creative Arts Therapies office for three years.

Data Collection II – Preliminary Information Collection (approximately 5 minutes)

Once informed consent was obtained, basic preliminary information was collected (Appendix C, p. 167):

- age
- gender
- ethnicity
- art experience

Data Collection III - Art Process (50-80 minutes)

1. Participants were asked to make a drawing telling the researcher about themselves using drawing materials (#2 pencil, colored pencils, thin markers, oil pastels and chalk pastels and an 11 x 14-inch sheet of white paper). When necessary, the researcher elaborated on the directive to make sure that participants understood what was being asked.
 - Before the drawing task, participants were asked: “On a scale of 0-5, how anxious or nervous do you feel about having to tell me about yourself in a drawing?”
 - After completing the task, participants were asked: “On a scale of 0-5, how would you rate your level of anxiety or nervousness now?”

- The researcher asked participants to explain their drawing and give associations to anything in it, in order to understand the image.
2. Participants were asked to make a piece of art about themselves using collage materials, an 11 x 14-inch sheet of white paper, a glue stick, liquid glue and scissors. Magazine images, phrases and words were selected in the categories of people (individual, dyads and groups), nature, animals and objects. There were 40 sheets of around eight images each, and 10 sheets of around 15 words or phrases each, for a total of 50 sheets containing 320 images and 150 words or phrases (see Appendices F and G for sample of collage sheets, pp. 170 and 171). To maintain consistency, color and black and white copies were made of all the clippings to make sure that each participant received an identical selection of collage materials and began the art making process with the same content. When necessary, the researcher elaborated on the directive to make sure that participants understood what was being asked.
- Before the collage task, participants were asked: “On a scale of 0-5, how anxious or nervous do you feel about having to tell me about yourself using collage materials?”
 - After completing the task, participants were asked: “On a scale of 0-5, how would you rate your level of anxiety or nervousness now?”
 - The researcher asked participants to explain their collage and give associations to anything in it, in order to understand the image.
3. Participants were told that they could choose which approach to use first.

4. Participants were given approximately 30 minutes to complete each task, and were offered a 10-minute break between tasks.

Data Collection IV – Interview (30-45 minutes)

- An open-ended interview guide was used (Appendix E, p. 169). The objectives of the open-ended guided interview were:
 1. To understand how the participants experienced the act of creating work about themselves using two different art approaches;
 2. To understand what the effect of each approach was on the anxiety level of the participants, who rated their anxiety/nervousness before and after each art task; and
 3. To understand what specific qualities of each approach may have helped generate, address, and lessen or exacerbate participants' reported anxiety.

Interview guide.

- What was it like being asked to make a piece of art about yourself?
- What was it like to use collage materials for this project?
- How easy was it for you to say what you wanted to say about yourself with collage?
- What was it like to use drawing materials for this project?
- How easy was it for you to say what you wanted to say about yourself with drawing materials?
- How would you describe any differences there might have been between the two approaches?
- What were the similarities between the two approaches?
- What informed your choice of using [drawing materials/collage] for your first piece?

Data Analysis

Following each session, the researcher transcribed the audiotapes. Since participants fell into a narrow age range (18-27 years), this information was not coded for analysis. The order in which the tasks were performed was noted to examine how it related to the self-reported anxiety ratings.

Paired sample testing was carried out on the anxiety ratings to determine the probability that differences between the anxiety reported for each art approach, and differences that occurred before and after each art approach, were not due to chance.

The interviews were analyzed for common and divergent themes relating to the main areas of interest of this study: the ability to use drawing and collage to address the task, the effect of the art approach on anxiety level, and the experience of working with the two approaches. The findings were grouped into major themes and described in narrative form.

The art products and the associations provided were examined to determine how effective each approach was in allowing participants to express themselves fully and reveal introductory information, which is the goal of using art as an additional form of self-expression in art therapy sessions. Drawings and collages were determined to be either revealing or concrete, and the results were presented in table and narrative form. The themes and patterns identified from the three data sources were cross-analyzed to establish broader patterns and themes relating to the research question (Patton, 2002; Weiss, 1994).

Operational Definitions of Concepts and Terms

- *Collage*: For the purposes of this study, collage is an approach in which clients are presented with a range of magazine images, phrases and words, from which they make a selection that they then glue onto a blank piece of paper.
- *Drawing*: For the purposes of this study, drawing means that participants will create an image about themselves without any sort of stimulus such as an already-existing picture, but instead will have to generate the image from within themselves, using only a blank sheet of paper and drawing materials (a #2 pencil, colored pencils, thin markers, oil pastels and chalk pastels).

Possible Risks and Discomfort to Participants

Possible risks and discomfort were expected to be minimal, since the participants were functioning adults who volunteered for the study. Participants may have felt some slight anxiety from being asked to make art about themselves, and being asked to talk about their experience with the researcher.

Special Precautions to Minimize Risks or Hazards

- Only volunteer participants who were fully aware of the process before their consent was accepted were involved in the study.
- Participants were functioning adults who were enrolled in a university undergraduate or graduate program.
- Since the objective of the interview was to find out about the participants' experience making art with different approaches, and not about personal issues whose discussion might evoke unduly uncomfortable or painful associations, questions were geared towards the experience, and not towards personal issues.

- No artistic talent was required.
- Although no participants reported experiencing excessive anxiety or discomfort as a result of the study, one did report feelings of loneliness as a result from being far away from home, and another seemed excessively preoccupied with eating issues and her artwork suggested the possibility that she may have been engaging in self-harming behaviors. In both cases, the researcher followed up at the end of the session by reminding them that the number of the Student Counseling Center, 215-762-7625, was included in the consent form, and that the Counseling Center was there to assist students who needed someone to talk to or who had problems or issues with which they wanted to deal.

CHAPTER 4: RESULTS

The research question of this study was: how does the experience of creating a piece of art about oneself with collage compare to that of working with drawing materials, as investigated through interviews with participants following the completion of two art tasks? The specific research objective was to gain a richer understanding of the nature of collage as it compares to drawing. The goal was to investigate how the specific characteristics of two popular art therapy approaches might influence the anxiety that individuals unaccustomed to making art often experience in initial therapy sessions.

This section will present the results of the study as seen in the quantitative data, the interviews and the art. First the anxiety scales will be analyzed. Then the interview responses will be aggregated and summarized and the main themes that emerged will be discussed. The art work will be examined in terms of how invested participants were in the art making when using each approach, and how successfully they were able to address the directive to produce a more revealing piece about themselves. Finally, the major themes that emerged when data from the anxiety scales, interviews and art were organized with respect to the research question will be identified.

Major Findings

The major findings were that collage was initially perceived as being easier and less intimidating than drawing because it provided stimuli for the art making and because the process seemed straightforward, in that it involved selecting ready-made images, cutting, and gluing them onto the page. It was found that collage encouraged insight and exploration because it provided the opportunity to think objectively about what images and words would be appropriate to describe oneself. It also allowed participants to depict

strengths as well as weaknesses, thereby addressing some of the concerns associated with the drawing portion of the study. There were, however, limitations inherent in using other people's images, someone else's selection, and in the collage-making process itself, which was seen to require some degree of organization, and to be somewhat mechanical.

Drawing, for its part, was initially perceived as intimidating because of artistic preconceptions, concerns regarding loss of meaning, fears about not coming up with ideas, and unfamiliarity with drawing materials. The anxiety scales showed that participants experienced significantly more anxiety before doing the drawing than before the collage.

A comparison of the two approaches found that collage was felt to be more restrictive, whereas drawing offered more opportunity to express oneself freely.

Participants

The participants were eight graduate and undergraduate students of Drexel University with no formal artistic experience (see Table 1, p. 55). One participant, Sid, said that when feeling lonely or sad he liked to copy pictures as a way of distracting himself, but that he had not done so in a while. Lia reported using drawing, although not frequently, as a way of working through problems, but since she met the inclusion criteria and was not and had never been in a fine arts program, she was considered eligible for the study. Participants' ages ranged from 18-27. For many participants, English was not their first language and their familiarity with colloquial English was limited. This is reflected in some of the quotations, which are reproduced verbatim.

Table 1

Participant Information

Gender		Ethnicity		Nationality			
M	F	Asian	Caucasian	American	Chinese	Indian	Korean
4	4	6	2	3	1	3	1

Quantitative Data

Before and after each task, participants were asked to rate their level of anxiety or nervousness on a scale of zero to five, where zero was no anxiety or nervousness and five represented extreme anxiety or nervousness.

Participants were allowed to choose which art making approach to use first.

Table 2 (p. 56) shows the order in which participants chose to do the two art tasks, and their reported anxiety levels before and after each task. The means of the anxiety ratings before drawing, after drawing, before collage, and after collage were calculated, as were the means of the changes that occurred in anxiety ratings before and after drawing, and before and after collage.

Table 2

Order of Task and Self-Reported Anxiety Ratings

Participant	Task order	Drawing task			Collage task		
		Anxiety before	Anxiety after	Change	Anxiety before	Anxiety after	Change
Tony	Collage – Drawing	4	3	-1	2	1	-1
Joy	Collage – Drawing	0.5	0.5	0	0.5	0.5	0
Emma	Collage – Drawing	3	0	-3	1	2	1
Raul	Collage – Drawing	2	1	-1	1	1	0
Hannah	Collage – Drawing	4	1	-3	0	0	0
Sid	Collage – Drawing	1	4	3	2	0	-2
Lia	Drawing – Collage	3	2	-1	4	2	-2
Bill	Drawing - Collage	3	2	-1	1	0	-1
<i>M</i>		2.56	1.69	0.88	1.44	0.81	0.63

Paired t-tests were carried out comparing the means before drawing and before collage, after drawing and after collage, and the means pre- and post drawing, and pre- and post-collage (Table 3, p. 57). These tests were undertaken to determine the probability that the differences between anxiety reports relating to collage and to drawing were not due to chance.

Table 3

Paired T-Tests Comparing Means of Anxiety Scale Ratings

Variable 1	compared with	Variable 2	<i>p</i> value
Anxiety before drawing (<i>M</i> = 2.56)		Anxiety before collage (<i>M</i> = 1.44)	0.05
Anxiety after drawing (<i>M</i> = 1.69)		Anxiety after collage (<i>M</i> = 0.81)	0.11
Anxiety before drawing (<i>M</i> = 2.56)		Anxiety after drawing (<i>M</i> = 1.69)	0.12
Anxiety before collage (<i>M</i> = 1.44)		Anxiety after collage (<i>M</i> = 0.81)	0.07

The mean level of anxiety was higher before the drawing task (*M* = 2.56) than before the collage task (*M* = 1.44). The paired t-test comparing these two anxiety means was significant (*p* = 0.05), suggesting that people who are not accustomed to making art are very likely to experience more anxiety before doing a drawing than before making a collage. A second t-test comparing mean levels of anxiety after the two tasks was not significant (*p* = 0.11), although the mean after drawing was higher than after collage, as can be seen in Table 2 (p. 56)

The mean reported anxiety level fell from 2.56 before the drawing task to 1.69 once the task had been completed, representing a 48 percent decrease. However, the paired t-test comparing the mean anxiety levels before and after the drawing was not significant (*p* = 0.12). The mean anxiety level before the collage was 1.44, dropping 43 percent to 0.81 upon completion. The paired t-test comparing these two values was marginally significant (*p* = 0.07).

Verbal Associations in Relation to Anxiety Scale

When asked to rate their anxiety or nervousness during the session, there were some responses that merit mentioning because they may have affected the results to some extent. Sid said that his anxiety level before the first task that he selected, the collage, was 2, but would have been 4 if he had been asked to rate it immediately at the start of the data collection session, before the demographic data had been gathered and his previous art experience discussed. He said that “when you told me first, I was really anxious, but now as I’ve talked about it, and I’ve now seen the pictures as well, I’m somehow kind of less anxious than I was before.” Bill, for his part, said that he had confused the words “anxiety” and “excitement,” and that he should have rated his pre-drawing anxiety as a 2 instead of 3.

Increased Anxiety During the Art Tasks

For most participants, the reported anxiety rating decreased or remained stable once the art tasks had been completed, with two exceptions. Emma reported that her anxiety had been exacerbated during the collage-making by the mechanics of trying to fit everything that represented her onto the paper. Sid noted a three-point hike in his anxiety level during the drawing portion, from a 1 before starting the drawing task to a 4 once it had been finished. He explained that the drawing task “kind of brought out my weaknesses,” the end product “didn’t look great as it should,” and noted that drawing “will kind of shake the person (...) you’ll be irritated, you will not be able to draw well.”

Anxiety Change During Drawing Portion

Six of the eight participants reported a decrease in anxiety once the drawing task was completed. Tony and Hannah ascribed the drop in their anxiety levels after the

drawing to relief at being done. Bill said that he had been “anxious” before the drawing task and attributed the drop to “finding out that you were not looking at my drawings. (...) There’s no judgment, you are not comparing with others. You just want to know why – and when I had a look, I thought, ok, it’s ok, it’s not that bad.” Raul noted that “I didn’t dislike this exercise [drawing] as much as I thought I would.” Emma said her anxiety level had dropped from 3 to 0 because of the feeling of “emotional release” that drawing provided, a response that she said surprised her. Lia attributed her pre-drawing anxiety score of 3 to the fact that she did not know what she was going to draw, or how she was going to do it. She said that as she was organizing her drawing, she was able to clarify and organize her thoughts, which led to decreased nervousness.

Anxiety Change During Collage Portion

Four participants reported a decrease once the collage task had been completed. Tony ascribed his initial nervousness to uncertainty regarding the study itself. “I wasn’t 100 percent sure what we were doing,” he noted, adding that “I figured it out pretty quickly.” Lia also attributed her pre-collage anxiety to unfamiliarity with the approach, saying that her nervousness decreased post-collage because “I finished it (...) and I think it wasn’t as bad as I had thought, and it also helped me evaluate about my problem.” Bill reported a one-point drop in anxiety level once the collage task was completed, saying that he was feeling “no nervousness and fully excited.” Sid’s anxiety fell from 2 to 0; later in the session, he commented on the soothing nature of collage, and the many images in the selection with which he was able to relate.

No Changes in Anxiety Level

Joy reported no change in anxiety level throughout her session. Raul and Hannah reported no change in anxiety level during the collage portion of the session. Joy said she was aware of her own limitations and therefore had not been worried about her art products. Hannah, when asked about her unchanged 0 anxiety rating during the collage task, noted that “I didn’t feel worried. I guess I saw, like, that there looked like there was a lot of selection, so I didn’t get too worried.” Raul’s anxiety level also did not change, remaining stable at 1. Before rating his anxiety/nervousness level pre-collage, he said that “it actually depends on the pictures there.” His unchanged rating, along with his descriptive explanation of his collage, suggests that he found enough images to satisfy his needs with respect to this task.

Time Spent on Art Tasks

A final note relates to the amount of time spent on the two pieces of art. Participants spent an average of 17.63 minutes on the drawing, and 28.25 minutes on the collage. These figures could reflect their heightened anxiety when addressing the drawing portion of the study, but could also reflect the additional time needed to complete a collage, which involves selecting, cutting, arranging and pasting a collection of images on a page.

Interviews

Responses to the open-ended interview questions are aggregated and summarized in Table 4 (p. 62). Shared responses are those that are common to at least two of the participants, while variant responses are those made by only one participant. The number in the parentheses at the end of each summary statement indicates how many participants

shared that response. Significant non-verbal behaviors are also represented in the table.

At the end of the section, there will be a discussion of the main themes that emerged from the interviews, with a sampling of participant responses.

Table 4

Aggregation and Summary of Open Ended Survey Responses

Question	Summary Notes
Q.1 What was it like being asked to make a piece of art about yourself?	<u>Shared Responses</u> • Not difficult, not a problem (4) <u>Variant Responses</u> • “I’m not good at art, so I was a little bit nervous” • Directive was too broad for collage portion • Directive “was free enough that I could just let myself free to open ideas,” “I could start from anywhere” • Not difficult but strange, because “not many people explicitly ask you to express yourself”
Q. 2 What was it like to use collage materials for this project?	<u>Shared Responses</u> • Perception that collage would be easier/was less intimidating than drawing (7) • Need to organize material when gluing/deciding how to place on paper/what must remain fully visible/what can be hidden (6) • Process is too mechanical/systematic/“high-tech”/just cutting and pasting (5) • Encourages exploration/insight/thinking about self from different perspectives (5) • Collages are easy to make (4) • Need to organize material when selecting/deciding what to keep and what to discard/looking through images to find right ones (3) • Materials are interesting/exciting (2) • Unfamiliarity with collage evoked anxiety (2) <u>Variant Responses</u> • Directive too broad and participant was “overwhelmed by the number of things that I felt like I identified with” • “One aspect of collage is satisfaction” <u>Non-verbal Responses</u> • Asked to keep some images selected but not used (2) • Asked for photos of their collages (2)

Q.3

How easy was it for you to say what you wanted to say about yourself with collage?

Shared Responses

- Images offered a starting point for the art making (5)
- Using someone else's selection imposed restrictions (5)
- Structure of collage imposes limitations (4)
- Images influenced/restricted/guided art content (3)
- Need to adapt focus of someone else's image to suit their needs (3)
- Range of images/words was broad enough (3)
- Image selection was too restricted (3)
- Number of images/words offered was overwhelming (2)
- Too small a selection would have been stressful (2)
- Images helped focus thoughts and art making (2)
- Once images glued, cannot be undone (2)
- Overlapping images can alter meaning (2)

Variant Responses

- When desired image cannot be found, "you just have to either make them like I did (...), or you just have to think of a new idea"
- Could show positive aspects of self with collage that could not with drawing
- Had to discard a significant image because too large

Non-verbal Responses

- Participant seemed to feel compelled to use every image originally selected

Q.4

What was it like to use drawing materials for this project?

Shared Responses

- Idea was challenging/intimidating (7)
- Negative comments about drawing skills/concerns that art would be judged (6)
- Concerns that art would be misinterpreted/misunderstood (6)
- Drawing ultimately allows more freedom in what artist can say about him/herself (6)
- Drawing is more personal/there is a greater feeling of attachment to it than to collage (4)
- Drawing encourages creative thinking/requires thinking "outside the box" (3)
- Limited drawing skills restrict self-expression (2)
- Drawing is more kinesthetically satisfying (2)

Variant Responses

- Drawing initially more intimidating, but provided “emotional release”
- Drawing will “shake the person up” and cancel out the positive aspect of trying to express oneself

Non-verbal responses

- Selection of more structured art materials: pencil, colored pencil, felt-tip markers, one request for pen (5)

Q.5

How easy was it for you to say what you wanted to say about yourself with drawing materials?

Shared Responses

- Negative comments regarding final drawing (7)
- Concerns about what to draw/having no ideas/dealing with blank page (7)
- Drawing abilities influenced content (4)
- Unfamiliarity with art materials posed a challenge (4)
- Prior collage task was stimulus for at least one aspect of the drawing (3)
- Unfamiliarity with materials influenced content (3)
- Unfamiliarity with materials/fear of “messing up” influenced choice of material (3)
- Providing associations helped address fear of being misunderstood (3)
- Drawing not as bad as originally expected (3)
- Being able to plan drawing/make draft/would have helped (2)

Variant Responses

- Availability of colors aided self-expression, influenced content; colors “did add some dimensions”
- Dissatisfaction with way colors looked on page
- Providing verbal associations to drawing did not help address fears of being misinterpreted

Q.6

How would you describe any differences there might have been between the two approaches?

Shared Responses

- Collage requires picking from imagery that already exists, while with drawing there is freedom to create what is needed (5)
- Collage is structured but restricted by selection, while drawing materials/blank page allow for freedom and creativity (5)
- Drawing is more personal, while collage is more of a process (4)

- With collage, once an image has been glued down, it becomes a restricting factor, whereas there is free use of space in drawing (2)
- There are no rules in drawing: possible to depict past, present, future/play with perspective/play with proportion, while in collage such factors limited by image/word selection (2)

Variant Responses

- Drawing exposes weaknesses, while collage does not; drawing “brings more out of a person” and “will kind of shake the person”
- With drawing, person can be “more bold” and surprise the researcher, because the researcher knows the collage selection, but doesn’t know what is going to come from within the person in the drawing

Q.7

What were the similarities between the two approaches?

Shared Responses

- Both encourage thinking about oneself (2)
- They are very dissimilar (2)

Variant Responses

- “They’re both making me create something” and “they both start with the same size paper”
- “I never felt suppressed” with either approach
- Both allowed for freedom of expression

Q.8

What informed your choice of using drawing materials/collage for your first piece?

Shared Responses re picking collage first

- Perception that it would be easier (3)

Variant Responses re picking collage first

- To collect herself/focus before going it on her own
- Collage was less intimidating
- Collage seemed interesting
- Collage provided visual stimuli

Responses re picking drawing first (both variant)

- It was more familiar
 - Wanted to start with more difficult, less exciting approach, and end with collage
-

Once the interviews had been aggregated and summarized, they were organized in response to the research question: how does the experience of creating a piece of art about oneself with collage compare to that of working with drawing materials? Responses were organized in terms of the study's objective of gaining a better understanding of the nature of collage as it compares to drawing, and of examining the specific characteristics of each approach and how they might have influenced anxiety levels. Some examples of participant responses are provided to illustrate each theme.

Working With Collage

Collage Seems Easier and Less Intimidating Than Drawing

Six of the eight participants chose to start with collage, explaining later that this was because it seemed easier and/or less intimidating than drawing, and almost all the participants, even the two who elected to do the drawing first, used a variation of the term “easy” to describe their perception of collage in relation to drawing. Bill began with the drawing because he wanted to leave the collage, the idea of which “excited” him, to last, saying that “collage is easier in the sense that you have a lot of choices to start from.” Lia, who started with drawing because she was unfamiliar with collage, noted afterwards that “collage is a lot easier, because all you have to do is organize it in a nice way, and I think pretty much everyone can do that.” The main reasons as described by participants is that it provides visual stimuli, helps focus the art making and is easy to put together.

Visual stimuli offer starting point for art making. Several participants noted that the selection of images and words offered a starting point when faced with the blank page. Tony said that “there’s already things you can pick from” and that the images were like “building blocks” that he could use to create something. With drawing, he pointed

out, “you are just kind of given a blank page and [asked to] fill [it].” Collage was preferable because “just looking at a blank sheet of paper and putting something on it is a bit easier when you have things to build with.” Raul said that having the images as a starting point “helped me to give [the] answer easily, a faster answer,” adding that “for me it’s kind of difficult because I don’t have – I don’t know exactly where to start, or what to draw.”

Visual stimuli help focus the art making. Emma said she chose to do the collage first “to kind of get into the whole idea of, like, collecting myself before I went into doing it on my own.” She also found that the images helped focus her creativity, since drawing triggered more nervousness because “I actually had to think about what described me,” whereas the images helped her identify elements that did. Sid talked about using the images to “channelize all my energy,” adding that they “will (...) focus me towards what I want to say, without any obstacles and much more, much more freedom of expression.”

Collages are easy to make. The perceived simplicity of collage-making seemed to appeal to several participants. Lia noted that “all you have to do is organize it in a nice way, and I think pretty much everyone can do that.” For Joy, all she had to do was select from “many different samples,” and then just “cut them off and collect it on the paper.” Hannah also reported feeling less nervous with the collage “because all you do is cut and paste, so everything is given to me,” a feeling echoed by Lia, who said that “the images are given to me, and all I have to do is just pick it up and put it in the order that I wanted.”

Challenges to Working With Collage

Participants also noted some challenges to working with collage. These related to working with someone else's selection and with someone else's imagery, and to the actual process of making a collage, which requires organization and the repetitive actions of cutting and pasting.

Working with someone else's selection. Working with a selection of images made by somebody else evoked varying responses from participants. Joy said that she would not use images of people to represent aspects of herself, while Bill felt that though there were "a lot of choices to start from," the selection "set you in one direction." Tony could not find the punctuation marks he wanted, and ended up cutting out the shapes of the marks he wanted to include. Raul said that he could not find pictures to represent the future in his collage, "so I found that this future part was missing" in his piece. He noted that "I had to think within the box. I could not go outside the box. I could use only those pictures."

Tony and Emma found the size of the selection to be overwhelming. Tony said that "the hardest part" of the collage "was finding the right pictures, because there were a ton of them." Emma reported feeling "overwhelmed by the number of things that I felt I identified with."

Hannah and Sid noted that if the selection had been more limited, it might have evoked anxiety, whereas having a broad range of images did not.

Working with someone else's images. Some participants commented on the fact that a ready-made image has its own inherent bias, in that it offers the photographer's perspective. Raul, Emma and Tony talked about the need to adapt the focus of an image

to suit their ideas. Raul said that “I didn’t really focus exactly [on] what the picture is trying to say; I focused on how I related to the picture, maybe a small part of the picture.” Emma said that “you have to (...) change the focus of the picture to make it, like, say, yours.” Tony used parts of pictures to make a composite to represent the persona he adopted for his online activities, modifying several images in the process.

Collage selection can influence content of the art. The selection of images/words seemed to have influenced the content of some of the collages. Raul noted being unable to depict elements relating to the future because “I couldn’t find some pictures, like the monuments or the plane.” He added that “if I had some of these kinds of pictures, I would have depicted what I want to of the future.” Lia left out an image of a sewn mouth which she had wanted to use to illustrate her inability to speak to her parents because it was too large for her collage. Furthermore, while she had several ideas in mind when starting the collage, the selection she was given determined which one she ultimately used.

Challenges imposed by collage-making process. Participants also commented on the mechanics of putting a collage together, with some expressing dissatisfaction with the process. Three elements that seemed to frustrate some participants were the need for organization, the fact that once glued onto the paper, an image cannot be moved, and the mechanical aspect of the process.

Need for organization when selecting the images. Organization when making a collage seems to apply to the process of selecting images and to the act of gluing them onto the paper. Lia attributed her difficulties in organizing her images to her inexperience with making collages. Tony commented that he had not kept the images “in

any sort of order, but I probably should have” to help him find those that he needed for his piece. Hannah reported going through “every single picture” before deciding which ones to use, saying that she might have missed certain significant images if she had not gone through the selection a second time, since she was not the one who put it together in the first place and was therefore unfamiliar with what was available.

Need for organization when gluing the images onto the page. Several participants noted the need for organization when committing the image to paper. Emma was observed carefully laying out her selection on the page before gluing it down, and using every image chosen. She reported an increased level of anxiety after making the collage, attributing it to “the actual procedure,” which had forced her to make decisions about what parts of her chosen images could be covered up by others, and which ones she felt had to be visible. In addition, she seemed to feel committed to the selection she made, and experienced some frustration when trying to fit all the images and words on the page. Bill also noted that organizing the images on the page can be “a bit difficult” and “if I overlap some pictures, then it will misrepresent, it will soon distort all that is under what is already there. So that was a limitation.”

Commitment. There was also the idea of not being able to alter the elements once they had been glued down. Joy and Lia both noted wanting to change part of their collages, but being unable to do so because they had already glued down some images. Lia attributed this element of commitment (Wadeson, 1987), or the inability to undo an action, to her unfamiliarity with the approach.

Mechanical aspect of collage-making. The way some participants described the act of making their collages made it sound mechanical. Joy said that for the collage, “I

just need to cut [the images] off and collect them on the paper.” Lia said that collage was easier “because all you have to do is organize it in a nice way.” Hannah noted that one of the aspects of collage that made it less intimidating than drawing was that “all you do is cut and paste” because everything the person needs is provided. Emma pointed out that once she had selected the elements that related to her, “it was just cut and glue, cut and glue.” Raul felt that “the pictures helped me to give [the] answer easily, a faster answer,” whereas the “drawing part was more of a free way.”

Collage Encourages Exploration and Insight

The act of picking images to which they could relate, and then organizing those images on the page in ways that they felt best represented who they were, seemed to encourage exploration and insight by participants. Lia, for example, said that the images and words suggested other ways for her to look at herself and her experience, perspectives she would not have considered when working on a drawing. Emma said that the process of deciding how to arrange her imagery made her think about which aspects of herself were more important and had to be visible, while Bill noted that layering images one on another could distort the meaning that he was trying to convey.

Collage “Easy, But It’s Not Complete”

Bill said that “collage is easy, but it’s not complete. Drawings are difficult, but you are free to do whatever you want.” He noted that collage provides a starting point, but those stimuli are also a limitation, since they will “set you in one direction,” as opposed to the freedom that the blank page provides. Hannah felt she could depict “different things that I wouldn’t, couldn’t say through a collage, like I couldn’t talk about

my family in the collage.” Sid also felt that drawing “kind of complemented whatever was left there [the collage].”

Need to Provide Appropriate Materials

A practical factor that emerged in this study that had not been considered by the researcher was that provisions must be made for people who are left-handed. Bill reported having trouble with the scissors because he is left-handed, and found the ones provided to be heavy and awkward. Left-handedness can be overlooked when providing collage materials, but it is an important factor when the goal is to facilitate creativity.

Working With Drawing Materials

Aesthetic Preconceptions

Seven participants made some type of negative comment regarding their drawing skills when asked about their reported anxiety levels, and the same seven were critical, to a greater or lesser degree, of their final product. Emma was the only one who did not specifically mention her artistic abilities, although she did say that “collage is easier because it’s less intimidating of a, like, of a venture” and reported being “worried” about “making the drawing.” Bill said that his anxiety level had decreased when he realized “that you were not looking at my drawing,” and that “there’s no judgment.” He said that “drawing has some benchmark” regarding what can be considered an acceptable piece of art. Even though Lia reported drawing on occasion, she noted that she would have made a more satisfactory drawing if she had had more time to plan it out. Sid felt that his limited drawing skills resulted in the exposure of more weaknesses than occurred in the collage, where he was able to show his humorous side as well as more serious aspects of

himself because he could resort to images to tell his story. “It definitely brings out your weaknesses,” he said, referring to drawing.

Fear That Art Could Be Misinterpreted or Lose Meaning

A related concern that emerged as participants were expressing their negative perceptions of their own artistic abilities was the idea that their art might be misinterpreted. Tony said that “you worry that you end up losing, like, meaning if you don’t do it correctly or something” and that “if you’re not a good artist, it kind of, like, what if I – it isn’t clear.” Emma noted that “sometimes you see one thing in your head and then, like, if it doesn’t translate the same on paper, it’s uncomfortable.” Raul said that he appreciated being able to explain his drawing, because “I’m not good at drawing, so probably you could have interpreted some things a little different than they would have actually been.” When referring to the labeling in her drawing, Hannah commented that “I didn’t think that you would be able to tell what this is unless I put it.” Sid also noted that his anxiety was related to the fact that “I couldn’t really translate it based on my skills” and “I bet you wouldn’t have understood it [the drawing] just by looking at it.”

Relief at being able to report their associations to the art. Raul, Sid and Bill said that being able to explain their drawings helped address their fear of being misunderstood. Raul said that giving his associations had allowed him to make the most of both approaches. Although Sid initially said that explaining his drawing had not helped allay his anxiety, because the collage experience had been so positive and the drawing task so stressful, he later commented that “explaining definitely overcomes all the (...) shortcomings in the drawing.” Bill, when discussing the reasons why his anxiety

level dropped once he had completed the drawing, noted that “I could explain a few things, and you were more interested in what I have done, not how I have done.”

Drawing Skills Influence Content of Artwork. Concerns about how to depict an idea with drawing materials influenced the content of some of the drawings. Lia said that “I couldn’t figure out how to draw it – my major problem – so I moved on to my secondary, sort of, problem.” Lia, who has no artistic training but draws on her own when she has a problem that she needs to deal with, said that “I still have limitation on my drawing, and when I feel like I cannot draw it specifically, I get stressed and I give up on it.” Joy abandoned a section of her drawing because she could not draw it well, according to her standards. Sid reported that because of his limited drawing skills, he was unable to give as well-rounded a self-portrait in the drawing portion as he was in the collage. In his collage, he used the image of a man making a funny face to represent his own humorous side, commenting later in the session: “But how could I draw funny? (...) Average people like me cannot draw funny.”

Feeling Intimidated by Blank Page and Not Having Ideas

There was also a general sense of intimidation regarding being faced with a blank page and being asked to draw without any stimulus. Tony said that with drawing, “you are just kind of given a blank page and [asked to] fill [it],” and that “just looking at a blank sheet of paper and putting something on it is a bit easier when you have things to build with.” Emma also said that collage is “less intimidating of a (...) venture” than drawing because the latter involves first thinking about what to draw, and then the actual process of drawing. Raul and Bill reported that knowing “where to start,” an expression both used, was a challenge. Hannah said that she had been worried about “not having

enough ideas,” sentiments echoed by Sid, who related his pre-drawing anxiety to concerns that “I couldn’t really have ideas, and I’m feeling anxious.” Joy said that “I don’t know what to draw, so I just draw something randomly,” and towards the end of her drawing session began copying elements from a poster that was hanging on the wall, suggesting that her ideas may have run dry, that dissatisfaction with the way her drawing was looking was discouraging her from further creativity, or that the exercise was proving too anxiety-provoking for another reason.

Emma, Hannah and Raul reported using the collage task as a stimulus for at least one aspect of their drawings. Emma said that “I think seeing the, like, the words specifically in the collage helped me narrow my focus for this [drawing].” Hannah reported that “if I had just sat down here [for the drawing], I’m not sure I would have put (...) something about nature” in the drawing. Raul said that “I saw the pictures, I started relating with the pictures, and then when I took the drawing, I had some direction.”

Working With Unfamiliar Materials

Another theme that emerged regarding the drawing experience related to the challenges of working with unfamiliar materials. Bill asked for a pen for the drawing part, explaining later that “I usually make strokes of pen better than pencil, and I’m not very confident with drawing so I make very light strokes, especially with pencil.” Tony said that using drawing materials was a little more “challenging,” since “I really have no experience with pastels, or oil pastels, or whatever else is out there.” He added that “I would use pencils, mostly” if asked to draw again. When discussing his drawing, he noted with dissatisfaction that he had tried to show the excitement of being with thousands of people who share the same interests by using lighter colors, but that “I

hadn't realized the red was so dark." Hannah opted for a medium she could control, saying that "I love pastels, but I didn't think I'd be able to control them the best, so I went with colored pencils, because I thought that they'd be the easiest for me to draw with." She added that "I used only one thing, though, because I was scared of touching the other stuff and messing up." Five participants used pencil, colored pencil or felt-tip pens, materials that offer more control.

A variant response from Sid was that the availability of colors influenced his drawing: "If you had given me just pencil, no colors, it would have been awful" and the colors "made me feel good about at least the colors, the brightness." He said that "if I wasn't given green and brown, I would not have drawn a tree, because that would have been more difficult, for me to draw a tree," a comment that also relates back to the idea of the content of the art being influenced by perceived artistic limitations.

Drawing Offers More Opportunity for Freedom of Expression

When participants were asked how easy it was to say something about themselves using each approach, most said that it was easier to do so with collage, but that despite this fact, drawing ultimately offered the opportunity to express themselves more freely because drawing can be more personal, is not restricted by the need to use ready-made images and words, promotes creative thinking, and provides a kinesthetic as well as visual experience.

Drawing is more personal. Emma said that self-expression "is easier to channel when doing a drawing," which makes it "more personal than [collage]," adding that during the drawing task "you were just more in touch with yourself." Sid felt that with drawing, "you kind of feel a bit more bold to draw whatever you want." He noted that

the researcher/therapist who selected images for another person would never be surprised or shocked by the content of a collage, “because these are pictures that you have given, so you have introduced a bias.” “Even if you [present] a few hideous pictures which I can pick, you won’t still be surprised because *you* put them in the first place. But if I want to shock you, I can still shock you by drawing something unexpected.” He also said that drawing “kind of makes you stand up for who you are, and it makes you kind of accept the way you are.” For Lia, drawing “provides me with more opportunities” because “there are some things I cannot express” with collage. Raul noted that with collage, “the main point was [the] picture, and [the] secondary part was me, how I related to the picture,” whereas with the drawing, since he had to decide how to show what he wanted, the focus “was first me, and then the drawing, the colors.”

Drawing is not bound by rules or structure. Raul reported that with drawing, he could “just go in any direction, talk about anything that I like, I don’t like,” feelings echoed by Hannah, who said that “I had any option I wanted, I could go in any direction I wanted,” whereas with the collage she had to choose from “a lot of different options.” Raul also noted that he “was drawing what I actually wanted to draw” and that he could relate the drawing “to past, present, future. So it was more freedom than” collage. Bill said that “drawings are difficult, but you are free to do whatever you want.”

Drawing encourages creative thinking. Raul said that with drawing, “I had to think outside the box” because there were no restrictions in terms of pre-selected images and words. Bill, for whom “drawing is all of your own” and offers “more freedom to express yourself because you’re not limited by the options,” said that “I was free to rotate page and use perspective.” Lia noted that as she drew, and as she organized her drawing,

“I organize my thoughts as well.” Tony did not seem to feel more creative when drawing, but did report having used “symbol shapes” to compensate for his lack of skills, which was a creative way to address his limitations.

Drawing provides kinesthetic experience. Both Emma and Hannah commented on the kinesthetic aspect of using drawing materials. Emma said that she wanted to pick just one medium - in her case it was oil pastels - to “have the same feel, like, throughout your hands as you do it.” Hannah noted that drawing “is more relaxing than sitting down and just cutting,” and that using drawing materials “feels better, and it feels more relaxing to me than cutting and pasting.” “It’s something to do with the way that your hand glides on the paper when you’re using something that’s smooth,” she said.

Catharsis. Emma, who used oil pastels to depict a state of mind, said that even though the idea of making a drawing had initially intimidated her, it ultimately was the more satisfying experience because it provided a feeling of “emotional release.”

Comparing Collage and Drawing

The major theme that emerged when participants were asked to compare the two experiences was that with collage, they had to select what fit them from a range of words and images, whereas with drawing materials, they were free to create what described them. Hannah said that “I’d have to pick what fits me” when it came to the drawing, while with collage “it was like, here’s a bunch of different pictures, tell me which ones fit you.” Raul said that the two approaches had given him “the opportunity to think about me in two separate ways. In one way was your forced way, like you forced me to think about myself within those pictures, like within the box. (...) But in this one [drawing] I had my own options. I had to think outside the box because there was no box in this. So

I could just go in any direction, talk about anything that I like, I don't like." Sid also said that with the drawing, the individual was "creating emotions," but with the collage, he was "picking emotions."

Artwork

After each art task was finished, participants were asked to explain their piece to the researcher. The artwork (Figures 1 – 8, pp. 81, 83, 85, 87, 90, 93, 96 and 99, respectively) was examined in terms of the concrete versus metaphorical nature of the participant's artwork, and in terms of level of investment.

The primary criterion for determining whether a piece was more metaphorical and revealing of emotional content as opposed to concrete was the depth of the associations provided, and the ability to use the imagery as metaphor for experience and emotion. Associations were felt to be more personal when they covered such elements as feeling states, ways of coping with a certain situation, and beliefs, and were not restricted to interests and concrete aspects of self. Since the purpose was to examine how each approach engaged participants in the art making and how effectively participants were able to introduce themselves to the researcher, defenses suggested by the artwork were not examined.

Level of investment was also examined to help get an overall sense of how engaged the participant was in the art making. Investment was determined by examining the art elements present, particularly in terms of use of space, color, pressure, how the collage images were prepared and organized, and line quality, where applicable.

Table 5 (p. 80) summarizes the principal findings of this examination.

Table 5

Summary of Analysis of Artwork

Participant	Drawing		Collage	
Tony	Feeling state	Metaphorical	Self-identity	Metaphorical
Joy	List of likes	Concrete	List of likes	Concrete
Emma	Feeling state	Metaphorical	Self-identity	Metaphorical
Raul	List of likes	Concrete	Self-identity	Metaphorical
Hannah	Concrete aspects of self	Concrete	Self-identity	Metaphorical
Sid	Feeling state	Metaphorical	Self-identity	Metaphorical
Lia	Feeling state	Metaphorical	Feeling state	Metaphorical
Bill	Concrete event	Concrete	Self-identity	Metaphorical

Each participant's art is reproduced next, in the order in which the task was completed, followed by a table giving a detailed analysis of the art elements and the associations provided by the participant.

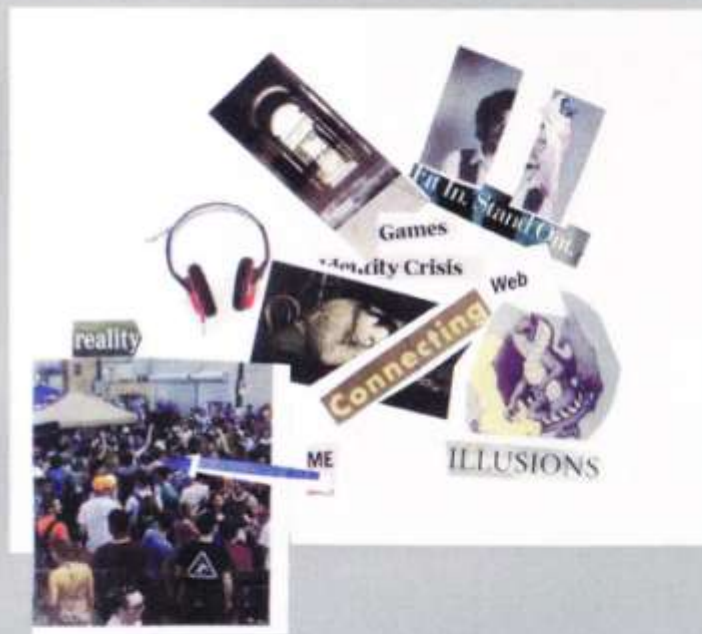


Figure 1. Tony

Table 6

Tony - Analysis of Artwork

Drawing		
Art elements	Content	Associations
• Weighted towards center-top, self image hanging off left bottom corner, rotated perspective • Full, symbolic use of color; self depicted in black line • Even pressure but self figure drawn more faintly than rest • Line quality strong	Metaphorical representation of feeling state	• Represents “excitement of going to my first anime convention” • “There was a lot of people there, and a lot of people who share the same interests which is nice, I guess, and I tried to show that with lighter colors but I hadn’t realized the red was so dark.” • Regarding the exclamation points: “Well you hear some very strange things there. It’s more one of those question marks with an exclamation point. They’re called iterobbang.”
Collage		
Art elements	Content	Associations
• Weighted towards center, with portion hanging off left bottom corner, somewhat disorganized • Somber colors • Multiple images layered over each other in seemingly spontaneous, unplanned way	Metaphorical representation of self-identity	• “I really didn’t have any friends (...) so what I ended up doing was I played a lot of online games to the point (...) where I was doing 40 plus hours a week every week during middle school.” • Split image represents his online and his offline personas: “Over time I’ve pretty much become more like my character [in the online game] than I had originally been, so I just kind of had to pick random stuff because there’s really nothing I can cut out of there that will modify that enough to make it look different.” • “In real life, I kind of just blended in.”

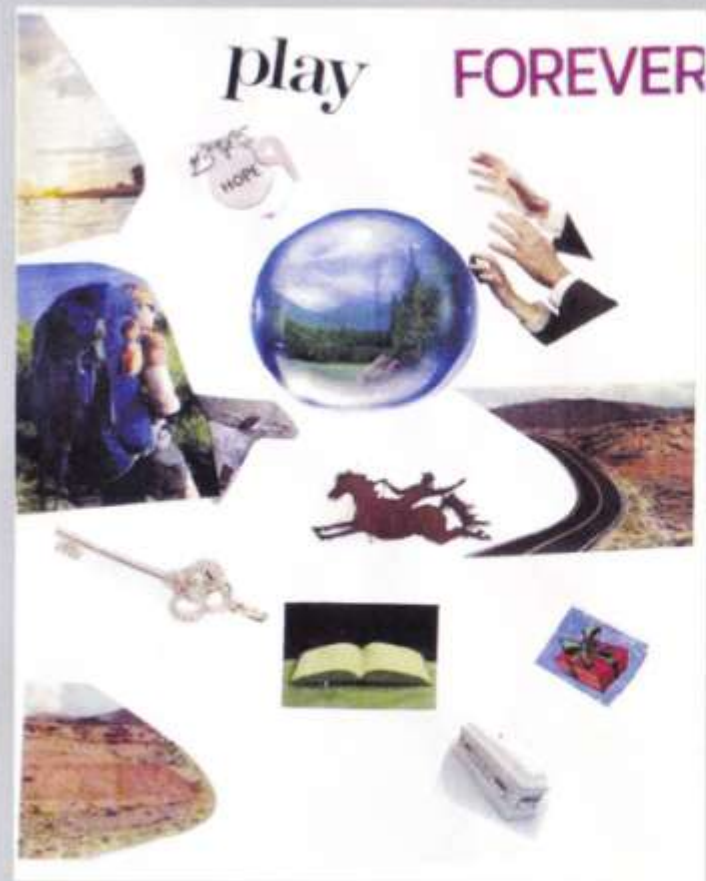


Figure 2. Joy

Table 7

Joy - Analysis of Artwork

Drawing		
Art elements	Content	Associations
• Full use of page, lots of white space, floating elements • Color sometimes randomly used • Varied pressure • Line quality broken, agitated, sketchy	Concrete representation of self, focusing on likes	• “I don’t know what to draw, so I just draw something randomly” • “I meant to draw the sea and the beach, and it didn’t look well” • Seagulls represent “my favorite poem” • Flowers represent movie she liked “so I draw the flower here” • “I just drew some lines here and then I think it could make an eye” • “I meant to draw a girl, because I’m a girl”
Collage		
Art elements	Content	Associations
• Full, most elements isolated from each other • Full range of color • Images neatly cut out and placed on page	Concrete representation of self, focusing on likes and desires	• “I love traveling” • “I love nature” • Words refer “to what I like and what I want” [the word “play”] “and this means that always look to bright side” [the word “hope”] • Coffin represents “a music box” that, along with the wrapped present, illustrate “that I like to decorate”



Figure 3. Emma

Table 8

Emma - Analysis of Artwork

Drawing		
Art elements	Content	Associations
• Expansive use of space • Intense use of color • Intense, pressured energy, colors smeared in controlled way • Strong, horizontal line quality and movement	Metaphorical representation of a feeling state	• “I’m definitely one of those people that [are] living in what could happen or what should happen or what I would like to happen,” and “you don’t know where you actually are, you don’t pay attention to what you’re actually living in, or what’s actually there.” • “All you see is what could be, or that things should be better for you.”
Collage		
Art elements	Content	Associations
• Expansive use of space • Full range of color • Multiple images carefully positioned, layered and glued onto paper	Metaphorical representation of self-identity	• “I am Muslim but I went to Catholic school, so I’m more of a believer (...) [that] they’re all under the same book” • “Beaches and water [have] always been like a very prominent thing so it’s weird to move to different cities without that at your disposal.” • “I am a social person. I do like to try to give back, I try to volunteer.” • “I do get jealous. I have an older sister. Severe second child syndrome. I am stubborn, comes from my dad. And if someone messes with me, I do fight back.”



Figure 4. Raul

Table 9

Raul - Analysis of Artwork

Drawing		
Art elements	Content	Associations
• Full, somewhat disorganized use of space • Full range of color, some color used randomly • Pressure seems to decrease moving from top to bottom of page • Line quality mix of agitated, pressured and firm, more controlled at first, then loosening up	More concrete representation of likes and desires for future, limited use of metaphor	• “I found lots of things that I like or want in my life, so I started with the Christmas drawing. I like the Christmas festival.” • “What I want is in the future to build a house of my own, and it should be in a, in an open place, like having – not, not into the city.” • “I would like to travel the world in my life, because after my MBA I want jobs not just specific to one place.” • “This is my family, my mom, my dad, my older brother, me, my younger sister. Actually, my dad has passed away. (...) But I always have this picture of this perfect family.” • “And the symbol of peace, (...) generally I don’t get into arguments. I try to resolve the arguments.”

Raul		Collage
Art elements	Content	Associations
• Full use of page, slightly weighted towards top • Full range of color • Some layering of imagery and words, with some elements partially obscured	Metaphorical representation of self-identity	• “I selected pictures that I thought I’d relate with, like some things I like, some things I don’t like.” • “I wanted to include colors in the collage (...) because I like colors in my life.” Image of cup and cigarette represent pride “that I have been (...) successful in” staying away from tea and cigarettes, because “my family has always encouraged me not to go towards smoking.” • “Most of all my family is my inspiration, because they inspire me, they encourage me to get to my goals.” • “I take things personally sometimes, so I keep telling myself, don’t be so sensitive. The first time I saw this I just included that into my collection.” • “Why did I choose lock? (...) Lock is like sometimes people tell me that – I don’t know if it’s true or not, again – but sometimes people tell me that I just not share some things with people that I should share.”



Figure 5. Hannah

Table 10

Hannah - Analysis of Artwork

Drawing		
Art elements	Content	Associations
• Full use of page, but elements isolated from each other • Full, appropriate use of color • Regular pressure • Steady, firm line quality	Concrete representation of self-identity	• “I drew a stethoscope, because I want to be a doctor.” • “Then I drew my family. My family’s all redheads, except for me. My brother is the tallest, I’m the shortest also, in my family, even though I’m the oldest.” • “And I’m a Capricorn, earth symbol. And then my birthday is Christmas Eve; Christmas is one of my favorite holidays.” • “My favorite season is fall. And again, I love nature, so I drew a butterfly.”

Hannah		Collage
Art elements	Content	Associations
• Expansive, balanced use of page, no white showing • Full range of color, but mostly dark backgrounds • Images carefully cut, layered and glued onto paper, with some elements obscured by others	Metaphorical representation of aspects of self-identity	• “I’m really messy, I love nature, my two addictions are chocolate and coffee.” • “I like this one because I love to bake, but I never really follow recipes and it drives like a lot of people crazy, and this one is like a recipe that’s been smudged, you can’t really see it anymore. I love food. Too much.” • “I thought that this was cool, because my best friend’s actually a Muslim, and I like how the Bible, the Torah and the Koran - because hopefully one day there can be some type of, you know, non-religious issue. I want to travel, so I put this one.” • “Everyone makes fun of me and calls me a grandma, because, I don’t know, sometimes I wear like a big grandma sweater, and also, like, [I’m] not typical college kid in a lot of ways.”



Figure 6. Sid

Table 11

Sid - Analysis of Artwork

Drawing		
Art elements	Content	Associations
• Full use of space • Full, appropriate use of color • Strong pressure • Line quality generally firm, but somewhat sketchy and broken in drawings of figures	Metaphorical representation of feeling state	• “That’s my house, I’m kind of locked away from it. That’s my mama, that’s my mother. That shows that I’m saturated, water is flowing out. So that’s river, that’s a tree, freedom, birds.” • “And amidst all the greenery, and rivers and beauty, I’m still kind of locked away, because I’m a long way from home.” • His limited drawing skills [referring to the two figures] “kind of occupy too much of my mental faculties, so I have to rub again and again, and still, I couldn’t really draw even a face quite right, so I kind of felt guilty of the fact that I am substandard in this.”

Sid		Collage
Art elements	Content	Associations
• Full, balanced use of space, slightly weighted to the bottom • Full range of color, with some darkness in backgrounds • Images carefully cut and organized on the page	Metaphorical representation of self-identity	• “When I was in school, I was, like, the front bencher, always raising hands on any question the teacher had, yeah, so I was good in studies.” • “So I’d love to have a family like this one day.” • “And, uh, sometimes I am sad, most of the time. I do be hopeful. This is light. I do believe that something definitely good is happening, even if I don’t know it’s happening, it is something good that’s happening.” • “I love being adventurous. It gives me a lot of adrenalin rush, which I like.” • “I can be very funny if I want. I love flowers, and I love to dance like crazy.”



Figure 7. Lia

Table 12

Lia - Analysis of Artwork

Drawing		
Art elements	Content	Associations
• Constricted use of space, image floating, surrounded by white area • Appropriate use of color • Strong pressure • Somewhat perseverative, agitated, broken line quality	Metaphorical representation of a state of mind	• “This is (...) my secondary problem. I am watching my weight right now.” • “I keep having this, how do I put it, perfectionism sort of thing about my body. (...) So I keep watching my thighs.” • “At the same time I like eating, so while I’m eating I feel guilty, sort of.” • Gash in stomach “sort of represents the guilt that I have when I’m eating something that I shouldn’t eat. And the thigh is that I keep exercising, but, exercising also is good for my body, but mainly it’s for the weight loss, so that sort of represents the effort that I put on my thigh.” • “The person also looks at the other person, so it’s like I keep comparing myself with other people.” • “I drew her naked because it’s about body, and I think when I draw a person naked, it feels more – it represents more vulnerability, so it feels more truthful.”

Lia		Collage
Art elements	Content	Associations
• Full, balanced use of space, with clear division between left and right sides • Full range of color • Images carefully trimmed and placed on page	Metaphorical representation of conflict	• “This is mainly about my – the conflict between what I want to do and what I am doing right now. I am majoring in business, and so far I thought that it would be the right choice, and that’s what my parents said, because it’s very practical and it is sort of a good decision to survive.” • “Mostly related to money and, like, being ahead of the game in the society (...). And that’s mostly related to reality (...) and that would also make my parents proud.” • “And in the center, this is like me, but at the same time I’m also leaning toward my interests.” • “Compared to the other side, it’s more about my dreams, and my passion.” • "This, like, it’s hidden, but it’s lightning, represents sort of like the conflict.” • “In the perspective of reality, satisfying all my needs is impossible.” • “It makes me feel like I can do what I want to do, because in the end we’re all going to die. So thinking about death makes me more leaning toward this side, like, the – my interest area.”



Figure 8. Bill

Table 13

Bill - Analysis of Artwork

Drawing		
Art elements	Content	Associations
• Sparse use of space, elements organized around a central opening • Restricted use of color • Very light pressure • Sketchy, feathered line quality	Concrete representation of a daily event	• “I tried to draw 33rd and Market stop, since it has become a part of my new life.” • “So this is the Athletic Center.” • “I like this green tree and later even more colored flower over here. I try to draw it from my memory.” • “And the Mario dragon, that is the best that I could draw. And I like these colors for 34th street station, especially this blue and yellow colors.” • “So I was just imagining myself while I was coming here. I was standing in this corner. I was having a good look at the places at the bus stop. I was here. There was time display, and this is a bad representation of myself.” • “And I do not usually see so many zebra [crossings] there in India, (...) but imagining, I thought of drawing them. I think I started with them, with this square and zebra crossings.”

Bill		Collage
Art elements	Content	Associations
• Full use of space, slightly weighted towards left • Full range of color • Images carefully trimmed and glued, with little layering	Metaphorical representation of self-identity	• “Here I tried to show my interest, mostly what I’m interested in. First of all I like travel, I like surprises, new things, mysteries and unexpected things.” • “That’s my own image for myself.” • “And I like swimming underwater, that is a good thing to be. Another adventure, so that’s, kind of one, one area.” • “As I said, I like something unexpected, I like this bilateral arrow, that is showing somewhere else and I’m going somewhere else.” • “Lightning is one of my fears, my primal fears. I like unknown things and, uh, it looks like something to me, maybe symbol of devil or something.” • “I have interest in religion and theology, pan-theology. I like this image. It should be on my collage. • “And finally, I – even though I like to visit new places and like unknown things, but I’m old-timey, I do not change my habits, I keep to myself, I like what I do. So I think my friends should consider me old-timey.”

Analysis of Art and Associations

An analysis of the art and of the participants' associations to it indicates that seven participants were able to use the approach to produce collages that revealed more than just concrete information about themselves, while one made a collage that was less revealing of emotional content. Four used drawing materials to express themselves through metaphor, and four made drawings that were concrete and elicited less revealing associations about themselves.

Tony, Emma, Sid and Lia each produced two pieces that were considered revealing.

In his collage, Tony (Figure 1, p. 81) depicted the way he dealt with feelings of loneliness in middle school and his growing identification with his online persona, while he used his drawing to describe the excitement of belonging to a group of people with similar interests.

Emma's (Figure 3, p. 85) associations to her collage covered many aspects of her personality, including likes and dislikes, habits, ways of dealing with certain situations, conflicts, and aspects of her personality. Her drawing reflects a feeling state she described as not living in the moment, but always looking at what might or should happen.

In his collage, Sid (Figure 6, p. 93) depicted aspects of his personality, positive and negative feeling states, future aspirations, and interests, which he described in detail. His drawing illustrates a specific negative feeling he was experiencing at the time the session took place.

Lia (Figure 7, p. 96) produced two pieces that she said illustrated her “problems.” In the collage, she shows the conflict between her parents’ ambitions for her, and her own, very different, interests, while her drawing illustrates a more personal issue relating to body image, which she discussed openly and in detail.

Raul, Hannah and Bill produced collages that were felt to be more metaphorical, and drawings that were more concrete.

Raul’s (Figure 4, p. 87) collage illustrates his likes and dislikes, aspects of his personality, certain beliefs, and the way he approaches certain situations. His drawing, however, is more limited to his likes and material things that he wants in the future, although he did refer briefly to a close personal loss.

In her collage, Hannah (Figure 5, p. 90) used the imagery to express beliefs, interests, and aspects of her temperament. In contrast, her drawing shows more concrete content, such as her astrological sign, the composition of her family, and the fact that she wants to be a doctor.

Bill (Figure 8, p. 99) depicted his interests, something of his fears, aspects of his personality, and likes and dislikes in his collage. His drawing is more concrete in that it illustrates a scene he sees every day. His associations to the drawing consisted of a description of what he had drawn. When rating his pre-drawing anxiety level, Bill said that it should have been a 2, and not a 3 as he first reported. His drawing, however, suggests that he may have been experiencing more anxiety than he was acknowledging: the line quality is faint and sketchy, his content is very concrete, there is minimal use of color, little effort seems to have been made to complete some of the elements, and he was critical of the way he represented himself in the drawing.

Both Joy's pieces (Figure 2, p. 83) were considered more concrete and less metaphorical. Her associations were less revealing of emotional content, consisting primarily of a list of her likes. While she reported a stable anxiety level of 0.5 throughout her session, her art suggests that she may have been experiencing more nervousness than her score would indicate. In her drawing, the line quality is pressured, broken and agitated. The content is very concrete. The image that she drew to represent herself is incomplete and fragmented, suggesting possible feelings of anxiety about how the researcher might be perceiving her. Furthermore, some of the elements in the drawing may suggest concern about being observed and a desire to be somewhere else - she drew an eye, a bird's eye view of a beach - an image she abandoned because "it didn't look well" -, a window, and birds in flight. The images in her collage are, for the most part, separate from one another, and have an isolated, isolating quality that may suggest an unconscious reluctance to open up to a stranger.

It should be noted that Joy was not American and not fluent in English. Her language limitations may have led her to be more guarded out of fear of being misunderstood. Another consideration is that opening up to a stranger might not have been culturally appropriate to her, and her more concrete associations were what would have been expected in an initial encounter.

Collage Influencing Composition of Drawing

Joy, Raul and Hannah (Figures 2, 4 and 5, pp. 83, 87 and 90) made drawings that reflected the way a collage is constructed: they drew disparate elements to represent different aspects of themselves, much as their collages were composed of different images and words glued on a piece of paper. Hannah (Figure 5, p. 90) expressed

dissatisfaction with the way her final drawing looked, saying that it was not “cohesive” and that although the image consisted of a group of images that represented her, it did not give “a full picture.”

Collage Allowing the Depiction of Strengths

For most participants, collage offered a way to say something about their likes and dislikes, good and negative feeling states, and strengths and weaknesses, which was an opportunity that drawing, because of concerns about artistic skills, unfamiliarity with using art materials, and fears of producing something inadequate, did not always seem to afford.

Major Themes from Anxiety Scales, Interviews and Art

Several major themes emerged when data from the anxiety scales, interviews and art were organized with respect to the research question. Collage was initially perceived as being easier and less intimidating than drawing, and evoked significantly less anxiety. The study suggested several reasons for this.

The visual stimuli of the imagery and the perceived ease of the process offered structure and control. The process also provided a reflective distance that allowed for safe exploration, insight and self-expression, giving participants the chance to look at themselves and their experience from different perspectives, as suggested by the images, yet allowing them to decide what to reveal. It also compelled participants to decide what was important and must be included in their final collage, what could be partially obscured when layering the images, and what could be discarded for the sake of space. It made it possible for participants to describe their strengths, since they could project these facets of themselves onto the images rather than rely on their own drawing skills. Seven

participants were able to use collage to produce more metaphorical pieces that revealed personal emotional content.

There were, however, limitations relating to the use of someone else's selection and someone else's images, because this involved working with another person's biases and focus. The collage-making process itself was also felt by some participants to be more mechanical and less personal.

Drawing, for its part, evoked significantly more anxiety than collage when the anxiety measures were analyzed. Half of the participants produced drawings that were considered more concrete and less revealing. Furthermore, six participants used mainly pencil, colored pencils or thin markers, which are considered to be at the more structured end of the materials spectrum, suggesting that there was a need for more control over the process. An analysis of the interviews showed that drawing was initially perceived to be intimidating because of the participants' preconceptions about what constituted a piece of acceptable art, concerns that meaning would be lost when translating an idea onto paper, fears that no ideas would be forthcoming when faced with the blank page, and because of the unfamiliar nature of the drawing materials.

A comparison of the two approaches, however, found that participants felt collage to be more restrictive in that it involved selecting imagery and words that described a person, whereas drawing offered more opportunity for freedom of expression because it involved creating imagery to depict the person.

CHAPTER 5: DISCUSSION

Overview

The purpose of this study was to compare the non-artist's experience of working with collage and with drawing materials, with the objective of gaining a richer understanding of the distinct characteristics of the two approaches and how these might exacerbate or address the anxiety that individuals new to art therapy often experience.

This study examined how eight volunteer student participants, ages 18-27, responded to the experience of making a piece of art about themselves using two popular art therapy approaches: collage and drawing. Their responses took the form of an anxiety/nervousness rating before and after each task, the option of choosing which approach to start with, two pieces of art, and an open-ended interview about their experience. The quantitative and qualitative data were first analyzed, aggregated and summarized, and then organized in terms of the research question.

The major theme that emerged when data from the anxiety scales, interviews and art were organized with respect to the research question was that the collage task evoked significantly less anxiety than drawing. Possible reasons for this lower level of anxiety, as elicited from this research, is that collage was perceived as being easier to do and was therefore less intimidating, since the process was straightforward and the imagery provided a starting point for the art making. Collage also allowed for the depiction of strengths, and it encouraged safe exploration, insight and self-expression. At the same time, there were limitations imposed by the collage-making process itself and by the need to use other people's images and selections. Drawing, for its part, evoked significantly more anxiety and resulted in fewer art pieces that were considered revealing and more

metaphorical, although participants ultimately felt that drawing allowed for more freedom of expression than collage.

In this chapter, participants' experiences with the two approaches will be discussed in the context of the literature reviewed for this study. To highlight the distinct nature of the two approaches, and their influence on anxiety, this part of the discussion has been organized into two sections, one covering collage and one covering drawing. Next, the clinical implications of the findings will be presented. Finally, the chapter will discuss the limitations and delimitations of the study, and implications for future research.

Collage

An analysis of the anxiety scales, the order in which participants chose to use the two approaches, the art, and the interview responses confirm a non-artist's perception of collage as being a less intimidating or challenging approach than drawing.

The anxiety scales showed that participants began the collage task with a statistically significant lower degree of anxiety or nervousness than they did the drawing portion of the study. Wadeson (2000) called collage "one of the least threatening activities" (p. 406) for clients new to art therapy, because it offers a starting point for the art process as well as containment. The drop in anxiety level was marginally significant pre- and post-collage, suggesting that collage may have helped address concerns participants had before starting this approach. Six of the eight participants chose to begin with collage, explaining later that this was because it seemed easier or less intimidating than drawing, while Bill began with the drawing because he wanted to leave the collage, which he was excited about working on, to last. Seven participants used collage to create

metaphors through which they were able to communicate aspects of their selves or their feeling states. In comparison, four participants used drawing in the same way, whereas four produced drawings that elicited more concrete, less revealing information. Johnston (2007) found that despite some expressions of concern by participants over time constraints and the art-making process, collage made it possible for her to gather a great deal of information in a short amount of time.

This study identified a group of characteristics that seem strongly linked to the non-threatening nature of collage as compared to drawing, and to its ability to address initial fears and anxiety. These characteristics have to do with the absence of preconceived notions about how a collage should look, and to the structure and control offered by the approach. Another reason that collage seemed to have evoked less anxiety is that it involves familiar objects – magazine images, glue, scissors – and the process was perceived as being easy. At the same time, it was not felt to provide full opportunities for self-expression, since it has inherent limitations relating to the fact that artists must select from images and words prepared and selected by others.

The findings of this study support and add to the clinical observations of therapists working with different populations, who have found collage to be an effective way to elicit personal material from individuals who are resistant, nervous, or having trouble focusing (Comfort, 1985; Landgarten, 1981; Linesch, 1988; Stallings, 2010; Takata, 2002; Vick, 1999; Wadeson, 1987; Wadeson, 2000). This study adds to our knowledge of collage as an art therapy approach, and to our awareness of how individuals new to therapy might respond to it.

In this section, the characteristics identified from the data are linked to the literature on anxiety, early engagement and art materials that was reviewed in Chapter 2.

Collage as Structured Medium

Based on participant responses, collage would seem to fall at the more structured end of the materials continuum, in that it offers the structure and control of visual stimuli, which provide a starting point for the art making. It also involves a reflective distance that may decrease affective involvement with the art process and product (Lusebrink, 1990). This control and distance can allow safe exploration of issues, insight, and self-expression in an initial session.

Provides a starting point. Clients new to art therapy who are not used to expressing themselves creatively, and who are already feeling the anxiety common to people just starting the therapy process, may experience heightened anxiety or nervousness when asked to use art materials. There are cultural expectations as to what constitutes art and the resulting fear of failing to produce something that will not be rejected (Sarason, 1990; Wadeson, 1987; Waller, 1993).

Seven participants voiced concerns relating to the blank page and the need to come up with an idea that would be deemed acceptable and would not be judged. Facing the blank page seemed to evoke fears of failing, of being judged, of producing something that could result in their being shamed or rejected, and of not being in control. All of these concerns are ones that research has shown must be addressed early in therapy in order to engage clients, build the therapeutic alliance, and provide for a better chance of a positive outcome (Hilsenroth & Cromer, 2007; Horvath et al., 2011; Horvath & Symonds, 1991; Martin et al., 2000; Restifo, 2010).

By providing a starting point for the art making, collage materials do not compel individuals to “fill” a blank piece of paper out of nothing, as Tony put it. Five participants reported feeling less anxious because the images and words gave them a starting point for the art making, suggesting that these stimuli helped address possible performance fears that people unused to making art can experience (Wadeson, 1987; Wittenberg, 1974). The marginally significant drop in anxiety levels during the collage portion of the study seems to support these findings, although further research focusing on this aspect of collage is needed to confirm just how influential this particular aspect of collage is in terms of decreased anxiety.

Provides reflective distance. Collage first involves the selection of images and words, and then their organization and assembly on the page. This process, mediated as it is by the steps involved and the physical act of cutting and pasting, seems to have provided a reflective distance that may have contributed to the lower degree of anxiety in participants, some of whom found it restrictive and repetitive, but only one of whom considered it stressful. Kagin & Lusebrink (1978) hypothesized that greater reflective distance decreases affective involvement in the art making process, thereby supporting defenses and engendering a feeling of more control over what is being revealed. The findings of this study indicate that for some participants, there was an intimacy to the act of drawing, a need to search within for ideas and ways to express their identity, that did not exist with collage, where the imagery had already been externalized by someone else, and all that was required was the selection of pictures to address the directive. Selecting the images seemed to involve a cognitive element that may have allayed some of their

anxiety, an element that may have been less evident with the drawing portion of the study.

Provides safe exploration and insight. Five participants reported that the collage imagery gave them the chance to think about themselves and their experiences from different, new perspectives. Some noted finding an image and immediately identifying with it. Raul, whose collage includes the phrase “don’t be so sensitive,” said that the minute he saw that phrase, he had to include it, because it was something that he was constantly telling himself. The externalized images and phrases, and the ability to decide which ones to use, appear to have given participants a safe way to investigate their own selves from various angles, and then reflect on their choices quite openly with the researcher. These findings reflect those of Johnston (2007), who described an initial free associative or unconscious thought process as images are being selected, and then the emergence of more coherent personal memories and reflections as the imagery is organized and assembled.

Provides safe self-expression. The reflective distance when creating a collage may also make this approach feel safer to individuals who are worried about trying to express feelings and thoughts without revealing too much too soon. Gabbard (2005) wrote that clients often fear changing the status quo, and are resistant to gaining insight into what is causing their symptoms. Brodsky (2011) noted that coerced and reluctant clients resent the intrusion of therapy. However, these feelings must be addressed if a therapeutic alliance is to be established. Hilsenroth and Cromer (2007) wrote that it is important to let clients take the lead and express themselves in ways that feel safe.

With collage, individuals can decide what imagery to pick and what to reveal about their selection, thereby allowing them to pace their revelations and not experience the therapy process as overly intrusive. Johnston (2007) found that the use of pre-fabricated, pre-selected images seems to give individuals something onto which they can project their thoughts and feelings, while at the same time allowing them to deny those projections because they were not responsible for the original images or for the selection. Seven participants seemed comfortable enough with the approach, despite being unfamiliar with it, to reveal something about themselves that was more personal. Even though this was not a clinical setting, it supports the observations of clinicians who use collage as a way to allow clients to communicate in a way that does not feel threatening to them, primarily in the early stages of therapy.

Mechanics of Collage-Making

Related to the structure of collage is the perceived ease of the collage-making process itself, which may also help address the fear of failure experienced by people just starting therapy (Restifo, 2010; Tantam, 2002; Wadeson, 1987; Whalen, 2004; Wittenberg, 1974). Collage requires individuals to go through a set of images that they had no part in creating, select those with which they identify or that appeal to them, cut them out, and glue them onto a piece of paper. Seven participants reported that collage seemed easier and less intimidating than drawing, and four specifically noted that collages are easy to make. Lia said that “all you have to do is organize it in a nice way, and I think pretty much everyone can do that. As long as you have the right image.”

Once participants had made the collage, however, they reported several limitations, including the fact that the process can feel too mechanical, the selection and

images are inherently biased, and there is a need for organization that some participants did not seem to anticipate.

Too mechanical. The description of the collage-making procedure by some participants made it sound mechanical and repetitive. Kagin & Lusebrink (1978) noted that mediated media (media that require the use of a tool, such as paintbrush or chisel) can provide a more cognitive experience. Collage is mediated by the need to select, cut and paste, which may increase feelings of control, yet at the same time may be a source of dissatisfaction, since some participants seemed to find it too “systematic,” as Hannah described it, and “less personal,” according to Emma. This perception may, however, have allayed some pre-art anxiety, since the straightforward nature of the approach seems to have made it easier for participants to achieve feelings of competency and mastery, as reflected in the absence of negative comments regarding their final collages. The decision-making and organization required also seems to reduce emotional involvement in the art making, making it feel safer to new clients.

Requires organization. Collage requires some degree of organization that some participants of this study did not seem to anticipate. Several commented on how they organized the images and glued them onto the page. Lia reported sorting the images into categories before deciding on a theme, which was ultimately determined by the imagery she found. Tony commented that the process might have been “easier” if he had organized the images while looking through the selection, because there were so many of them. Hannah said that she had found one important image by chance, after leafing through the selection a few times. Emma and Bill reported concerns about the distortion of meaning that might be caused by layering images over each other, and Emma also felt

more anxious after the collage task because she had had a hard time fitting everything she had selected onto the paper. Lia and Joy noted that they had been forced to work around images that they had glued down before deciding where everything would fit.

Moon (2010) wrote that organizing collage materials “can be symbolic of creating order out of fragmented aspects of life or chaotic feelings” (p. 18) and “engages the client in the actual layering of materials, and the symbolic layering of inner experiences, images, emotions, thoughts and ideas” (p. 18). The different responses in this study reflect the individual needs of the participants and their ability to focus, make decisions, organize and problem-solve. Emma, for example, seems to require a more specific directive if the goal is to address early anxiety by using collage. Tony, on the other hand, might benefit from a more limited range of imagery, or from a selection organized by category. Bill, for his part, would need a chance to explore the approach and experiment with layering images to become comfortable making a collage without fearing a distortion of meaning.

Commitment factor. A characteristic that has to be considered when selecting materials is that of commitment, or whether the mark, once made, can be undone (Wadeson, 1987). Gluing would appear to transform an image from a flexible projective tool into a resistive, overly structured medium for someone who begins committing images to the paper before knowing how everything will fit. Lia and Joy, the participants who reported dissatisfaction because they began gluing before organizing the images, attributed their actions to their lack of experience with the approach, suggesting that familiarity with it might have increased their feelings of control over it.

Importance of image selection. Most participants felt that the collage selection was satisfactory, with the exception of Emma and Tony. Emma felt overwhelmed by the number of choices that seemed to suit her, and Tony said he would have liked more non-human images. For their part, Hannah and Sid commented that a more limited selection might have evoked some anxiety.

Although generally satisfied with the imagery provided, participants commented on having to work with the bias of the person making the selection, and of the person who took the original photograph. Alderfer (2005) found that when participants were presented with a piece of paper onto which one image had already been glued, it evoked feelings of being overwhelmed and manipulated in some, who felt that the researcher was trying to point them in a specific, unknown direction. This sense of being led was experienced by some participants in the present study. Bill noted that the images “set you in one direction.” Raul, while he seemed to enjoy making the collage, said that the researcher had “forced” him to think about himself “inside the box.” Tony had to cut out the question mark he wanted, since none had been provided. The size of the selection, as well as content, is therefore an important consideration, a point emphasized by Landgarten (1993), who recommended having a broad range of images that have been checked by someone else for bias whenever possible.

Allows for Depiction of Strengths

Participants were able to use the imagery to depict strengths as well as more vulnerable aspects of themselves. This may also have influenced the marginally significant drop in anxiety level pre- and post-collage. MacKinnon et al. (2006) wrote that one way to reassure anxious new clients is by showing an interest in their strengths

and assets, and not just in the problems that brought them to therapy. For most participants in this study, collage offered a way for them to tell the researcher about their interests, good and negative feeling states, and strengths and weaknesses, which was an opportunity that drawing, because of concerns about artistic skills, unfamiliarity with using art materials, fears of producing something inadequate, and fears of revealing too much of themselves, did not always seem to afford. This raises the question of how participants would have responded to a directive that focused on a negative aspect of self. The directive in the study was very broad; a more specific theme focusing on a negative experience or aspect of self might have evoked greater anxiety, or more guarded responses, regardless of the approach.

No Aesthetic Preconceptions

The interviews with participants suggested that there do not seem to be the same concerns regarding artistic output with collage as there are with drawing. With the drawing task, participants directed their criticism towards their artistic abilities and product. They seemed comfortable enough with collage, however, to criticize the process itself, suggesting that they did not have the concerns about the aesthetic appeal of their collages that they had with the drawings. When comparing the two approaches, Sid said that the aesthetic expectations he had regarding his drawing did not exist when it came to the collage, because the images “were already there” and all he had to do was choose the ones that reflected how he was feeling. For Bill, collage does not have the “benchmarks” that drawing has, which meant that there is no right or wrong way to do it. Lia noted that collage only involves organizing the images in a “nice way,” which she felt anyone could do.

Landgarten (1981) wrote that collage lessens “superego demands on performance” since it calls for little technical skill, while Linesch (1988) found that when working with adolescents, the approach bypassed concerns about artistic abilities because it was both “controlled and controllable” (p. 77) and responded well to their defensive needs. Stallings (2010) also found that feelings of not being in control may be offset by the control a person wields over materials and process when making a collage. While no participant asked to keep their drawings, two did ask the researcher to email images of their collages, suggesting greater satisfaction with these than with the drawings. Future research could include asking participants to rate their drawings and collages and explain the reasons for their ratings, in order to investigate why collage does not seem to carry with it the same aesthetic burden as drawing.

Focuses Creativity

The availability of ready-made and selected imagery can also help focus an individual’s ideas. Some participants chose a theme based on the images available, some reported starting with collage in order to channel their ideas before moving into the drawing part, and some said that the collage had provided at least some ideas for the drawing task. Three of the six participants who started with collage made drawings that reflected the composition of a collage, consisting of different elements scattered over the page, suggesting the possibility that even when the content of the collage did not influence that of the drawing, the construction of it did. Further research is needed to examine how making a collage first might influence the nature of the drawing and vice versa, in terms of generating ideas, determining content, and influencing art elements such as use of space and use of color.

Moriarty (1973) found that when used with patients with schizophrenia, the collage materials gave patients “less opportunity to wander off into autistic productions” (p. 153). In the case of the high functioning, healthy participants in this study, the images helped them externalize aspects of themselves or of their experience, although in some cases these aspects and experiences were determined by the content of the selection. Comfort (1985) reported that asking clients to find images that illustrated their inner experiences gave them a way to identify the metaphor that seemed most appropriate to them. While six participants voiced concerns that their drawing might be misinterpreted, only two – Emma and Bill – reported fearing a distortion of meaning in their collages as a result of having to layer the images, suggesting that fears of being misunderstood, which were so prevailing with respect to the drawing portion of the study, are much less present with collage.

Allowing individuals to select their metaphor, rather than make them create it on the blank page using unfamiliar materials, seems to have addressed concerns about being misunderstood. Horvath et al. (2011), when discussing the need to build a therapeutic alliance to promote positive therapy outcome, underscored the importance of persuading clients to collaborate in their treatment, and of tailoring therapy tasks to meet the client’s capabilities. It would seem that asking them to find the metaphor for their experiences in a way that they feel is easy and non-threatening would satisfy these requirements in initial art-making ventures.

Not Culture Bound

Culture, ethnicity and gender did not seem to affect the participants’ ability to relate to the images and words and use them to make a piece of art about themselves.

Participants consisted of four men and four women of different backgrounds: three Americans, one Chinese, three Indians, and one Korean. Six identified as Asian, and two as Caucasian. Seven of the collages produced, taken together with the associations given by the artist, were felt to be revealing and to provide in-depth introductory information about the individual. Landgarten (1993) developed the Magazine Photo Collage assessment as a tool that was not culture bound, arguing that if the selection is sensitively made, the approach can be used with different cultures, to address different issues. In Japan, Takata (2002) found that collage helped promote communication and socialization in adolescents who had been feeling alienated from their peers and families and had high rates of absenteeism. Wadeson (1987) used it when working with Hispanic women with chronic mental health problems, noting that the act of leafing through magazines together, finding themes to discuss, and then cutting out and gluing the images are steps compatible with the Hispanic way of socializing. The findings of this study suggest that magazine images and words are familiar to individuals from different countries and cultures and that as long as the selection is thoughtfully assembled, most populations will find something with which they can identify.

Influence of Directive

Most participants reported having no problems with the directive, the range of images and words available, or with using collage to address the theme. For Emma, however, the directive to use collage to tell the researcher something about herself was so broad that she felt overwhelmed by the number of images she felt she could relate to, and she experienced some anxiety when trying to fit everything she had selected onto the page. She felt that a more specific directive, and a smaller collage selection, might have

helped prevent any increased nervousness. In her thesis study, Radl (2005) found that a directive that is not carefully chosen for the target population can evoke more anxiety than an unstructured task, regardless of the art approach being used. Some participants in Alderfer's (2005) study felt overwhelmed and manipulated by the single pre-selected image with which they had to work. Further research into the use of collage with people unused to making art could look at how participants respond to a narrower directive, and how they view a smaller selection of images and words. Research could also examine how different clinical populations respond to these variables.

Drawing

Participants started the drawing task with a significantly higher level of anxiety than they did the collage portion of the study. Even though the mean anxiety level for this portion of the study fell 48 percent once the drawing had been completed, the post-drawing mean was still higher than the post-collage mean. In addition, the drop in pre- and post-drawing anxiety ratings was not statistically significant, suggesting that nervousness about having to use drawing materials may still have been present even after the task had been completed.

In their interviews, participants attributed their higher levels of anxiety before the drawing task to concerns about their ability to express themselves adequately in a drawing, to concerns that their drawing might be misinterpreted as a result of their artistic limitations, to concerns that they might not have any ideas with which to fill the blank page, and to their unfamiliarity with the drawing materials. Joy and Sid, the two participants whose anxiety/nervousness ratings before the drawing were lower or the same as their post-drawing ratings, also expressed concerns regarding their final pieces

when discussing them in the interviews. Sid, for example, reported feeling relaxed and confident following the collage task, which would explain his lower anxiety rating before he started using the drawing materials. However, doing the drawing shook his confidence and brought out negative feelings, resulting in an increase in his anxiety level.

Four participants were able to use drawing metaphorically to depict a feeling state, in comparison to none in the collage portion, where the focus was on self-identity. This may have been because of the restrictive nature of working with ready-made images that participants identified.

Four made drawings that were concrete and elicited less revealing associations, suggesting that concerns about their final product may have persisted, or that uncomfortable feelings and associations may have been evoked by the directive. Another possibility is that for the six who began with the collage, material was brought up during that portion of the art making that may have consciously or unconsciously influenced how they approached the second task, which in their case was the drawing. Further research is needed to determine how one task or approach influences the next in a single session.

Aesthetic Preconceptions

Concerns about artistic abilities can pose a huge obstacle to the work of art therapists when the new client is not used to making art, especially with individuals who may already be resistant to the process because they find it childish (Sarason, 1990; Wadeson, 1987), are afraid of failing at it (Whalen, 2004; Wittenberg, 1974), fear being judged on the final product (Waller, 1993; Sarason, 1990; Wadeson, 1987), or because they are embarrassed about expressing their selves through art (Gladding & Newsome,

2003). Clients just starting out in any form of therapy often feel disempowered, embarrassed by what may be revealed about themselves, and afraid of change (Brodsky, 2011; MacKinnon et al., 2006; Restifo, 2010; Tantam, 2002). Drawing, to those unfamiliar with it, can intensify these feelings because the final product is a concrete object that clients may think can be judged, interpreted and rejected by the therapist, consequences often feared by clients just entering therapy who are having to discuss their problems with a stranger (Brodsky, 2011; MacKinnon et al., 2006; Sarason, 1990; Tantam, 2002; Wadeson, 1987). It is also an unfamiliar process that takes them out of their comfort zone. Lowenfeld and Brittain (1987) wrote that artistic development can end around the age of 12 unless individuals make a deliberate effort to continue their artistic development. Stone (1991) found that adults, especially those with higher levels of education, may have unrealistic expectations about their art creations because of assumptions about what constitutes “art”. Wadeson (1987) wrote that “performance fear” (p. 56) and “fear of self-revelation” (p. 56) can raise significant obstacles to creative self-expression in art therapy. As Bill described it, “drawing has some benchmarks. Collage [does] not have such benchmarks; anything you put can be ok, acceptable.”

Seven of the eight participants made some type of negative comment about their artistic skills when asked about their pre-drawing anxiety ratings. The only one who did not, Emma, at another point in the interview noted that she had been “worried” about making the drawing, which she described as a “more intimidating” venture because it involved having to come up with an idea about what to draw, and then drawing it. Bill, when talking about why his anxiety level had dropped post-drawing, said it was due to “finding out that you were not looking at my drawing.” He said that as he was doing the

drawing, he had been thinking that his piece would be compared to others and “judged with others on how good it will be.” Even Lia, who reported sometimes using drawing to work through problems, seemed worried about how her art piece would be received, since she said that “if I had more time, then I think I could have done a lot better.” A more structured directive may have helped address some of these concerns, because it would have helped focus the art making.

Participants’ perceptions of their drawing skills influenced the content of some of the drawings. Sarason (1990), who wrote that people in Western society tend to stop engaging in the arts due to cultural expectations about what constitutes art and their own dissatisfaction with their ability to portray reality, called the inhibition of creative expression in Western adults a “form of learned helplessness” (p. 5). In this study, participants came from a range of backgrounds, indicating that these expectations are not restricted to Western society. Joy, who is Chinese, abandoned a section of her drawing because it did not look right. Sid, from India, became anxious about the way the figures of self and mother were developing on the page. Bill, who is also from India, talked about how existing “benchmarks” about art make it “easier to differentiate” an ugly drawing from an acceptable one.

Fear of Being Misunderstood

Six participants attributed some of the anxiety in relation to the drawing to concerns that their piece might be misinterpreted, or meaning lost, as a result of inadequate drawing skills. These concerns seem to reflect new clients’ fears of failing, of producing something inadequate, or of being misunderstood (Restifo, 2010; Tantam, 2002; Wadeson, 1987; Whalen, 2004; Wittenberg, 1974). Some participants viewed the

ability to give their associations favorably, and seemed reassured by having the chance to do so. Tony and Joy, however, said that being able to explain their drawings did not allay their concerns about their final piece. Despite the fact that participants knew that they would have a chance to explain their drawings, some still labeled elements as a way of clarifying the content, suggesting that concerns about being misunderstood can be very powerful and need to be taken into consideration when asking someone new to art therapy to communicate with unfamiliar materials.

Anxiety When Facing a Blank Page

Seven participants also voiced concerns about not coming up with any ideas for the drawing. This seems to reflect fears of failing (Restifo, 2010; Tantam, 2002; Wadeson, 1987; Whalen, 2004; Wittenberg, 1974), of unintentionally producing something potentially embarrassing (Waller, 1993; Wadeson, 1987), of not being in control (Tantam, 2002), and of being a disappointment (Restifo, 2010; Wittenberg, 1974). Tantam (2002) wrote that therapy can be “disempowering” (p. 23). Several participants reported preferring collage because of the visual stimulus and structure that it provided. Some reported using it as a stimulus for the drawing. Three made drawings whose composition reflected that of a collage, consisting of different elements scattered over the page.

Using Unfamiliar Materials

Unfamiliarity with the drawing materials led most participants to use pencil, colored pencils or felt tip pens. These are materials that may be more familiar to people who have not done any drawing since middle and high school, and that are also considered to be at the more structured end of the materials continuum and therefore

more controllable (Kagin & Lusebrink, 1978; Moon, 2010). The choice of more structured materials suggests that participants may have been trying to offset any discomfort by retaining as much control over the process as possible. Despite being given the option of more controllable materials, four participants still produced drawings that were considered to be less revealing of emotional content, although the other four were able to depict emotions. In therapy, new clients may feel a loss of control because they perceive their treatment to be in the hands of an all-knowing therapist with the power to reject and shame them (Tantam, 2002). Structured materials help bolster defenses in new clients who are afraid of revealing too much of themselves too quickly (Wadeson, 1987). Findings of this study suggest that even high-functioning university students may need the reassurance of more structure when setting off on an unfamiliar, uncomfortable venture.

More Opportunity for Free Expression

When comparing the two art experiences, a theme that emerged was that self-expression was easier with collage, but ultimately, drawing offered more chances for free expression. It can be more personal, it is not bound by anything except the size of the paper, it promotes creative thinking, and it provides a kinesthetic as well as visual experience. Depending on the situation, clinicians often prefer not to offer specific materials or directives to clients because a great deal of information can be gathered by allowing them to make their own choices (McNeilly, 1983; Wadeson, 1987). Amabile and Gitomer (1984) found that more freedom to select materials resulted in greater creativity in children. Despite participants' assertions that drawing offers more opportunity to express oneself freely, however, it would seem that in initial art-making

sessions, aesthetic concerns and anxiety about the unfamiliar may still repress self-expression, since four participants produced concrete drawings, in comparison to only one who made a concrete collage.

Drawing is more personal. Several participants said that if they had the choice, they would still prefer to use collage to describe themselves because it is easier and less challenging than drawing. However, almost all participants said that drawing would ultimately allow individuals to express themselves more freely, because there would be no restricting factors such as someone else's selection or someone else's images. Their responses suggest that once the performance anxiety often experienced by new therapy clients (Restifo, 2010; Wadeson, 1987) has been addressed, they would be willing to use the more unfamiliar drawing materials to express themselves. Hilsenroth and Cromer (2007) wrote that therapists have to show clients that they are interested in understanding their experience. In art therapy, "the meta-message would be 'I'm interested in your picture' rather than 'This picture is better than last week's.'" (Wadeson, 1987, p. 57). According to Sid, drawing, despite the anxiety it causes, "kind of makes you stand up for who you are, and it makes you kind of accept the way you are." This belief that drawing can ultimately provide a more personal experience suggests that even when using collage with a new client, the art therapist will be able to start incorporating other less structured and more affective materials early on to complement the creative process, in efforts to give clients more opportunity to say what they really want.

In this study, the collages produced were, for the most part, like lists: showing interests, dislikes, positive feeling states, negative feeling states, etc. The two exceptions were Tony's, which showed a single feeling state and evoked detailed associations about

it, and Lia's, which depicted a conflict. In comparison, four drawings described a single feeling state and evoked in-depth associations, whereas only four reflected the list-like construction of a collage. This may have been because of the directive, but it may also be because a collage, since it is an assortment of images and words, suggests something more like a list, whereas a drawing compels the artist to look within for material and find a theme that will provide the framework for the art.

Drawing is not bound by rules or structure. Some participants commented on the absence of rules in drawing, which allows different perspectives – literally and figuratively – to be shown on the same page. With collage, in comparison, what can be shown is delimited by the imagery available. One of the advantages to art therapy defined by Wadeson (1980) is that there are no rules of logic in art, and that many relationships, feelings, perspectives, time periods, etc. can be depicted in the same “spatial matrix” (p. 11). Bill, who rotated his page as he was drawing, felt that “drawings are difficult, but you are free to do whatever you want.”

Drawing encourages creative thinking. Sarason (1990) believed that the desire to express oneself artistically is never fully extinguished, despite the dissatisfaction that results from cultural ideals of what constitutes art. This desire seemed evident in some of the drawings, in which participants resorted to creative solutions to try to offset the limitations imposed by their unfamiliarity with the materials and their drawing skills. They also used expressions such as “think outside the box” and “go in any direction” to describe their drawing experience.

Despite their perceived limitations, four participants were able to use drawing materials to depict a feeling state. Tony used “symbol shapes” to compensate for his lack

of skills. Lia drew a figure with cuts on her body to symbolize her feelings about her eating habits, and also noted that as she drew, she had been organizing her thoughts. Emma produced a landscape divided by a river to symbolize her habit of always looking at where she wants to be or feels she should be, and not at the here and now. Sid was able to use pencil and colors to reflect his feeling of being “locked away” from his family back home in India. Although he was dissatisfied with the way the figures turned out, and said he had only been able to draw trees because of the presence of green and brown crayons, his image eloquently reflects what he was trying to show. Bill drew a concrete representation of what he sees every day from the university bus stop, but in doing so rotated the page to show the four street corners to compensate for his inability to draw a complex image with perspective and overlapping shapes. While participants were more focused on the shortcomings of their drawings, and not on their own creative problem-solving, the latter could be identified, examined and underscored in an art therapy session.

Drawing provides sensory experience. Emma and Hannah noted the kinesthetic feeling of using drawing materials, whereas there were no comments regarding any sensory element in collage except its mechanical nature. Emma also found that drawing provided a feeling of “emotional release” that she had not been expecting. While both women commented on the kinesthetic aspect of drawing, their approaches were very different. Emma chose colored pencils because she was afraid of “messing up,” and her drawing is a concrete representation of herself. Emma used oil pastels with great intensity to show a state of mind. One thing research has shown is that people respond differently to the same art materials, and that generalizing findings is difficult.

Schlossberg (1983) described how two individuals with different diagnoses had radically opposing responses to clay. Level of regression and disorganization also plays a role (Hartwich & Brandecker, 1997; Kagin & Lusebrink, 1978). Moon (2010) underscored the importance of cultural, social and political associations to materials, noting that their connotations will vary according to group and individual. The art therapist must understand all these factors if he or she is to make an informed choice about when to use which approach to encourage a more sensory, affective experience, for example, or to address anxiety and bolster defenses.

Clinical Implications

Collage Selection

The selection presented to participants consisted of 320 images and 150 words or phrases. About half the images depicted nature, animals, objects and food, and the other half depicted people, either individually, in dyads or in groups. There were family groupings, different types of couples in different attitudes, women and men engaged in sports activities, work and relaxation, and as wide a range of facial expressions and ethnicities as possible. The words consisted primarily of phrases taken out of context, verbs and adjectives. The images were not separated into categories, but presented on sheets of paper that had been photocopied so that each person started off with the same content.

Preparing the selection took many hours, and was much more complex than expected, possibly due to the researcher's inexperience with what might work as adequate projective tools. Soon after starting the selection, this researcher realized that she was focusing on images from the usual women's magazines, which cover a limited,

unrealistic socioeconomic spectrum and ignores men and anyone over the age of 35. It took an effort to find unglamorous, realistic pictures, reinforcing Landgarten's (1993) recommendation that the selection be thoughtfully assembled to include situations in which real people might find themselves.

Another question that arose was whether the selection also depended on the researcher's mood at the time it was being made. It occurred to her that maybe she would have been better off buying an identical selection of magazines for each participant, rather than trying to pre-select the images, since every time she went through the same magazines, she found different pictures that she felt would be appropriate.

The images and words were all neatly trimmed for uniformity's sake, which she felt was not inappropriate since the goal of the study was not to see the process of collage-making as part of a therapy session, but to understand how the participants perceived the task. However, when using preselected images with clients in therapy, careful trimming would deprive the therapist of information such as their level of organization, engagement in the task, and decision-making process.

Following Landgarten's (1993) suggestion, three peers were recruited to vet the selection for possible bias. They were told the directive and the prospective population. The general consensus was that there had to be more images that were ambiguous in both the people and the objects/miscellaneous sections. With their suggestions in mind, the selection was reviewed and appropriate changes made.

Based on their reactions, the researcher considered how a therapist, when preselecting images, could easily send an unconscious message to the client about what was expected in the artwork, and about what she (the therapist) might find unacceptable.

Alderfer's (2005) study explored how individuals perceive what is being communicated by the art therapist when presented with varying levels of structure, and found that individual responses can include feelings of being manipulated, challenged and restricted. For example, while this researcher was actively looking for images of gay and lesbian couples, they were difficult to find, so the initial selection had just a few ambiguous images of dyads. To a gay client who is confronted with many images of heterosexual couples and only a few questionable ones of same-sex dyads, this might be perceived, unconsciously or consciously, as an indication of disapproval of that lifestyle. The therapist has to be careful, then, to balance selections. While not always practical, having others review the images for bias seems to be an important step in ensuring this balance.

One peer noted that there is already a bias in what magazines choose to publish, and in what photographers themselves decide to photograph. The way they frame a scene is also a form of bias. It was interesting, therefore, that some participants, when discussing the limitations of working with collage, referred to this bias on the part of the photographer, and reported having trimmed certain images to transform the focus to suit their own needs.

Clinical Applications

The importance of addressing a new client's anxiety in art therapy relates to the need to engage them early in the therapy process and promote a positive client-therapist alliance, since studies have shown a consistent association between an early alliance and positive outcome (Gomes-Schwartz, 1978; Horvath, 2001; Horvath et al., 2011; Horvath & Symonds, 1991; Martin et al., 2000). In art therapy, nervousness about having to make art can exacerbate these feelings of anxiety (Sarason, 1990; Waller, 1993; Wadeson,

1987), which can increase resistance to the therapy process and lead to a reluctance to collaborate and the possible alienation of the client.

Sarason (1990) wrote that humans never completely lose the desire to express themselves in creative ways. By knowing their materials, art therapists can make informed decisions about what approach to use and how much structure to provide in the directive, so as to address the anxiety that clients new to art therapy might experience and to encourage creative self-expression. Two common approaches in art therapy are drawing and collage. A deeper understanding of how individuals might respond to these approaches, and what qualities each one possesses, can help art therapists make that informed choice.

The findings in this study indicate that being asked to make a drawing evokes significantly more anxiety than being asked to make a collage. Clients new to therapy, even those who are willing to undergo the process, often fear being judged, shamed and possibly rejected by the therapist. Therapy involves exposure; exposure means making oneself vulnerable. This exposure can be symbolically represented by the artwork, which becomes a concrete object that embodies an individual's vulnerabilities. One reason is that the idea of what constitutes good and bad art seems to be deeply ingrained, regardless of cultural background. Even before a pencil is put to paper, someone who is not used to making art may be worrying about how it will look, especially to the therapist, who is a stranger and who may be perceived as being all-knowing and powerful, and therefore capable of judging and rejecting (Tantam, 2002; Wadeson, 1987). Another reason is that because of artistic limitations, some individuals fear that the product might be misinterpreted. Misinterpretation can lead to misunderstanding and

distortion, which can increase feelings of anxiety in someone who is trying to tell their story to a stranger. There are also concerns of drawing a blank (excuse the pun) when faced with an empty page and of failing in the treatment process. Finally, unfamiliarity with the drawing materials themselves can hamper self-expression. People unaccustomed to being creative with art materials may feel that drawing will only highlight their weaknesses and not allow them to say anything about their strengths, putting them at a disadvantage with the therapist and taking away any feeling of control within the therapy process. In addition, artistic limitations may influence what is communicated, since they will restrict an individual's ability to translate an idea into imagery.

Manipulation of anxiety. Based on the findings, it would appear that in a clinical setting, drawing may be more effective than collage for evoking anxiety to encourage regression. Experienced clinicians know that asking clients to draw can be anxiety-inducing. However, in some cases, such as when the client is too defended, is using the same materials to produce the same thing over and over again, or is ready to work through an important issue, using an approach that raises the anxiety level can be therapeutically powerful. It can break through defenses, help the individual face the challenge of not being in complete control, and encourage therapeutic regression to elicit unconscious material that can then be safely examined with the therapist.

Collage, for its part, could be used to “soothe” the client when the anxiety level is too high, to use Sid's word. It is an approach that seems to address many of the concerns found in individuals new to art therapy. Ready-made images and words are provided onto which clients can project meaning of their own, while at the same time denying

responsibility for the final appearance because they were not the creators of the pictures, or writers of the words, in the first place. The imagery can help focus the art making and suggest ideas about what to depict. When individuals find a picture to which they can relate, it is because that image seems to say clearly what they are trying to express, thus addressing fears of being misinterpreted. The images are universally understood for what they are by everyone looking at them, even if the projections vary from person to person. Collage is a simple process that most people can relate to, since almost everyone has, at some point, used scissors and glue in some way. Because clients have to think about the images and words and make decisions about which to use, collage-making provides a distance from the art product that may help strengthen the sense of control over what is being revealed.

Collage as bridge to creativity. With populations that are more self-conscious about their product, such as adolescents who are defensive and self-critical, and adults who have preconceived ideas about art, collage would appear to be an effective way to introduce the idea of using creativity to communicate, while at the same time bypassing the self-awareness, fear of being judged or shamed, and fear of revealing too much of oneself too soon that can plague some art therapy clients. Most participants in this study felt that drawing permitted greater freedom of expression, yet were self-conscious about their pieces, half of which were concrete and less revealing of emotional content.

Collage involves images and words taken from magazines, a familiar source of imagery and information. Magazines are virtually ubiquitous, and can be found everywhere, from doctors' offices and checkout lines to people's homes. Using such a familiar element to make a collage can form a bridge between the familiar and the

personal, and between the familiar and the art product. Joy, Hannah and Raul, for example, made drawings that reflected the way a collage is constructed: different aspects of themselves drawn on the paper.

Most participants attributed their anxiety levels pre-drawing to concerns about their ability to use drawing materials to express themselves. Only Tony and Hannah, however, reported that their anxiety had lessened post-drawing out of relief that the task had been completed. Bill and Raul noted that the experience had not been as bad as they had expected, Emma ascribed her drop in anxiety level to the feeling of emotional release that the exercise had provided, and Lia said that the exercise had helped her clarify and organize her thoughts, which helped decrease her nervousness. When comparing the two approaches, the consensus was that while drawing is more intimidating and anxiety-evoking, it allows for more freedom of expression.

These findings suggest that familiarity may be a significant factor when addressing the anxiety caused by being asked to make a drawing. Moon (2010) discussed the influence that familiarity with a specific art material might have on the way it is perceived by the individual. Stone (1991) found that with older adults, more experience with art making resulted in a more positive attitude towards the art produced. It would appear that given a chance to gain more experience and familiarity with drawing materials, the participants of this study might approach them with less anxiety over time.

The ease with which participants addressed the collage task reinforces its potential as a bridging approach. In a clinical setting, art therapists might consider having clients begin with collage, and then gradually introduce less comfortable materials once clients have become familiar with the collage approach and aware of, and possibly

dissatisfied with, its limitations. Participants seemed to feel that there were things they could say with drawing materials that they could not with collage. In a very short-term setting such as a hospital ward, the researcher suggests a two-part directive that first asks clients to select a few images and words that represent aspects of themselves and glue them onto the page, and then asks them to use drawing materials to depict facets of themselves that they could not with the magazine images.

Collage for early engagement. Collage seems to be an effective approach in the early stages of therapy for encouraging clients to use art to introduce themselves, helping to generate ideas, addressing aesthetic preoccupations, engaging them in the art making, and encouraging them to think about what they feel they really want to say. It has the ability to focus the individual by encouraging them to look for images that relate to what they are experiencing. In a situation where a task or material has evoked too much anxiety, collage can help by giving individuals permission to look outwards, so to speak. In the process, because the images act as projective tools, it can still yield a wealth of information.

Collage also seems to reflect some of the factors that researchers have found contribute to the establishment of the early positive alliance that has been linked to favorable therapy outcome. It can give clients the chance to explore their experience at a pace that they will not find stressful. It shows attunement with their fears and needs by allowing them to find the metaphor that best describes their experience. It lets them show their strengths, and not just the weaknesses some feel are displayed against their will in drawings because of their limited artistic abilities. Being given a medium that allows them to depict strengths as well as less positive aspects reflects the therapist's interest in

them as a whole person, and not as someone who is identified by their problem. It may reinforce the therapist's assertions that she/he is not there to judge them, since there seem to be much less rigid aesthetic expectations when it comes to collage. This allows individuals to express their emotions safely, without concerns that the final product will be judged.

Drawing for expression of emotions. Once initial fears have been addressed and a relationship established, findings indicate that drawing would be more effective in eliciting emotional content and allowing for the identification and working through of feelings. While participants reported being more comfortable with collage because of its non-threatening nature, the consensus was that it was a more restrictive approach than drawing in that it did not provide the same opportunity to express oneself freely. Already, in this single session with the unknown researcher, four of the drawings depicted feeling states, while only one collage referred to an emotional conflict. Because of its lack of rules, and the need to look inwards to come up with the appropriate imagery, drawing seems generally more effective for the expression of feelings.

Collage to promote feelings of mastery. While the art products themselves were not rated and most participants chose not to keep their pieces, Sid and Bill asked for photos of their collages to be emailed to them. This suggests that collage may encourage greater feelings of mastery and competence than drawing in individuals who are not accustomed to making art, a possibility that also seemed to be reflected in the fact that most participants did not criticize their collages, whereas they commented, sometimes very negatively, on their drawings. Because this was research, this author was only recording the participants' descriptions of their experiences. In therapy, the art therapist

and client would discuss the client's responses, negative and positive, to the creative experience and the content of the art, in order to identify and understand anxieties and concerns and to enhance communication and understanding. However, when clients' needs include bolstering self-esteem and promoting feelings of achievement, a simple collage directive would seem to be appropriate. A group directive developed by art therapist Linda Conley Soffer involves first drawing the outline of a bare tree on a sheet of mural paper, and then asking members to "fill it with whatever it needs to come to life" using different types of collage materials. This broad directive gives group members control over decisions about what is life-giving, and also allows for imaginative choices of images and textures that together can create a colorful, playful symbol of the group.

Control and containment with collage and with drawing. The collage-making process is perceived as being low risk, since it involves selecting, cutting and pasting images, and the creator can decide how much of him or herself to reveal when talking about the final piece. The low risk factor could ensure the control needed by many clients who are new to therapy, addressing the fear of self-revelation and fear of failing that can be barriers in the early stages. It allows the individual to pace what is being disclosed, and to take the lead in the therapy process. At the same time, the collage produced can provide a rich source of introductory information to the art therapist in the images selected, the way the individual undertook the collage-making process itself, and in the degree of openness and psychological depth of the associations. In comparison, making a drawing can feel riskier, in that it is often perceived as demanding skill and

mastery of the materials, and requires that the creator come up with an idea about what to depict in the first place.

In a clinical situation, art therapists could use collage by asking clients to select images that appeal to them and glue them onto the page in any way they like, and then talk about their choices. This will encourage the clients to focus externally on the images, as well as work on their problem-solving and decision-making skills, since they have to decide how to create a composition that is meaningful to them. Since expectations about what a collage should look like seem to be substantially less defined than with drawing, the piece is more likely to be pleasing and offer a sense of achievement, as well as providing introductory information for the art therapist. In the four-part Magazine Photo Collage assessment designed by Landgarten (1993), the first task is to “pick pictures that catch your attention” (p.9), glue them onto the page, and then “write directly onto the page, or tell me, anything that comes to your mind about each picture” (p. 9). Landgarten (1993) designed this task as a way of introducing clients to collage and to show how easy it is to master.

To address the anxiety that can be caused by being asked to use drawing materials, art therapists could give clients step-by-step directives that encourage them to use these materials to create symbols and colors that say something about themselves or their experiences. Dr. Betty Hartzell (personal communication, March 14, 2012) has developed a directive in which clients are asked to draw a symbol to represent themselves and place it in the center of the page. The next step is to draw symbols representing the people who give their life meaning and place them around the self-symbol, then to draw symbols to represent the things and activities that are important to them, and finally,

symbols that stand for something important that the art therapist has not named. This detailed structure, and the use of symbols rather than realistic representations, bypasses the need to come up with ideas, does not require artistic skill, and is a way for individuals to become used to using drawing materials in creative, non-representational ways. In addition, the resulting piece is concrete, and may feel safer to the artist than a typical drawing because it does not reveal too much of themselves all at once.

Collage for depicting strengths. Collage also allows individuals who are not skilled with art materials to illustrate strengths as well as more ambiguous facets of themselves, whereas drawing seems to underscore their weaknesses because of possible expectations regarding how the final product should look. It should be noted that this may not be the case with individuals who have some drawing skills, as was the case with Lia. Being able to show different facets of their selves can address fears of being shamed or embarrassed by the problem that brought them to therapy, or of being misunderstood. Positive and negative aspects can easily be depicted side-by-side, so that the client feels that the therapist is getting a well-rounded portrait of their experience, rather than one that just focuses on the problem. With new clients, or clients in short-term settings such as inpatient psychiatric units, this researcher suggests a directive that asks them to “make a self-portrait using magazine images and words.” Being asked to depict all sides of who they are underscores the therapist’s interest in them as a whole person, and not as someone viewed solely through the prism of their illness. Another directive suggested by this researcher would be to have them select an image that represents one problem that brought them to treatment and glue it in the middle of the page. Then they could find

pictures to represent ways that they have dealt with that problem in the past, as a way to highlight their coping skills.

Reflective distance, insight and universality. The reflective distance provided by collage can promote insight, in that the associations triggered by the different imagery encourage individuals to think about themselves in different ways, until they find images with which they specifically identify. Seeing their experience or problem from an alternative viewpoint, one that is externalized in a published image, may also offer a reassurance as to the universality of their emotions, since it can suggest that they are not alone in those feelings.

Collage could be used to emphasize this sense of universality in individual therapy. The researcher feels that one way of achieving this would be by asking clients to think about how they are feeling, and then to pick images of individuals that seem to illustrate those emotions. The final piece becomes objective evidence that such feelings are not unique to the client. However, while the universality factor can be important, the richness of the person's uniqueness could be undermined, so timing the use of this type of directive is important.

Some participants used the same images and words in their pieces. In a group setting, the use of the same imagery by more than one person could also be used to reinforce the therapeutic factor of universality, with members being able to see a concrete representation that others share similar feelings or problems. In addition, deliberately using an image chosen by another member in one's own piece may reflect the factors of imitative behavior and altruism. In imitative behavior, individuals have a chance to try out new ways of viewing their experience by looking at it through the eyes of other

members, in this case by deliberately using imagery that someone else has said that they identify with. The factor of altruism may come into play when one member sees that another one has deliberately chosen the same picture or word. This action may boost the first member's feelings of self-worth, since it is evidence that his or her choice was of value to someone else. Altruism is also being shown by the person imitating the other, since choosing the same image can be a way of offering support for what the other is feeling.

Playful aspect of collage. Some of the participants seemed to find the images themselves appealing. Joy and Lia asked to take home some that had attracted their attention but had not been used in their pieces. Bill, early in the session, reported being “excited” by the idea of making a collage, while Sid said that the images “attracted my attention” and collage “seemed interesting.” Shepard and Guenette (2010) felt that there was a playful, liberating quality to collage that it seems drawing, for most participants in the present study, did not have. The researcher feels that clients could be asked to play with the images by finding ones that represent feelings, objects or situations over which they have no control. Once they have selected a few, they could be directed to cut them up in ways that make them less powerful before gluing them onto the page. The physical manipulation of images and words representing feelings, situations or objects that individuals feel control them could introduce new ways to look at and address those elements of their lives.

Playful aspect of drawing. Findings of this study suggest that there is also a playful, liberating side to drawing. Some participants were able to use the drawing materials in creative ways by rotating the page, showing past, present and future at the

same time, or using symbols to represent elements that they were unable to draw. But because of preconceived notions about what constitutes an art product, these characteristics were not always seen by these participants as expressing creativity. It nevertheless suggests that once concerns about what the product should look like, what to draw, and how to use unfamiliar materials have been addressed, this creative aspect can be fostered and used to engage individuals in the therapy process and in using different materials for communication and self-expression. Furthermore, the transformation of a once intimidating approach into one that can be a source of fun and experimentation would provide true feelings of mastery.

One way to encourage this playful, liberating way of using drawing materials and bypass worries about drawing something realistic would be to show clients reproductions of abstract art, and suggest that they use drawing materials to make colors, shapes and lines that express how they felt during a stressful time in their life.

Collage as a multicultural tool. The findings of this study support the use of collage with individuals of different cultural backgrounds (Landgarten, 1993). The participants consisted of eight Drexel University graduate and undergraduate students with no formal artistic experience. While the sample was small and the age range more limited than desired, the diversity of backgrounds and the equal number of men and women offered an opportunity to take into consideration the effectiveness of the two approaches from a multicultural perspective. In terms of drawing, two non-Americans (one Indian male and one Korean female), and two Americans (one Asian female and one Caucasian male), produced pieces that were felt to be revealing. The other four participants produced concrete drawings and provided more concrete associations. It

would appear from this response that ideas of what constitutes art may vary according to culture, but exist nonetheless. Participant responses indicate that concerns about creating from within and having to communicate with unfamiliar materials are not limited to Western clients.

Organization in collage. Collage requires an element of organization that some clients might find challenging. For some participants, there was the sense of being overwhelmed by the number of images and by the restrictions imposed by images once they had been glued or committed to the paper. Familiarity with the approach would probably address these issues, but in an initial art-making session using collage with certain populations, such as groups that need more containment, the therapist might want to present a more limited number of images, or group them into boxes with categories such as “people,” “nature,” “things,” “animals.” However, this researcher has used collage with low-functioning patients in psychiatric, school and medical settings, and has found few expressions of frustration regarding organization or image selection. One difference that may be significant is that in clinical settings, clients are often presented with magazines, and not with a collection of isolated images. Leafing through magazines may offer an opportunity to gather one’s thoughts and settle into the process, while being presented with a wide range of isolated images, out of any sort of context, might be bewildering to patients who are less organized, or who are having trouble concentrating because of the effects of their medication or of their illness.

Tailoring the size of the selection, and how it is presented – whether as multiple loose images, in magazines, or grouped by category -, seem to be important considerations when working with different clients and populations. This may pose a

challenge to busy art therapists, who are caught between spending large amounts of time tearing or cutting out images on the one hand, and running the risk of giving clients magazines, only to watch them start reading the articles instead of selecting images, on the other.

Limitations and Delimitations of the Study

The limitations of this study include the small sample size, the high functioning nature of the student volunteers, and problems with communication, given that most participants did not speak fluent colloquial English.

The sample consisted of eight university students aged 18-27. While the fact that they were high-functioning allowed them to express themselves very eloquently for the most part, despite language limitations, it makes it difficult to generalize the findings to a clinical situation. The restricted age range also makes it difficult to generalize findings to younger or older groups.

For most participants, English was not the language in which they were most fluent. As a result, there were some misunderstandings regarding the meaning of words. Bill thought that “anxiety” meant “excited” in a positive sense, leading to some confusion when asked to rate his anxiety level. Joy seemed to have a hard time expressing herself, and her responses were very concrete, which may have been partly due to concerns about saying the wrong thing and being misunderstood, but also may have been culturally appropriate.

Delimitations included the data collection procedure, which began with the gathering of demographic information and a brief discussion about each participant’s past art experience. Sid reported that this conversation helped put him at ease, suggesting that

it could also have influenced the anxiety level of other participants in some way. Future studies could leave this part of the data collection session to the end, so that it does not influence the anxiety ratings.

Another delimitation relates to the interviewer's inexperience as both therapist and researcher. At times the line between researcher and therapist came close to being blurred, and excessive caution not to act as a therapist may have resulted in reluctance to ask more probing questions about the art that might have provided more information about the experience. While the interaction when investigating the participants' past art experience may have helped create a feeling of safety, the researcher's own initial nervousness at the start of the data collection process may have contributed to the atmosphere in the room in a less comforting way. At the beginning, especially with Tony and Joy, the first two participants, the researcher was worried about making sure that the sessions went according to the methodology established. Despite these concerns, she was not initially consistent about asking participants to rate their anxiety levels before they gave their associations to the art. The act of processing may have helped bolster their defenses, since talking about their art may have led to their intellectualizing the content of their pieces. Over time, her nervousness waned as she became more comfortable with the procedure, and more consistent about it.

The researcher's cultural background and age may also have been variables that could have affected the results and her interpretation of them. Being Caucasian and American, she represented the dominant culture, which may have provoked anxiety in participants when asked to talk about themselves to her and possibly prove themselves in a strange situation. Furthermore, it was likely that she was around the same age as their

mothers, which may have had transference implications that affected the quality of their associations and the content of the artwork. However, the age difference may also have increased their level of comfort in speaking about themselves, given that most participants seemed very willing to speak openly about themselves and their feelings.

Participants were allowed to choose which approach to use first, which was also a delimitation. In three cases, the composition and content of the drawing seemed to reflect that of the collage. This raises the question of whether, and how, the first task might have influenced the second in terms of composition. There is also the possibility that the first one – in six cases in this study, the collage – raised material that evoked anxiety that influenced the content of the drawing and the pre-drawing nervousness level. Although participant responses did not seem to reflect this, it cannot be ruled out. Future research could examine the ways one approach influences the other, with a more in-depth interview that focuses on this aspect of the art-making experience. This would provide even more information about the characteristics of different approaches.

Implications for Future Research

This study has provided further understanding of how individuals unaccustomed to making art might respond to two popular art therapy approaches: collage and drawing.

- Future studies could examine how individuals unaccustomed to making art might respond to specific approaches after being given a chance to become familiar with them, since unfamiliarity with art materials on the one hand, and collage-making on the other, seems to have influenced some participants' anxiety ratings. A study allowing non-artist participants to make two pieces of art using the same approach might reveal information about how the chance to become more familiar with that approach affects the change in

anxiety levels. Research could examine how quickly participants become comfortable with different approaches, and whether, and in what situation, some seem to require more experimentation and practice than others.

- The preliminary collection of demographic data seems to have allayed some of the anxiety some participants felt coming into the study. Further research could be designed taking this into consideration, leaving the collection of demographic data to the end of the session so as to have more realistic, untainted, anxiety scores.
- Participants were very critical of their drawings, but seemed much less concerned with their collages. Self-criticism and expectations as to what constitutes an acceptable piece of art were significant factors influencing the drawing anxiety levels. A study asking participants to use and compare different approaches, rate their final art products, and then describe the reasons for their ratings would further our understanding of what individuals expect when using different materials. Knowledge of these expectations could enhance art therapists' understanding of what a new client is experiencing when asked to draw, sculpt in clay, make a collage or use found materials, for example.
- The marginally significant drop in anxiety levels during the collage portion of the study seems to support the finding that providing imagery and words as stimuli helps address much of the anxiety that accompanies the need to come up with ideas out of nothing. Further research focusing on this aspect of collage is needed to confirm these results.
- The directive in this study was broad enough that it could be interpreted in different ways, which may have helped address some of the anxiety present when making

the collage. Research could examine how a directive that specifically focuses on a negative experience might influence the way individuals respond to collage.

- Participants seemed to find the selection in this study, which consisted of 320 images and 150 words or phrases, to be ample. Two participants commented that a narrower selection would have been the source of some anxiety. Future studies into the collage-making experience could look at how participants respond to a smaller selection of images and words.

- In order to investigate how collage might relate to Yalom's (1985) therapeutic factors in group therapy, a study could examine how a group responds to a directive when each member is given the same selection of imagery from which to choose, compared to a group addressing the same directive but having to select their images from a common box.

- Since collage seems effective in addressing initial fears, in giving individuals a less intimidating way to introduce themselves to the therapist, and in acting as a bridge to creative communication, future research could examine the impact of having a collage component to a multi-part assessment. The study could examine the type of information gleaned as well as the effect of collage on the way clients approach the rest of the assessment.

- Further studies could use the same selection of images and words as this present study, to examine how different populations respond to the same stimuli when, for example, addressing different directives.

- Finally, further research is recommended into the nature of the different materials used in art therapy, and the connotations they possess to different groups. This also

applies to the new technologies that are making their way into the field. A deeper knowledge of how different groups might respond to a specific material can help streamline art therapy sessions, especially in these days when short-term therapy is becoming increasingly prevalent.

CHAPTER 6: SUMMARY AND CONCLUSIONS

The purpose of this study was to explore how non-artist adults experienced art making using two popular approaches, collage and drawing materials, with the objective of understanding how the characteristics of each approach might affect the anxiety experienced in initial art therapy sessions. Eight student volunteers, aged 18-27, were recruited for the study, which used a descriptive survey design in which participants were asked to make two pieces of art about themselves, one using collage and one using drawing materials. Before and after each art task, participants rated their level of anxiety on a scale of 0 – 5. The researcher then asked them to describe their experiences, using an open-ended interview guide. The results were first aggregated and summarized, then organized in terms of the research question.

Clinically significant findings were that drawing evoked significantly more anxiety than collage because of:

- preconceptions about what constituted an acceptable piece of art
- concerns that meaning could be lost or misunderstood when an idea was translated onto the page because of limited drawing abilities
- concerns about facing the challenge of the blank page and coming up with ideas for the artwork, and
- concerns about the challenge of having to communicate using the unfamiliar medium of drawing materials

Findings also indicate, however, that drawing:

- can be more personal, because the imagery produced is unique to the creator
- is not bound by rules or structure

- promotes creative problem-solving to address artistic limitations, and
- can provide a positive sensory experience that collage does not seem to offer.

Collage was approached with significantly less anxiety than drawing because:

- the process itself was perceived as being easier
- it provided structure in the form of visual stimuli, and
- it involved a controlled, cognitive process that allowed for the safe exploration of self, insight and self expression, and for the depiction of strengths.

Limitations identified when working with collage were that:

- it required the individual to work with someone else's selection and someone else's imagery, and
- it required a level of organization that seemed to exacerbate anxiety in some individuals.

Drawing seemed to evoke the fears of failure, being misunderstood, judged and shamed, or of revealing too much of oneself too soon that many clients new to therapy often experience. Findings indicate, however, that once these concerns have been addressed, drawing can offer a freer vehicle for communication. It also seems to provide more opportunity for individuals to express feeling states. Four drawings depicted a feeling state, whereas most collages dealt with self-identity.

Collage seemed to address many of the initial fears by providing structure in the ready-made images and a cognitive component that gave participants a sense of control over what was being revealed. Seven participants used collage to make pieces that were considered more metaphorical and revealing of emotional content. At the same time,

there were limitations imposed by the collage-making process, in that it compelled the artist to choose from images selected and prepared by someone else.

The results of this study suggest that in a clinical situation, when used with individuals who are not used to expressing themselves creatively:

- Collage can be used to address initial anxiety in new art therapy clients, and to engage them in therapy. It is a less threatening medium that can underscore the art therapist's attunement with the client and reinforce the fact that the goal is not to judge, but to understand, since it can address concerns about failing, being judged and not being in control, and allows for the expression of strengths.
- Drawing would be more effective in evoking anxiety when needed for the purposes of therapy, while collage can be used to address it.
- Collage can act as a bridge to creativity by taking the familiar and asking individuals to use it in a new way.
- Collage can also be used as a basis for the incorporation of less familiar materials.
- Collage is an approach that can be used with a range of populations and cultures, as long as the selection is appropriately put together, and incorporates images with which clients can identify.
- Once the client has gained some familiarity with drawing materials, however, and a therapeutic relationship has been established, it would appear that drawing can become a more useful tool for the expression and examination of emotions.

Further research is recommended into the nature of different materials, into the impact of familiarization on an individual's ability to communicate with a specific

medium, into the influence of directive on anxiety level when collage is being used, and into the influence of the same selection of imagery in a group situation.

List of References

- Alderfer, C.E. (2005). *A phenomenological exploration of the feelings induced by varying forms of art therapy structure within a single group session*. Unpublished Master's thesis of Drexel University; Philadelphia, PA.
- Amabile, T.M., & Gitomer, J. (1984). Children's artistic creativity: Effects of choice in task materials. *Personality and Social Psychology Bulletin*, 10(2), 209-215.
- Austin, D.S., & Dvorkin, J.M. (1993). Resistance in individual music therapy. *The Arts in Psychotherapy*, 20, 423-429.
- Boylan, J.B. (2001). *Exploring the use of two-dimensional and three-dimensional art media for increasing attentional abilities in the treatment of children diagnosed with attention deficit hyperactivity disorder*. Unpublished Master's thesis of Drexel University; Philadelphia, PA.
- Brodsky, S.L. (2011). *Therapy with coerced and reluctant clients*. Washington, DC: American Psychological Association.
- Bromberg, E.L. (2003). Brief report: The demographics and practices of art therapists working with cancer patients in the New York metropolitan area. *Art Therapy: Journal of the American Art Therapy Association*, 20(4), 219-225.
- Brubaker, M.M. (1996). *Degree of structure in a selected art therapy task for an institutionalized geriatric population*. Unpublished Master's thesis of Drexel University; Philadelphia, PA.
- Carr, M.B., & Vandiver, T.A. (2003). Effects of instructional art projects on children's behavioral responses and creativity within an emergency shelter. *Art Therapy: Journal of the American Art Therapy Association*, 20(3), 157-162.
- Comfort, C.E. (1985). Published pictures as psychotherapeutic tools. *Arts in Psychotherapy*, 12, 245-256.
- Crisp, H.L. (1997). *Exploring the use of three-dimensional media with hospitalized geriatric medical patients*. Unpublished Master's thesis of Drexel University; Philadelphia, PA.
- Foster, F. (1997). Fear of three-dimensionality: clay and plasticine as experimental bodies, in K. Killick & J. Schaverien (Eds), *Art, Psychotherapy and Psychosis* (pp. 52-71). London, GBR: Routledge.

- Foy, M. (1997). *The interpersonal reaction of groups with chronic schizophrenia to various art media and structured tasks*. Unpublished Master's thesis of Drexel University; Philadelphia, PA.
- Freud, S. (1913). On beginning the treatment: Further recommendations on the technique of psychoanalysis I, in *The complete psychological works*, standard ed., vol.12, pp.121-144. London, UK: The Hogarth Press.
- Gabbard, G.O. (2005). *Psychodynamic psychiatry in clinical practice* (4th ed.). Washington, DC: APA.
- Gladding, S.T., & Newsome, D.W. (2003). Art in counseling. In C.A. Malchiodi (Ed.), *Handbook of art therapy* (pp. 243-253). New York, NY: The Guilford Press.
- Gomes-Schwartz, B. (1978). Effective ingredients in psychotherapy: Prediction of outcome from process variables. *Journal of Consulting and Clinical Psychology*, 46(5), 1023-1035.
- Hartwich, P., & Brandecker, R. (1997). Computer-based art therapy with inpatients: Acute and chronic schizophrenics and borderline cases. *The Arts in Psychotherapy*, 24(4), 367-373.
- Henderson, P., Rosen, D., & Mascaro, N. (2007). Empirical study on the healing nature of mandalas. *Psychology of Aesthetics, Creativity and the Arts*, 1(3), 148-154.
- Hilsenroth, M.J., & Cromer, T.D. (2007). Practice Review: Clinician interventions related to alliance during the initial interview and psychological assessment. *Psychotherapy: Theory, Research, Practice, Training*, 44(2), 205-218.
- Horvath, A.O. (2001). The alliance. *Psychotherapy: Theory, Research, Practice, Training*, 38(4), 365-372.
- Horvath, A.O., Del Re, A.C., Fluckiger, C., & Symonds, D. (2011). Alliance in individual psychotherapy. *Psychotherapy* 48(1), 9-16.
- Horvath, A.O., & Symonds, B.D. (1991). Relation between working alliance and outcome in psychotherapy: A meta-analysis. *Journal of Counseling Psychology*. 38(2), 139-149.
- Johnston, K.A. (2007). *A phenomenological study exploring the cognitive and emotional experience of using magazine photo collage media in art therapy*. Unpublished Master's thesis of Drexel University; Philadelphia, PA.
- Kagin, S.L., & Lusebrink, V.B. (1978). The expressive therapies continuum. *Art Psychotherapy*, 5, 171-180.

- Kramer, E. (2001). Sublimation and art therapy. In J.A. Rubin (Ed.), *Approaches to art therapy: Theory and technique* (2nd ed.) (pp. 28-39).
- LaGiglia, J.A. (2007). *What is the cognitive and emotional experience of creating art work on different sizes of paper?* Unpublished Master's thesis of Drexel University; Philadelphia, PA.
- Landgarten, H.B. (1981). *Clinical art therapy: A comprehensive guide*. New York, NY: Brunner-Routledge.
- Landgarten, H.B. (1993). *Magazine photo collage: A multicultural assessment and treatment technique*. New York, NY: Brunner/Mazel, Inc.
- Linesch, D.G. (1988). *Adolescent art therapy*. New York, NY: Brunner/Mazel, Inc.
- Lowenfeld, V., & Brittain, W.L. (1987). *Creative and mental growth* (8th ed.). Upper Saddle River, NJ: Prentice-Hall, Inc.
- Lusebrink, V.B. (1990). *Imagery and visual expression in therapy*. New York, NY: Plenum Press.
- MacKinnon, R.A., Michels, R. & Buckley, P.J. (2006). *The psychiatric interview in clinical practice* (2nd ed.). Arlington, VA: American Psychological Association.
- Martin, D.J., Garske, J.P., & Davis, M.K. (2000). Relation of the therapeutic alliance with outcome and other variables: A meta-analytic review. *Journal of Counseling and Clinical Psychology*, 68(3), 438-450.
- McNeilly, G. (1983). Directive and non-directive approaches in art therapy. *The Arts in Psychotherapy*, 10, 211-219.
- Moon, C.H. (Ed.). (2010). *Materials and media in art therapy: Critical understandings of diverse artistic vocabularies*. New York, NY: Routledge.
- Moriarty, J. (1973). Collage group therapy with female chronic schizophrenic inpatients. *Psychotherapy: Theory, Research and Practice*, 10(2), 153-154.
- Naumburg, M. (1966). *Dynamically oriented art therapy: Its principles and practices*. New York, NY: Grune & Stratton.
- Patton, M.Q. (2002). *Qualitative research and evaluation methods*. Thousand Oaks, CA: Sage Publications.
- Radl, D.M. (2005). *Structured art therapy verses [sic] unstructured art therapy: A pilot study measuring the anxiety within a response during group therapy*. Unpublished Master's thesis of Drexel University; Philadelphia, PA.

- Restifo, S. (2010). Patients' performance anxiety and related aspects as factors in resistance to change. *Australasian Psychiatry*, 18(3), 226-229.
doi:10.3109/10398560903473678
- Robbins, A. (1987). *The artist as therapist*. Philadelphia, PA: Jessica Kingsley publishers.
- Robbins, A., & Sibley, L.B. (1976). *Creative art therapy*. New York, NY: Brunner/Mazel.
- Rubin, J.A. (2010). *Introduction to art therapy: Sources and resources*. New York, NY: Routledge.
- Rubin, L.C. (2000). The use of paint-by-number art in therapy. *The Arts in Psychotherapy*, 27(4), 269-272.
- Sarason, S.B. (1990). *The challenge of art to psychology*. New Haven, CT: Yale University Press.
- Schlossberg, I.A. (1983). The ego organizing qualities of various media in an acute psychiatric population. *Pratt Institute Creative Arts Therapy Review*, 4, 22-28.
- Shepard, B.C., & Guenette, F.L. (2010). Magazine picture collage in group supervision. *Canadian Journal of Counseling*, 44(3), 296-306.
- Stager, D. (2004). *A phenomenological study of visually impaired people's experiences of creating artwork using two and three-dimensional media*. Unpublished Master's thesis of Drexel University; Philadelphia, PA.
- Stallings, J.W. (2010). Collage as a therapeutic modality for reminiscence in patients with dementia. *Art Therapy: Journal of the American Art Therapy Association*, 27(3), 136-140.
- Stone, D.L. (1991). Self-perceptions of older adults as artists. *Activities, Adaptation & Aging*, 16(2), 31-37.
- Takata, Y. (2002). Supporting by a nurse teacher in a school infirmary using collage therapy. *Psychiatry and Clinical Neurosciences*, 56, 371-379.
- Tantam, D. (2002). *Psychotherapy and counseling in practice: A narrative framework*. UK: Cambridge University Press.
- Vick, R.M. (1999). Utilizing prestructured art elements in brief group art therapy with adolescents. *Art Therapy: Journal of the American Art Therapy Association*, 16(2), 68-77.
- Wadeson, H. (1980). *Art psychotherapy*. New York, NY: John Wiley & Sons.

- Wadeson, H. (1987). *The dynamics of art psychotherapy*. New York, NY: John Wiley & Sons.
- Wadeson, H. (2000). *Art therapy practice: Innovative approaches with diverse populations*. New York, NY: John Wiley & Sons, Inc.
- Waller, D. (1993). *Group interactive psychotherapy: Its use in training and treatment*. New York, NY: Routledge.
- Weiss, R.S. (1994). *Learning from strangers: The art and method of qualitative interview studies*. New York, NY: Free Press.
- Whalen, D. (2004). Meeting your metaphor: The use of the arts and the imagination with dying persons. In R.C.P. Magniant (Ed.), *Art therapy with older adults: A sourcebook* (pp. 101-121). Springfield, IL: Charles C. Thomas Pub. Ltd.
- Williams, B. (2000). Collage work as a medium for guided reflection in the clinical supervision relationship. *Nurse Education Today*, 20, 273-278.
- Wittenberg, D. (1974). Art therapy for adolescent drug abusers. *American Journal of Art Therapy*, 13(2), 114-149.
- Yalom, I.D. (1985). *The theory and practice of group psychotherapy* (3rd ed.). New York, NY: Basic Books.

Appendix A

[Drexel logo]

Recruiting Volunteers for a Research Study

Research Title:

A descriptive survey examining the compared experiences of non-artist adults working with collage and with drawing materials in an initial art making session

Research Objectives:

The purpose of this study is to describe a non-artist adult's experience of making a piece of art about him or herself using two approaches, collage and drawing materials, with the idea of understanding how the characteristics of each approach might influence the anxiety experienced in initial art therapy sessions.

Length of time of study:

Approximately two hours 15 minutes.

You can participate in this study if:

- you are 18-65 years of age
- you are enrolled at Drexel University as an undergraduate or graduate student
- you have no background in fine arts, and
- you are not currently enrolled in an art or art-related program.

Volunteers will be recruited on a first-come basis

Remuneration:

You will receive a \$30 gift certificate for Barnes and Noble once you have completed the session.

Location of the research and person to contact for further information:

The meeting will take place at the Drexel University Center City campus, Bellet Building, in a room to be specified when the data collection session is organized.

If you are interested in participating, please contact: Teresa Raffaelli, tel. (215) 327-9372. If nobody is available to take your call, you may leave a message and your call will be returned.

This research is approved by the Institutional Review Board.

This research is being conducted by a researcher who is a member of Drexel University

Appendix B

DREXEL UNIVERSITY Consent to Take Part In a Research Study

1. **NAME OF SUBJECT:** _____

2. **TITLE OF RESEARCH:** A descriptive survey examining the compared experiences of non-artist adults working with collage and with drawing materials in an initial art making session

3. **PRINCIPAL INVESTIGATOR:** Betty Hartzell, PhD, ATR-BC, LPC
CO-INVESTIGATOR: Teresa M. Raffaelli, BA

4. **RESEARCH ENTITY:** Drexel University.

5. **CONSENTING FOR THE RESEARCH STUDY:** This is a long and an important document. If you sign it, you will be authorizing Drexel University and its researchers to perform research studies on you. You should take your time and read it carefully. You can also take a copy of this consent form and discuss it with a family member, attorney or anyone else you would like to before you sign it. Do not sign it unless you are comfortable about participating in this study.

6. **PURPOSE OF RESEARCH:** You are being asked to participate in a research study. The purpose of this study is to describe a non-artist adult's experience of making a piece of art about him or herself using two approaches, collage and drawing materials, with the idea of understanding how the characteristics of each approach might affect the anxiety experienced in initial art therapy sessions. This study is in partial fulfillment of a master's degree in the Creative Arts Therapies Program at Drexel University's Center City Campus. Eight volunteers will be selected to participate. They will be selected in the order that they respond and qualify for the study. To qualify for the study, you must be between 18 and 65 years old and an undergraduate or graduate student at Drexel University. You will not qualify if you have a background in fine arts or if you are currently enrolled, on a full- or part-time basis, in any art or art-related program at Drexel University or any other institution. This is a voluntary study, so you may withdraw from it at any time.

7. **PROCEDURES AND DURATION:**
 You understand that the following things will be done to you.
 - You will meet with the researcher on one occasion for approximately two hours and 15 minutes. This meeting will be audiotaped. It will take place as follows:
 - The purpose and procedures of the study will be explained, the researcher will answer any questions you may have, you will be asked to explain what the

procedures are, using your own words, to make sure that you understand them, and you will be asked to sign two copies of this consent form. One copy will be given to you, and one will be kept in a locked cabinet in the Creative Arts Therapies office for a period of three years (approximately 15 minutes).

- The researcher will collect preliminary information, specifically: your age, gender, ethnicity and information on any art experience you may have (approximately 5 minutes).
- Art process: you will be asked to make two pieces of art, as follows:
 1. you will be asked to make a piece of art using collage materials
 - you will be asked to make a collage telling the researcher about yourself
 - you will be offered a range of magazine images, words and phrases, an 11 x 14-inch sheet of white paper, a glue stick, liquid glue and scissors
 - You will have approximately 30 minutes to make the collage
 2. you will be asked to make a drawing
 - you will be asked to make a drawing telling the researcher about yourself
 - you will be offered a selection of #2 pencils, colored pencils, thin markers, oil pastels and chalk pastels and an 11 x 14-inch sheet of white paper
 - You will have approximately 30 minutes to work on the drawing
- You will be able to choose whether to make the drawing or the collage first.
- Before and after each task, you will be asked to rate your level of anxiety.
- Between the two tasks, you will be offered a 10-minute break.
- After the art tasks have been completed, you will be asked a series of questions about what it was like to make the two art pieces (approximately 30-45 minutes).
- Total amount of time: approximately two hours and 15 minutes.

8. RISKS AND DISCOMFORTS/CONSTRAINTS:

You may feel some minimal anxiety or discomfort during the art making process, since you are not familiar with making art. While the interview is designed to ask questions about what it feels like to make the art, and not about personal issues, you may feel uncomfortable about some of the questions. If you do feel uncomfortable with any question, let the researcher know and she will move on to the next one. You may also choose to withdraw from the study at any time. In the unlikely event that you become excessively anxious following the data collection procedure, you can call the Drexel Student Counseling Center at 215-762-7625.

9. UNFORESEEN RISKS: Participation in this study may involve unforeseen risks. If the unforeseen risks occur, they will be reported to the Office of Research Compliance at 215-255-7857.

10. BENEFITS: There may be no direct benefit from participating in this study.

11. ALTERNATIVE PROCEDURES: The alternative is not to participate in this study.

12. REASONS FOR REMOVAL FROM THE STUDY:

You may be required to stop the study before the end for any of the following reasons:

- If all or part of the study is discontinued for any reason by the investigator, or university authorities.
 - If you are a student, and participation in the study is adversely affecting your academic performance.
 - If you fail to adhere to requirements for participation established by the researcher.
13. **VOLUNTARY PARTICIPATION:** Participation in this study is voluntary, and you can refuse to be in the study or stop at any time. There will be no negative consequences if you decide not to participate or if you decide to stop.
 14. **STIPEND:** A stipend will be provided by the researcher in the form of a \$30 gift card from Barnes and Noble if all the requirements for the art making and data collection are met. The card will be given to you at the end of the session.
 15. **RESPONSIBILITY FOR COSTS:** The researcher will provide any materials used during this study. The researcher will not cover transportation costs.
 16. **IN CASE OF INJURY:** If you have any questions or believe you have been injured in any way by being in this research study, you should contact Dr. Betty Hartzell at telephone number 215-762-3767. However, neither the investigator nor Drexel University will make payment for injury, illness, or other loss resulting from your being in this research project. If you are injured by this research activity, medical care including hospitalization is available, but may result in costs to you or your insurance company because the University does not agree to pay for such costs. If you are injured or have an adverse reaction, you should also contact the Office of Regulatory Research Compliance at 215-255-7857.
 17. **CONFIDENTIALITY:** In any publication or presentation of research results, your identity will be kept confidential, but there is a possibility that records which identify you may be inspected by authorized individuals such as representatives of the institutional review boards (IRBs) or employees conducting peer review activities. You consent to such inspections and to the copying of excerpts of your records, if required by any of these representatives. The artwork will be photographed to be included in the researcher's unpublished thesis. You have the choice of keeping your artwork. In the event that you do not wish to keep your artwork, it will be destroyed via shredding except for the copies. Audio recordings will be transcribed by the researcher and will be deleted at the conclusion of the study. Transcriptions will be stored in a secure, locked space in the offices of Drexel University's Department of Creative Arts Therapies for a period of three years. Any reference to you will be through a pseudonym, and not your name. Your contact information and telephone number will be stored in a secure, locked space in the offices of Drexel University's Department of Creative Arts Therapies, and will be destroyed at the conclusion of the study.

18. **OTHER CONSIDERATIONS:** If new information becomes known that will affect you or might change your decision to be in this study, you will be informed by the investigator. If you have any questions at any time about this study or your rights as a research subject, you may contact Dr. Betty Hartzell, Interim Director of Graduate Art Therapy Programs, at 215-762-3767, or the Office of Research Regulatory Compliance, at 215-255-7857.

19. CONSENT:

- I have been informed of the reasons for this study.
- I have had the study explained to me.
- I have had all my questions answered.
- I have carefully read this consent form, have initialed each page, and have received a signed copy.
- I give consent voluntarily.

 Subject

 Date

 Investigator or Individual Obtaining this Consent

 Date

 Witness

 Date
List of Individuals Authorized to Obtain Consent

<u>Name</u>	<u>Title</u>	<u>Day Phone #</u>	<u>24 Hr Phone #</u>
Betty Hartzell, PhD, Principal Investigator		215-762-3767	215-762-3767
Teresa Raffaelli, BA, Co-Investigator		215-327-9372	215-327-9372

Appendix C

PRELIMINARY INFORMATION

Participant: _____

Age: _____

Gender: M/F

Ethnicity: _____

What sort of art experience have you had?

Appendix D

ANXIETY SCALE

BEFORE THE DRAWING TASK:

On a scale of 0-5, how would you rate your level of anxiety or nervousness about having to tell me about yourself in a drawing?

0 1 2 3 4 5

AFTER THE DRAWING TASK:

On a scale of 0-5, how would you rate your level of anxiety or nervousness now?

0 1 2 3 4 5

BEFORE THE COLLAGE TASK:

On a scale of 0-5, how would you rate your level of anxiety or nervousness about having to tell me about yourself using collage materials?

0 1 2 3 4 5

AFTER THE COLLAGE TASK:

On a scale of 0-5, how would you rate your level of anxiety or nervousness now?

0 1 2 3 4 5

Appendix E

INTERVIEW GUIDE

- What was it like being asked to make a piece of art about yourself?
- What was it like to use collage materials for this project?
- How easy was it for you to say what you wanted to say about yourself with collage?
- What was it like to use drawing materials for this project?
- How easy was it for you to say what you wanted to say about yourself with drawing materials?
- How would you describe any differences there might have been between the two approaches?
- What were the similarities between the two approaches?
- What informed your choice of using [drawing materials/collage] for your first piece?

Appendix F

SAMPLE COLLAGE SHEET – IMAGES
(original size: 8.5 x 14 inches)

Appendix G

SAMPLE COLLAGE SHEET – WORDS AND PHRASES
 (original size: 8.5 x 14 inches)



